

1.

Nurse-Led Computerized Cognitive Training for Mild Cognitive Impairment: A Preliminary Study.

Oh MW, Ki YJ, Jeon BH, Kim SY, Oh Y, Yoo S, Cho A, Seo JH, Lee GJ

Rehabilitation Nursing Journal. 2020 04 15.

[Journal Article]

UI: 32304482

PURPOSE: This preliminary study aimed to compare the outcomes of an occupational therapist-led and a nurse-led computerized cognitive training (CCT) for mild cognitive impairment (MCI) in older adults.

DESIGN: A single-blind randomized controlled trial was performed.

METHODS: Participants 65 years of age and older with MCI were randomly assigned to a group led by an occupational therapist or by a nurse. Both groups received CCT for 4 weeks.

FINDINGS: Six participants in the occupational therapist-led group and nine in the nurse-led group completed CCT. The nurse-led group showed significant improvement in scores on the Seoul Verbal Learning Test-Elderly's version immediate recall scores ($p = .030$) and the Korean-Boston Naming Test ($p = .012$).

CONCLUSIONS: Nurse-led CCT demonstrated improvement in some language and memory areas in older adults with MCI.

CLINICAL RELEVANCE: This study supports the idea of educating nurses to use a CCT program for treating older adults with MCI to improve their cognitive function.

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2.

Shenqi Xingnao Granules ameliorates cognitive impairments and Alzheimer's disease-like pathologies in APP/PS1 mouse model.

Yang CC, Jia XY, Zhang L, Li YL, Zhang ZJ, Li L, Zhang L
Chinese Herbal Medicines. 12(4):421-429, 2020 Oct.

[Journal Article]

UI: 36120170

Objective: Alzheimer's disease (AD) is along with cognitive decline due to amyloid-beta (Abeta) plaques, tau hyperphosphorylation, and neuron loss. Shenqi Xingnao Granules (SQXN), a traditional Chinese medicine, significantly ameliorated the cognitive function and daily living abilities of patients with AD. However, till date, no study has investigated the mechanism of action of SQXN on AD. The present study aimed to verify the effects of SQXN treatment on cognitive impairments and AD-like pathologies in APP/PS1 mice.

Methods: Four-month-old APP/PS1 transgenic (Tg) mice were randomly divided into a model group and SQXN-treated (3.5, 7, 14 g/kg per day) groups. Learning-memory abilities were determined by Morris water maze and object recognition test. All mice were sacrificed and the brain samples were collected after 75 d. The soluble Abeta contents were detected by Elisa kit; The levels of expression of NeuN, APP, phosphorylated tau and related protein were measured by Western blotting; The inflammation factors were detected by the proinflammatory panel kit.

Results: Four-month-old APP/PS1 mice were administered SQXN by oral gavage for 2.5 months. Using the Morris water maze tests and Novel object recognition, we found that SQXN restored behavioral deficits in the experimental group of Tg mice when compared with the controls. SQXN also inhibited neuronal loss (NeuN marker). SQXN treatment decreased soluble Abeta42 through inhibiting the expression of sAPPbeta and BACE-1 without regulating full-length amyloid precursor protein (FL APP). Insulin degrading enzyme (IDE), the Abeta degrading enzyme, were increased by SQXN. In addition, SQXN reduced hyperphosphorylated tau protein levels and prevented excessive activation of p-GSK-3beta in the brain of APP/PS1 mice.

Compared with APP/PS1 transgenic negative mice, IFN-gamma, IL-1beta, IL-2, IL-4, IL-5, IL-6, IL-12p70, KC/GRO and TNF-alpha were not obviously changed in the brain of 6.5-month-old APP/PS1 transgenic (Tg) mice. However, SQXN could inhibited the expression of IL-2.

Conclusion: These results demonstrate that SQXN ameliorates the cognitive impairments in APP/PS1 mice. The possible mechanisms involve its inhibition of neuronal loss, soluble Abeta deposition, tau hyperphosphorylation and inflammation.

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3.

Psychological treatments for people with epilepsy. [Review]
Michaelis R, Tang V, Nevitt SJ, Wagner JL, Modi AC, LaFrance WC Jr, Goldstein LH, Gandy M, Bresnahan R, Valente K, Donald KA, Reuber M
Cochrane Database of Systematic Reviews. 8:CD012081, 2020 Sep 07.
[Journal Article. Review]
UI: 35653266
BACKGROUND: Given the significant impact epilepsy may have on the health-related quality of life (HRQOL) of individuals with epilepsy and their families, there is increasing clinical interest in evidence-based psychological treatments, aimed at enhancing psychological and seizure-related outcomes for this group. This is an updated version of the original Cochrane Review published in Issue 10, 2017.
OBJECTIVES: To assess the impact of psychological treatments for people with epilepsy on HRQOL outcomes.
SEARCH METHODS: For this update, we searched the following databases on 12 August 2019, without language restrictions: Cochrane Register of Studies (CRS Web), which includes randomized or quasi-randomized controlled trials from the

Specialized Registers of Cochrane Review Groups including Epilepsy, the Cochrane Central Register of Controlled Trials (CENTRAL), MEDLINE (Ovid, 1946 to 09 August 2019), and PsycINFO (EBSCOhost, 1887 onwards), and from PubMed, Embase, ClinicalTrials.gov, and the World Health Organization International Clinical Trials Registry Platform (ICTRP). We screened the references from included studies and relevant reviews, and contacted researchers in the field for unpublished studies.

SELECTION CRITERIA: We considered randomized controlled trials (RCTs) and quasi-RCTs for this review. HRQOL was the main outcome. For the operational definition of 'psychological treatments', we included a broad range of skills-based psychological treatments and education-only interventions designed to improve HRQOL, seizure frequency and severity, as well as psychiatric and behavioral health comorbidities for adults and children with epilepsy. These psychological treatments were compared to treatment as usual (TAU), an active control group (such as social support group), or antidepressant pharmacotherapy.

DATA COLLECTION AND ANALYSIS: We used standard methodological procedures expected by Cochrane.

MAIN RESULTS: We included 36 completed RCTs, with a total of 3526 participants. Of these studies, 27 investigated skills-based psychological interventions. The remaining nine studies were education-only interventions. Six studies investigated interventions for children and adolescents, three studies investigated interventions for adolescents and adults, and the remaining studies investigated interventions for adults. Based on satisfactory clinical and methodological homogeneity, we pooled data from 11 studies (643 participants) that used the Quality of Life in Epilepsy-31 (QOLIE-31) or other QOLIE inventories (such as QOLIE-89 or QOLIE-31-P) convertible to QOLIE-31. We found significant mean changes for the QOLIE-31 total score and six subscales (emotional well-being, energy and fatigue, overall QoL, seizure worry, medication effects, and cognitive functioning). The mean changes in the QOLIE-31 total score (mean improvement of 5.23 points, 95% CI 3.02 to 7.44; $P < 0.001$), and the overall QoL score (mean improvement of 5.95 points, 95% CI 3.05 to 8.85; $P < 0.001$) exceeded the threshold of minimally important change (MIC: total score: 4.73 points; QoL score: 5.22 points), indicating a clinically meaningful postintervention improvement in HRQOL. We downgraded the certainty of the evidence provided by the meta-analysis due to serious risks of bias in some of the included studies. Consequently, these results provided moderate-certainty evidence that psychological treatments for adults with epilepsy may enhance overall HRQOL.

AUTHORS' CONCLUSIONS: Implications for practice: Skills-based psychological interventions improve HRQOL in adults and adolescents with epilepsy. Adjunctive use of skills-based psychological treatments for adults and adolescents with epilepsy may provide additional benefits in HRQOL when these are incorporated into patient-centered management. We judge the evidence to be of moderate certainty.

IMPLICATIONS FOR RESEARCH: Investigators should strictly adhere to the CONSORT guidelines to improve the quality of reporting on their interventions. A thorough description of intervention protocols is necessary to ensure reproducibility. When examining the effectiveness of psychological treatments for people with epilepsy, the use of standardized HRQOL inventories, such as the Quality of Life in Epilepsy Inventories (QOLIE-31, QOLIE-31-P, and QOLIE-89) would increase comparability. Unfortunately, there is a critical gap in pediatric RCTs and RCTs including people with epilepsy and intellectual disabilities. Finally, in order to increase the overall quality of RCT study designs, adequate randomization with allocation concealment and blinded outcome assessment should be pursued. As attrition is often high in research that requires active participation, an intention-to-treat analysis should be carried out. Treatment fidelity and treatment competence should also be assessed. These important dimensions, which are related to 'Risk of bias' assessment, should always be reported.

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Other Abstract

Publisher

: (HRQOL) , . 2017 10 . : HRQOL . : 2019 8 12 . CRS (Cochrane Register of Studies, CRS Web) , Cochrane Central Register of Controlled Trials() , MEDLINE(Ovid, 1946 ~ 2019 8) PsycINFO(EBSCO , 1887) . ClinicalTrials.gov (WHO) (ICTRP) . : (RCT) RCT . HRQOL . ' ' , HRQOL , , (TAU), () , : : 3526 36 RCT . 27 . 9 . . 6, . 3, . 31(QOLIE-31) QOLIE-31 QOLIE (QOLIE-89 QOLIE-31-P) 11 (643) . QOLIE-31 6 (, , QoL , ,) . QOLIE-31 (5.23, 95% CI 3.02~7.44, P < 0.001), QoL (5.95, 95% CI 3.05~8.85, P < 0.001) (MIC:) . 4.73, QoL : 5.22)) HRQOL . , HRQOL . : : HRQOL . HRQOL . : : , (QOLIE-31, QOLIE-31-P, QOLIE-89) HRQOL . RCT RCT . , RCT

Language: Korean Publisher

: (health-related quality of life; HRQOL) , , , . 10, 2017 . : HRQOL . : 12 2019, , : (CRS Web), - , (CENTRAL) MEDLINE (Ovid 1946 09 2019) PsycINFO (EBSCOhost 1887) PubMed Embase ClinicalTrials.gov (ICTRP) . : (randomised controlled trials; RCTs) - RCT . HRQOL . << >>, HRQoL, (seizure) . , (treatment as usual; TAU), () , - . : : 36 RCT , 3526 . , 27 . . . , 11 (643) - 31 (QOLIE-31) QOLIE (QOLIE-89 QOLIE-31-P) QOLIE-31 , . QOLIE-31 ((well-being) , , QoL , ,) . QOLIE-31 (5.23 95% CI 3.02 7.44 P <0.001) QoL (5.95 95% CI 3.05 8.85 P < 0.001) : (MIC: : 4.73 QoL: 5.22) , HRQOL . (meta-analysis) (bias) , , HRQOL . : : , HRQOL . HRQOL - . : CONSORT . , HRQOL (QOLIE-31 QOLIE-31-P QOLIE-89) . RCT RCT . , , RCT . . . ,

(attrition) , (intention-to-treat) . . , << (bias)>> , .

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CONTEXTE: Etant donne l'impact significatif que l'épilepsie peut avoir sur la qualite de vie liee a la sante (QVLS) des personnes epileptiques et de leurs familles, il existe un interet clinique croissant pour les traitements psychologiques bases sur des donnees probantes visant a ameliorer les criteres de jugement psychologiques et lies aux crises pour ce groupe. Ceci est une version mise a jour de la revue Cochrane originale publiee dans le numero 10, 2017.OBJECTIFS: Evaluer l'impact des traitements psychologiques pour les personnes epileptiques sur la QVLS.

STRATEGIE DE RECHERCHE DOCUMENTAIRE: Pour cette mise a jour, nous avons effectue des recherches dans les bases de donnees suivantes et sans restrictions de langues jusqu'au 12 aout 2019: Le Registre Cochrane des etudes (CRS Web), qui comprend des essais controles randomises ou quasi randomises provenant des registres specialises des groupes de revues Cochrane, dont celui sur l'épilepsie, le registre Cochrane des essais controles (CENTRAL), MEDLINE (Ovid, de 1946 au 9 aout 2019) et PsycINFO (EBSCOhost, a partir de 1887), ainsi que PubMed, Embase, ClinicalTrials.gov et le Systeme d'enregistrement international des essais cliniques (ICTRP) de l'OMS. Nous avons examine les references des etudes incluses et des revues pertinentes, et nous avons contacte des chercheurs dans le domaine pour des etudes non publiees. CRITERES DE SELECTION: Nous avons pris en compte les essais controles randomises (ECR) et les quasi-ECR pour cette revue. La QVLS en a ete le principal critere de jugement. Pour la definition operationnelle de << traitement psychologiques >>, nous avons inclus un large eventail de traitements psychologiques bases sur les competences, et d'interventions educatives concues pour ameliorer la QVLS, la frequence et la gravite des crises, ainsi que les comorbidites psychiatriques et comportementales pour les adultes et les enfants epileptiques. Ces traitements psychologiques ont ete compares au traitement habituel (TH), a un groupe temoin actif (tel qu'un groupe de soutien social) ou a une pharmacotherapie antidepressive. RECUEIL ET ANALYSE DES

DONNEES: Nous avons suivi les procedures methodologiques standard definies par Cochrane. RESULTATS PRINCIPAUX: Nous avons inclus 36 ECR terminee, avec un total de 3 526 participants. Parmi ces etudes, 27 portaient sur des interventions psychologiques basees sur les competences. Les neuf autres etudes etaient des interventions a caractere educatif uniquement. Six etudes ont porte sur les interventions aupres des enfants et des adolescents, trois sur les interventions aupres des adolescents et des adultes, et les autres sur les interventions aupres des adultes. Sur la base d'une homogeneite clinique et methodologique satisfaisante, nous avons mis en commun les donnees de 11 etudes (643 participants) qui ont utilise le questionnaire sur la qualite de vie en cas d'épilepsie (QOLIE-31) ou d'autres questionnaires QOLIE (tels que QOLIE-89 ou QOLIE-31-P) convertibles en QOLIE-31. Nous avons constate des changements moyens significatifs pour le score total de QOLIE-31 et pour six sous-echelles (bien-etre emotionnel, energie et fatigue, qualite de vie globale, inquietudes liees aux crises, effets des medicaments et fonctionnement cognitif). Les changements moyens du score total de QOLIE-31 (amelioration moyenne de 5,23 points, IC a 95 %, de 3,02 a 7,44 ; $P < 0,001$), et du score global de QV (amelioration moyenne de 5,95 points, IC a 95 %, de 3,05 a 8,85 ; $P < 0,001$) ont depasse le seuil de changement important minimal (MIC) (MIC : score total : 4,73 points ; score de la qualite de vie: 5,22 points), ce qui indique une amelioration cliniquement significative de la qualite de vie liee a la sante apres l'intervention. Nous avons declasse le niveau de confiance des donnees probantes fournies par la meta-analyse en raison de risques serieux de biais dans certaines des etudes incluses. Par consequent, la valeur probante de ces resultats etait moderee a propos de l'amelioration de la qualite de vie globale chez les adultes epileptiques par les traitements psychologiques.CONCLUSIONS DES AUTEURS: Implications pour la pratique: Les interventions psychologiques basees sur les competences ameliorent la qualite de vie des adultes et des adolescents epileptiques. L'utilisation complementaire de traitements psychologiques bases sur

les competences chez les adultes et les adolescents epileptiques pourrait apporter des avantages supplementaires en matiere de qualite de vie liee a la sante lorsque ceux-ci sont integres dans une gestion centree sur le patient. Nous estimons que le niveau de confiance des donnees probantes est modere. Implications pour la recherche: Les enqueteurs doivent se conformer strictement aux recommandations CONSORT afin d'ameliorer la qualite des rapports sur leurs interventions. Une description complete des protocoles d'intervention est necessaire pour assurer la reproductibilite. Lors de l'examen de l'efficacite des traitements psychologiques pour les personnes souffrant d'epilepsie, l'utilisation de questionnaires standardises sur la qualite de vie, tels que les questionnaires sur la qualite de vie dans l'epilepsie (QOLIE-31, QOLIE-31-P et QOLIE-89), permettrait d'accroitre la comparabilite. Malheureusement, il existe une lacune importante dans les ECR pediatriques et les ECR incluant les personnes souffrant d'epilepsie et de handicap intellectuel. Enfin, afin d'ameliorer la qualite globale des plans d'etude des ECR, il convient de poursuivre une randomisation adequate avec assignation secrete et evaluation des criteres de jugement en aveugle. Comme le taux d'attrition est souvent eleve dans les recherches qui necessitent une participation active, il convient de proceder a une analyse des intentions de traitement. La fidelite au traitement et la competence en matiere de traitement doivent egalement etre evaluees. Ces dimensions importantes, qui sont liees a l'evaluation du << risque de biais >>, devraient toujours etre signalees.

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ANTECEDENTES: Debido al impacto significativo que podria tener la epilepsia sobre la calidad de vida relacionada con la salud (CdVRS) en los individuos con epilepsia y sus familias, hay un creciente interes clinico en los tratamientos psicologicos basados en evidencia, dirigidos a mejorar los desenlaces psicologicos y relacionados con las crisis para este grupo de personas. Esta es una version actualizada de la revision Cochrane original publicada en el numero 10, 2017.**OBJETIVOS:** Evaluar los efectos de los tratamientos psicologicos para los pacientes con epilepsia en los resultados de la CdVRS. **METODOS DE BUSQUEDA:** Para esta actualizacion, se realizaron busquedas en las siguientes bases de datos el 12 de agosto de 2019 sin restricciones de idioma: La Cochrane Register of Studies (CRS Web), que incluye ensayos controlados aleatorizados o cuasialeatorizados del registro especializado de los Grupos Cochrane de Revision de Epilepsia, Cochrane Central de Ensayos Controlados (CENTRAL), MEDLINE (Ovid, 1946 a 9 de agosto de 2019), y PsycINFO (EBSCOhost, 1887 en adelante), y de PubMed, EMBASE, ClinicalTrials.gov y la plataforma de registros internacionales de ensayos clinicos (ICTRP) de la Organizacion Mundial de la Salud. Se examinaron las referencias de los estudios incluidos y las revisiones pertinentes, y se establecio contacto con los investigadores del tema para obtener estudios no publicados. **CRITERIOS DE SELECCION:** Se consideraron los ensayos controlados aleatorizados (ECA) y los ensayos controlados cuasialeatorizados para esta revision. La CdVRS fue el desenlace principal. Para la definicion operativa de "tratamientos psicologicos", se incluyo un rango amplio de tratamientos psicologicos basados en habilidades e intervenciones solo educativas disenadas para mejorar la CdVRS, la frecuencia y la gravedad de las crisis, y las comorbilidades psiquiatricas y conductuales para los adultos y los ninos con epilepsia. Estos tratamientos psicologicos se compararon con el tratamiento habitual (TH), un grupo de control activo (como el grupo de apoyo social) o la farmacoterapia antidepressiva. **OBTENCION Y ANALISIS DE LOS DATOS:** Se utilizaron los procedimientos metodologicos estandares previstos por Cochrane. **RESULTADOS PRINCIPALES:** Se incluyeron 36 ECA completos con 3526 participantes. De estos, 27 estudios investigaron intervenciones psicologicas basadas en habilidades. Los nueve estudios restantes fueron intervenciones solo educativas. Seis estudios investigaron intervenciones para ninos y adolescentes, tres estudios investigaron intervenciones para adolescentes y adultos, y los restantes investigaron intervenciones en adultos. De acuerdo con una homogeneidad clinica y metodologica satisfactoria, se agruparon los datos de 11

estudios (643 participantes) que utilizaron la escala para medir la calidad de vida en la epilepsia Quality of Life in Epilepsy-31 (QOLIE-31) u otros listados QOLIE (como el QOLIE-89 o el QOLIE-31-P) convertibles a QOLIE-31. Se encontraron cambios medios significativos para la puntuacion total de la QOLIE-31 y seis subescalas (bienestar emocional, energia y fatiga, CdV general, preocupacion sobre la crisis, efectos de la medicacion y funcion cognitiva). Los cambios medios en la puntuacion total de la QOLIE-31 (mejora media de 5,23 puntos, IC del 95%: 3,02 a 7,44; $P < 0,001$), y la puntuacion global de la CdV (mejora media de 5,95 puntos, IC del 95%: 3,05 a 8,85; $P < 0,001$) superaron el umbral de cambio minimo importante (CMI: puntuacion total: 4,73 puntos; puntuacion de CdV: 5,22 puntos), lo que indica una mejoria posintervencion clinicamente significativa en la CdVRS. La certeza de la evidencia aportada por el metanálisis se redujo debido a riesgos graves de sesgo en algunos de los estudios incluidos. En consecuencia, estos resultados aportaron evidencia de certeza moderada de que los tratamientos psicologicos para los adultos con epilepsia pueden mejorar la CdVRS general.

CONCLUSIONES DE LOS AUTORES: Implicaciones para la practica: Las intervenciones psicologicas basadas en habilidades mejoran la CdVRS en adultos y adolescentes con epilepsia. El uso adyuvante de terapias psicologicas basadas en habilidades para los adultos y adolescentes con epilepsia podria proporcionar beneficios adicionales a la CdVRS cuando se incorporan a un tratamiento centrado en el paciente. Se considero que la evidencia era de certeza moderada. Implicaciones para la investigacion: Los investigadores deben adherirse estrictamente a las guias CONSORT para mejorar la calidad del informe sobre las intervenciones. Una descripcion minuciosa de los protocolos de intervencion es necesaria para asegurar la reproducibilidad. Cuando se examina la efectividad de los tratamientos psicologicos para los pacientes con epilepsia, el uso de listados estandarizados de CdVRS (QOLIE-31, QOLIE-31-P, y QOLIE-89) aumentaria la comparabilidad. Por desgracia, existe una laguna critica en los ECA pediatricos y ECA que incluyen a personas con epilepsia y discapacidades intelectuales. Finalmente, para aumentar la calidad general de los disenos de estudio de ECA, debe aplicarse una asignacion al azar adecuada con ocultacion de la asignacion y una evaluacion de resultados cegada. Debido a que la desercion a menudo es alta en la investigacion que requiere la participacion activa, debe realizarse un analisis de intencion de tratar. Tambien deben evaluarse la fidelidad al tratamiento y la competencia del tratamiento. Estas importantes dimensiones, relacionadas con la evaluacion del "riesgo de sesgo", deben ser siempre comunicadas.

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4.

Chinese herbal medicine Qinggongshoutao for the treatment of amnesic mild cognitive impairment: A 52-week randomized controlled trial.

Tian J, Shi J, Wei M, Ni J, Fang Z, Gao J, Wang H, Yao H, Zhang J, Li J, Min M, Su L, Sun X, Wang B, Wang B, Yang F, Zou Y, Hu Y, Lin Y, Xu G, Li K, Li L, Zhen H, Xu J, Chen K, Wang Y, CHARM study group

Alzheimer's & Dementia : Translational Research & Clinical Interventions. 5:441-449,

2019.

[Journal Article]

UI: 31517031

INTRODUCTION: This randomized, double-blind trial aimed to test effect of a Chinese herbal medicine, Qinggongshoutao (QGST) pill, on the cognition and progression of amnesic mild cognitive impairment (aMCI).

METHODS: Patients with aMCI were randomly assigned to receive QGST, Ginkgo biloba extract, or placebo for 52 weeks. The primary outcome measures were progression to possible or probable Alzheimer's disease (AD) and change in Alzheimer's Disease Assessment Scale-cognitive subscale scores; secondary outcome measures included assessments for cognition and function.

RESULTS: Total 350 patients were enrolled, possible or probable AD developed in 10. There were significant differences in the probability of progression to AD in the QGST group (1.15%) compared with placebo group (10%). There was significant difference in Alzheimer's Disease Assessment Scale-cognitive subscale scores in favor of QGST over the placebo group. Secondary outcome measure (Mini-Mental State Examination) also showed benefit in QGST at end point.

DISCUSSION: In patients with aMCI, QGST showed lower AD progression rate than placebo at 8.85%, and may have benefit on global cognition.

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Comments

Erratum in (EIN)

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5.

A pilot feasibility randomized controlled trial on combining mind-body physical exercise, cognitive training, and nurse-led risk factor modification to reduce cognitive decline among older adults with mild cognitive impairment in primary care.

Xu Z, Zhang D, Lee ATC, Sit RWS, Wong C, Lee EKP, Yip BHK, Tiu JYS, Lam LCW, Wong SYS

PeerJ. 8:e9845, 2020.

[Clinical Trial]

UI: 33194354

OBJECTIVES: To examine the feasibility and preliminary effectiveness of (1) combining cognitive training, mind-body physical exercise, and nurse-led risk factor modification (CPR), (2) nurse-led risk factor modification (RFM), and (3) health advice (HA) on reducing cognitive decline among older adults with mild cognitive impairment (MCI).

METHODS: It was a 3-arm open-labeled pilot randomized controlled trial in the primary care setting in Hong Kong. Nineteen older adults with MCI were randomized to either CPR (n = 6), RFM (n = 7), or HA (n = 6) for 6 months. The primary outcome was the feasibility of the study. Secondary outcomes included the Alzheimer's Disease Assessment Scale-Cognitive Subscale (ADAS-Cog), the Montreal Cognitive Assessment Hong Kong version (HK-MoCA), the Clinical Dementia Rating (CDR),

the Disability Assessment for Dementia (DAD), quality of life, depression, anxiety, physical activity, health service utilization, and diet.

RESULTS: Nineteen out the 98 potential patients were recruited, with a recruitment rate of 19% (95% CI [12-29]%, $P = 0.243$). The adherence rate of risk factor modification was 89% (95% CI [65-98]%, $P = 0.139$) for CPR group and 86% (95% CI [63-96]%, $P = 0.182$) for RFM group. In the CPR group, 53% (95% CI [36-70]%, $P = 0.038$) of the Tai Chi exercise sessions and 54% (95% CI [37-71]%, $P = 0.051$) of cognitive sessions were completed. The overall dropout rate was 11% (95% CI [2-34]%, $P = 0.456$). Significant within group changes were observed in HK-MoCA in RFM (4.50 ± 2.59 , $P = 0.008$), cost of health service utilization in CPR (-4000, quartiles: -6800 to -200, $P = 0.043$), fish and seafood in HA (-1.10 ± 1.02 , $P = 0.047$), and sugar in HA (2.69 ± 1.80 , $P = 0.015$). Group x time interactions were noted on HK-MoCA favoring the RFM group ($P = 0.000$), DAD score favoring CPR group ($P = 0.027$), GAS-20 favoring CPR group ($P = 0.026$), number of servings of fish and seafood ($P = 0.004$), and sugar ($P < 0.001$) ate per day.

CONCLUSIONS: In this pilot study, RFM and the multi-domain approach CPR were feasible and had preliminary beneficial effects in older adults with MCI in primary care setting in Hong Kong.

TRIAL REGISTRATION: Chinese Clinical Trial Registry (ChiCTR1800015324).

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6.

Depression after Subarachnoid Hemorrhage: A Systematic Review. [Review]
Tang WK, Wang L, Kwok Chu Wong G, Ungvari GS, Yasuno F, Tsoi KKF, Kim JS
Journal of Stroke. 22(1):11-28, 2020 Jan.

[Journal Article. Review]

UI: 32027789

BACKGROUND AND PURPOSE: Depression is common and debilitating illness accompanying many neurological disorders including non-traumatic subarachnoid hemorrhage (SAH). The aim of this systematic review was to identify and critically appraise all published studies that have reported the frequency, severity and time course of depression after SAH, the factors associated with its development and the impact of depression on patients' quality of life after SAH.

METHODS: The PubMed database was searched for studies published in English that recruited at least 40 patients (>18 years old) after SAH who were also diagnosed with depression.

RESULTS: Altogether 55 studies covering 6,327 patients met study entry criteria. The frequency of depression ranged from 0% to 61.7%, with a weighted proportion of 28.1%. Depression remained common even several years after the index SAH.

Depression after SAH was associated with female sex, premorbid depression, anxiety, substance use disorders or any psychiatric disorders, and coping styles. Comorbid cognitive impairment, fatigue, and physical disability also increased the risk of depression. Aneurysmal SAH and infarction may be related to depression as well. Depression reduces the quality of life and life satisfaction in patients after SAH.

CONCLUSIONS: Depression is common after SAH and seems to persist. Further research is needed to clarify its time course and identify the neuroendocrine and neurochemical factors and brain circuits associated with the development of post-SAH depression. Randomized controlled treatment trials targeting SAH-related depression are warranted.

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7.

Prevalence of Obesity in India and Its Neurological Implications: A Multifactor
Analysis of a Nationwide Cross-Sectional Study.

Venkatrao M, Nagarathna R, Majumdar V, Patil SS, Rath S, Nagendra H
Annals of Neurosciences. 27(3-4):153-161, 2020 Jul.

[Journal Article]

UI: 34556954

BACKGROUND: India is undergoing a rapid epidemiological transition, from
underweight to overweight/obese population. Obesity is a major risk factor in type 2
diabetes and cardiovascular diseases, and is also implicated as a factor in
neurological diseases such as Alzheimer's disease. A robust, pan-Indian estimate of
obesity is not yet available.

PURPOSE: This study estimates the pan-Indian prevalence of obesity, stratified
across nonmodifiable (age and gender) and modifiable (education and physical
activity levels) factors, and across zones and urban/rural.

METHODOLOGY: Data for 1,00,531 adults from a nationwide randomized cluster
sample survey (Niyantrita Madhumeha Bharata 2017, phase 1) were analyzed.

Obesity was determined using body mass index, and cross-tabulations were
calculated across zones, age, gender, education, physical activity, and area. To
determine statistical significance, t-tests were used. The odds of obesity within each
category of the various factors were calculated using binary logistic regression.

RESULTS: Prevalence of obesity in India is 40.3%. Zonal variations were seen as
follows: south highest at 46.51% and east lowest at 32.96%. Obesity was higher
among women than men (41.88% vs. 38.67%), urban than rural (44.17% vs.
36.08%), and over 40 than under 40 (45.81% vs. 34.58%). More education implied a
higher obesity (44.6% college vs. 38% uneducated), as did lowered physical activity
(43.71% inactive vs. 32.56% vigorously active). The odds ratio for physical activity
was 3.83, stronger than age (1.58), education (1.4), urban (1.3), and gender (1.2).

CONCLUSION: Obesity levels in India are very high, across all zones. The odds of
being obese increases with age, and is higher among women and among urban
dwellers. Obesity is the highest among aging urban men and women who are college
educated and are sedentary. Physical activity and aging are the strongest
determinants of obesity. Given the high cost of obesity in terms of type 2 diabetes,
cardiovascular diseases, and Alzheimer's disease, urgent public health measures are
necessary to reduce its impact.

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8.

Ameliorative Effects of Different Transcranial Electrical Stimulation Paradigms on the Novel Object Recognition Task in a Rat Model of Alzheimer Disease.

Zarifkar AH, Zarifkar A, Nami M, Rafati A, Aligholi H, Vafaei F

Galen Medical Journal. 8:e1440, 2019.

[Journal Article]

UI: 34466513

BACKGROUND: Treatment of Alzheimer as a disease that is associated with cognitive impairment has been associated with some restrictions. Recently, researchers have focused on non-pharmacological treatments, including non-invasive stimulation of the brain by transcranial electrical stimulation (tES). Four main paradigms of transcranial electrical current include transcranial direct current stimulation (tDCS), transcranial alternative current stimulation (tACS), transcranial random noise stimulation (tRNS), transcranial pulse current stimulation (tPCS). The tDCS is a possible new therapeutic option for patients with cognitive impairment, including Alzheimer disease.

MATERIALS AND METHODS: The study was done on Sprague-Dawley male rats weighing 250-270 g. to develop Alzheimer's model, the cannula was implanted bilaterally into the hippocampus. Abeta 25-35 (5µg/ 2.5µl/day) was microinjected bilaterally for 4 days. Then, an electrical stimulation paradigm was applied to the animal for 6 days. Animal cognitive capacity was evaluated on day 11 and 12 by novel object recognition (NOR) test.

RESULTS: Our results showed that application of tDCS; tACS; tRNS and tPCS reversed beta-amyloid-induced impairment ($P<0.05$). The tRNS Group spent total exploration time around the objects compared to other groups ($P<0.05$). There was

no significant difference between the four different paradigms in discrimination ratio and the percentage of total exploration time.

CONCLUSION: The results of this study showed that the use of multiple sessions of different tES paradigms could improve Abeta-induced memory impairment in the NOR test. Therefore, based on evidence, it can be expected that in addition to using tDCS, other stimulatory paradigms may also be considered in the treatment of AD.

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9.

Effect of Cuscuta epithymum Acquainted with Risperidone on the Improvement of Clinical Symptoms and Cognitive Impairment in Patients with Schizophrenia: A Triple-Blind Randomized Placebo-Controlled Trial.

Parvizi M, Fadaei F, Khodaei-Ardakani MR, Amin G, Abdi L, Noroozi M, Ansari I
Galen Medical Journal. 8:e1334, 2019.

[Journal Article]

UI: 34466495

BACKGROUND: Cuscuta epithymum (CE) is an established medicinal herb utilized for treating psychosis in Persian medicine. The aim of this study was to investigate the effect of CE combined with risperidone on the clinical symptoms and the cognitive impairment in patients diagnosed with schizophrenia.

MATERIALS AND METHODS: In this triple-blind randomized placebo-controlled trial, the intervention group received a dose of 500 mg of CE in the form of a capsule to be taken twice a day accompanied by an appropriate dose of risperidone. The control group was presented with a placebo identical to that of the CE capsule plus the

allocated dose of risperidone. The PANSS and SCoRS questionnaires were used to assess the status of subjects prior to the initiation of the intervention as well as being put to use at the end of the second, fourth, and eighth week post-intervention. Registering and recording intel concerning positive and negative symptoms felt by participants (PANNS), and a test to assess the cognitive impairment of the individuals.

RESULTS: After eight weeks of treatment, all negative and positive symptoms besides hostility and somatic concern exhibited a significant improvement in the CE group ($P < 0.05$). In contrast, the CE placebo group displayed no substantial improvement in the cases of the positive, negative and general symptoms ($P > 0.05$) regarding cognitive impairment, after eight weeks of treatment, all symptoms were greatly improved in the CE group ($P < 0.05$), while the effect of the placebo on the patients cognitive impairment remained mostly stationary ($P > 0.05$). Consequently, after eight weeks after the intervention, we can determine that the CE treatment has been noticeably more effective at improving positive, negative and cognitive symptoms of patients with schizophrenia.

CONCLUSION: The results of this study demonstrated that CE, possessing possible antioxidant and neuroprotective properties, safely improved the positive and negative symptoms, and cognitive impairment of patients with schizophrenia.

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Zeinivand M, Nahavandi A, Baluchnejadmojarad T, Roghani M, Golab F
Basic and Clinical Neuroscience. 11(6):795-804, 2020 Nov-Dec.

[Journal Article]

UI: 33850616

INTRODUCTION: Hepcidin is the main modulator of systemic iron metabolism, and its role in the brain has been clarified recently. Studies have shown that hepcidin plays an important role in neuronal iron load and inflammation. This issue is of significance because neuronal iron load and inflammation are pathophysiological processes that are highly linked to neurodegeneration. Moreover, the activity of hepcidin has recently been manipulated to recover the neuronal impairment caused by brain inflammation in animal models.

METHODS: Streptozotocin (STZ) was used to induce type 1 diabetes. Male Wistar rats (n = 40) with a weight range of 200-250 g were divided into control, diabetic, diabetic + insulin, and diabetic + dalteparin groups. Dalteparin (100 mg/kg IP) and insulin (100 mg/kg SC) were administered for 8 weeks. At the end of the experiment, Y-maze and passive avoidance tasks were carried out. The animals were perfused randomly and their hippocampal tissue was isolated for the analysis of markers such as lipid peroxidation like Malondialdehyde (MDA), hepcidin expression, iron, and ferritin. Blood samples were taken for the measurement of serum inflammatory cytokine Interleukin (IL)-6.

RESULTS: The findings indicated that treatment with dalteparin reduced IL-6, MDA, ferritin, and hepcidin expression in diabetic rats compared to treatment with insulin (P<0.05). Moreover, treatment with dalteparin did not decrease the iron level or prevented its decline.

CONCLUSION: Treatment with dalteparin improved the cognitive dysfunctions and symptoms of Alzheimer disease in STZ-induced diabetic rats by appropriately modulating and reducing oxidative stress and neuroinflammation. This may enhance the existing knowledge of therapeutics to reduce cognitive impairment in diabetes and is suggested to be a potential therapeutic agent in diabetes.

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11.

Association of Variants in PINX1 and TREM2 With Late-Onset Alzheimer Disease.
Tosto G, Vardarajan B, Sariya S, Brickman AM, Andrews H, Manly JJ, Schupf N,
Reyes-Dumeyer D, Lantigua R, Bennett DA, De Jager PL, Mayeux R
JAMA Neurology. 76(8):942-948, 2019 Aug 01.

[Journal Article]

UI: 31058951

IMPORTANCE: Genetic causes of late-onset Alzheimer disease (LOAD) are not completely explained by known genetic loci. Whole-exome and whole-genome sequencing can improve the understanding of the causes of LOAD and provide initial steps required to identify potential therapeutic targets.

OBJECTIVE: To identify the genetic loci for LOAD across different ethnic groups.

DESIGN, SETTING, AND PARTICIPANTS: This multicenter cohort study was designed to analyze whole-exome sequencing data from a multiethnic cohort using a transethnic gene-kernel association test meta-analysis, adjusted for sex, age, and principal components, to identify genetic variants associated with LOAD. A meta-analysis was conducted on the results of 2 independent studies of whole-exome and whole-genome sequence data from individuals of European ancestry. This group of European American, African American, and Caribbean Hispanic individuals participating in an urban population-based study were the discovery cohort; the additional cohorts included affected individuals and control participants from 2 publicly available data sets. Replication was achieved using independent data sets from Caribbean Hispanic families with multiple family members affected by LOAD and the International Genetics of Alzheimer Project.

MAIN OUTCOMES AND MEASURES: Late-onset Alzheimer disease.

RESULTS: The discovery cohort included 3595 affected individuals, while the additional cohorts included 5931 individuals with LOAD and 5504 control participants. Of 3916 individuals in the discovery cohort, we included 3595 individuals (1397 with LOAD and 2198 cognitively healthy controls; 2451 [68.2%] women; mean [SD] age, 80.3 [6.83] years). Another 321 individuals (8.2%) were excluded because of non-LOAD diagnosis, age younger than 60 years, missing covariates, duplicate data, or genetic outlier status. Gene-based tests that compared affected individuals ($n = 7328$) and control participants ($n = 7702$) and included only rare and uncommon variants annotated as having moderate-high functional effect supported PINX1 (8p23.1) as a locus with gene-wide significance ($P = 2.81 \times 10^{-6}$) after meta-analysis across the 3 studies. The PINX1 finding was replicated using data from the family-based study and the International Genetics of Alzheimer Project. Full meta-analysis of discovery and replication cohorts reached a P value of 6.16×10^{-7} for PINX1 (in 7620 affected individuals vs 7768 control participants). We also identified TREM2 in an annotation model that prioritized highly deleterious variants with a combined annotation dependent depletion greater than 20 ($P = 1.0 \times 10^{-7}$).

CONCLUSIONS AND RELEVANCE: This gene-based, transethnic approach identified PINX1, a gene involved in telomere integrity, and TREM2, a gene with a product of an immune receptor found in microglia, as associated with LOAD. Both genes have well-established roles in aging and neurodegeneration.

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1

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12.

Bufo viridis secretions improve anxiety and depression-like behavior following intracerebroventricular injection of amyloid beta.

Shirzad S, Neamati A, Vafaei F, Ghazavi H

Research in Pharmaceutical Sciences. 15(6):571-582, 2020 Dec.

[Journal Article]

UI: 33828600

BACKGROUND AND PURPOSE: Venenum Bufonis is a Chinese traditional medicine produced from the glandular secretions of toads that contain biogenic amines, which have anti-inflammatory properties. The present study aimed to examine the effect of Bufo viridis secretions (BVS) on anxiety and depression-like behavior and hippocampal senile plaques volume in an animal model of Alzheimer's disease (AD).

EXPERIMENTAL APPROACH: Thirty-eight male Wistar rats were used. AD was induced by amyloid-beta (A β 1-42) (10 μ g/2 μ L, intracerebroventricular injection, icv) and then BVS at 20, 40, and 80 mg/kg were injected intraperitoneally (ip) in six equal intervals over 21 days. Anxiety and depression-like behavior were assessed using behavioral tests including open field test (OFT), elevated plus maze (EPM), and forced swimming test (FST) 21 days after the surgery. The volume of

senile plaques was assessed based on the Cavalieri principle.

FINDINGS/RESULTS: Results of the OFT showed that the central crossing number and the time in the AD group were significantly decreased compared to the sham group ($P < 0.01$ and $P < 0.001$, respectively). Also, the values of these two parameters significantly increased in the AD + BVS80 group than the AD group ($P < 0.05$ and $P < 0.001$, respectively). The time spent in the closed arm in the EPM dramatically increased in the AD group compared to the sham group ($P < 0.05$) and significantly decreased in the AD + BVS80 group compared to the AD group ($P < 0.05$). Results of the FST indicated that immobility time had a reduction in the AD + BVS20 ($P < 0.01$), AD + BVS40, and AD + BVS80 groups compared to the AD group ($P < 0.001$). The volume of senile plaques in the hippocampus showed a reduction in the treatment groups in comparison with the AD group ($P < 0.001$ for all).

CONCLUSION AND IMPLICATIONS: Results revealed that BVS injection could improve symptoms of anxiety and depression and decrease senile plaques in the hippocampus in an animal model of AD.

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Year of Publication

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13.

Endoscopic Third Ventriculostomy: Role of Image Guidance in Reducing the Complications.

Wasi MSI, Sharif S, Shaikh Y

Asian Journal of Neurosurgery. 15(4):926-930, 2020 Oct-Dec.

[Journal Article]

UI: 33708664

INTRODUCTION: Endoscopic third ventriculostomy (ETV) is performed by neurosurgeons around the world for the management of hydrocephalus. ETV has been associated with multiple complications, the most significant being iatrogenic

injury to the fornix. We aim to establish the fact that the use of image guidance while planning a trajectory can reduce the incidence of complications as it significantly alters the usual approach for ETV, i.e., the coronal burr hole can be useful for young neurosurgeons to overcome the learning curve associated with the procedure.

MATERIALS AND METHODS: This is a prospective, observational study conducted at Liaquat National Hospital. In this study, 43 patients were included who underwent ETV for hydrocephalus. Complications were divided into three major groups: arterial hemorrhage, venous hemorrhage, and injury to neural structures (fornix, hypothalamus, and oculomotor nerve). The data were compared with studies showing the complications of ETV with and without usage of image guidance.

RESULTS: Among the 43 patients who underwent ETV with image guidance, only two patients (4.65%) had iatrogenic fornix contusions. Neither of them developed memory impairment. None of the patients (0%) encountered other major iatrogenic complications, including injury to the mammillary body, basilar artery, or oculomotor nerve.

CONCLUSION: The use of image guidance can reduce trajectory-related complications, including hemorrhage and iatrogenic injuries to the fornix. This study showed that the altered trajectory was beneficial in avoiding major neurological structures while introducing an endoscope through the cortex into the ventricular system.

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14.

Efficacy of Bacopa Monnieri (Brahmi) and Donepezil in Alzheimer's Disease and Mild Cognitive Impairment: A Randomized Double-Blind Parallel Phase 2b Study.

Prabhakar S, Vishnu VY, Modi M, Mohanty M, Sharma A, Medhi B, Mittal BR, Khandelwal N, Goyal MK, Lal V, Singla R, Kansal A, Avasthi A

Annals of Indian Academy of Neurology. 23(6):767-773, 2020 Nov-Dec.

[Journal Article]

UI: 33688125

OBJECTIVES: Alzheimer's disease (AD) is the most common cause of dementia worldwide in the older population. There is no disease-modifying therapy available for AD. The current standard of care drug therapy for AD is cholinesterase inhibitors,

including donepezil. Bacopa monnieri or brahmi is used in traditional Indian medicine for memory loss. We conducted a phase 2b randomized controlled trial (RCT) to find out the efficacy of brahmi and donepezil in AD and mild cognitive impairment (MCI).

PATIENTS AND METHODS: The study was planned as a 52 week, randomized, double-blind, parallel-group, phase-2 single-center clinical trial comparing the efficacy and safety of Bacopa monnieri (brahmi) 300 mg OD and donepezil 10 mg OD for 12 months in 48 patients with AD and MCI-AD including cognitive and quality of life outcomes. The primary outcome was differences in the change from baseline of the neuropsychological tests [Alzheimer's disease assessment scale-cognitive subscale (ADAS-Cog) and postgraduate institute (PGI) memory scale] at 12 months between the intervention group (brahmi) and active comparison group (donepezil).

RESULTS: The study was terminated after 3 years and 9 months, after recruiting 34 patients, because of slow recruitment and a high dropout rate. Intention to treat analysis after adjusting for baseline confounders showed no difference in the rate of change in ADAS-Cog score from baseline at any time point, including the last follow-up. There was no difference in the rate of change in PGI Memory scale (PGIMS) at 3, 6, and 9 months. In the last follow-up, there was a significant difference in the change in total PGIMS score between brahmi and donepezil, while there was no difference in individual scores of the PGI memory scale.

CONCLUSION: This phase-2 RCT on the efficacy of brahmi vs. donepezil showed no significant difference between them after 1 year of treatment. Larger phase-3 trials, preferably multicentric, are required to find the superiority of brahmi over donepezil.

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15.

Heterogeneous Influence of Frailty Phenotypes in Age-Related Hearing Loss and Tinnitus in Chinese Older Adults: An Explorative Study.

Ruan Q, Chen J, Zhang R, Zhang W, Ruan J, Zhang M, Han C, Yu Z

Frontiers in Psychology. 11:617610, 2020.

[Journal Article]

UI: 33664689

BACKGROUND: Fried physical frailty, with mobility frailty and non-motor frailty phenotypes, is a heterogeneous syndrome. The coexistence of the two phenotypes and cognitive impairment is referred to as cognitive frailty (CF). It remains unknown whether frailty phenotype has a different association with hearing loss (HL) and tinnitus.

METHODS: Of the 5,328 community-dwelling older adults, 429 participants aged ≥ 58 years were enrolled in the study. The participants were divided into robust, mobility, and non-mobility frailty, mobility and non-mobility CF (subdivided into reversible and potentially reversible CF, RCF, and PRCF), and cognitive decline [subdivided into mild cognitive impairment (MCI) and pre-MCI] groups. The severity and presentations of HL and/or tinnitus were used as dependent variables in the multivariate logistic or nominal regression analyses with forward elimination adjusted for frailty phenotype stratifications and other covariates.

RESULTS: Patients with physical frailty (mobility frailty) or who are robust were found to have lower probability of developing severe HL and tinnitus, and presented HL and/or tinnitus than those with only cognitive decline, or CF. Patients with RCF and non-mobility RCF had higher probability with less HL and tinnitus, and the presentation of HL and/or tinnitus than those with PRCF and mobility RCF. Other confounders, age, cognitive and social function, cardiovascular disease, depression, and body mass index, independently mediated the severity of HL and tinnitus, and presented HL and/or tinnitus.

CONCLUSION: Frailty phenotypes have divergent association with HL and tinnitus. Further research is required to understand the differential mechanisms and the personalized intervention of HL and tinnitus.

CLINICAL TRIAL REGISTRATION: ClinicalTrials.gov identifier, NCT2017K020.

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16.

Comparing Copper Serum Level and Cognitive Functioning in Patients With Schizophrenia and Healthy Controls.

Mazhari S, Arjmand S, Eslami Shahrabaki M, Karimi Goughari E
 Basic and Clinical Neuroscience. 11(5):649-657, 2020 Sep-Oct.
 [Journal Article]

UI: 33643558

INTRODUCTION: The altered serum profiles of several trace elements have been reported in Schizophrenia (SCZ). This study was designed to elucidate whether the serum levels of Copper (Cu) and Magnesium (Mg), the two essential trace elements which contribute to neurotransmitter transmission, are altered in patients with SCZ. We also investigated whether there is an interrelation between cognitive functioning and the serum levels of Cu and Mg.

METHODS: Sixty patients with SCZ and 30 healthy controls participated in this study. The patient group was divided into the following: i) early patients (n=35, <=5 years of illness initiation), and ii) chronic patients (n=25, >=5 years of illness duration). The serum levels of Cu and Mg were measured by atomic absorption spectroscopy and ion-selective electrode potentiometry, respectively. To assess cognitive abilities, a Persian adaptation of the Brief Assessment of Cognition in Schizophrenia (BACS) was administered.

RESULTS: The present research results revealed significantly higher Cu serum levels in both patient groups [early patients (M=94.6), chronic patients (M=97.5)], compared to the controls (M=71.0) (P<0.001); however, no significant difference was observed among the study groups for Mg [patients with the recent onset (M=2.0), chronic patients (M=2.0), and controls (M=1.9)] P=0.1. While the serum Cu profile of

healthy individuals revealed a negative correlation with working memory ($r=-0.42$, $P=0.02$), and executive functioning ($r=-0.40$, $P=0.03$), no significant correlation was observed between Cu serum levels of patients and BACS cognitive domains.

CONCLUSION: findings suggested that the high Cu serum concentration might impact the cognitive decline in healthy individuals; however, no significant correlation was observed in the Patients; i.e. most likely because cognition is severely impaired in SCZ. Additional studies examining trace elements in drug-naive patients with SCZ are required.

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17.

Risk of Fall among Older Adults and its Association with Cognitive Impairment in a Semi-Urban Community.

Monachan D, Vargese SS, Johny V, Mathew E

Indian Journal of Community Medicine. 45(4):463-466, 2020 Oct-Dec.

[Journal Article]

UI: 33623202

CONTEXT: Fall in older people is a major public health concern. Two-third of the death due to fall are preventable. Risk assessment in older adults therefore is the first step to identify the high-risk group to plan need-based intervention.

AIMS: The aim of the study was to determine the prevalence of risk of fall among older adults and its association with cognitive impairment and sociodemographic characteristics.

SETTINGS AND DESIGN: A cross-sectional study was conducted in the field practice areas of the department of community medicine in a teaching hospital in

South Kerala, India.

SUBJECTS AND METHODS: A semi-structured questionnaire was used to collect the data. Berg Balance Scale and Mini-Cog Test were used for measuring risk of fall and cognitive impairment.

STATISTICAL ANALYSIS USED: Descriptive statistics and logistic regression were used for the statistical analysis using SPSS.

RESULTS: Among the older adults, 45% were at risk of fall, 42.4% in males and 57.6% in females. The risk of fall was found to be significantly associated with cognitive impairment, (odds ratio = 3.89, confidence interval at 95% = 2.06-7.31, $P < 0.001$). Advanced age, female gender, and unemployed status were significantly associated with the risk of fall.

CONCLUSIONS: The risk of fall prevalence was high and significantly related to cognitive impairment, advanced age, female gender, and occupational status, with more than half of those currently not working having a higher risk. The study would recommend regular follow-up of risk groups for prevention a good percentage of fall and thereby the related injuries.

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18.

Differential Effect of Amyloid Beta1-40 on Short-term and Long-term Plasticity in Dentate Gyrus of a Rat Model of Alzheimer Disease.

Fahanik-Babaei J, Baluchnejadmojarad T, Roghani M

Basic and Clinical Neuroscience. 11(4):517-524, 2020 Jul-Aug.

[Journal Article]

UI: 33613890

INTRODUCTION: Synaptic plasticity is inappropriately affected by neurodegenerative diseases, including Alzheimer Disease (AD). In this study, we examined the effect of intrahippocampal amyloid-beta (Abeta1-40) on dentate gyrus Long-term Potentiation (LTP) and presynaptic short-term plasticity in a rat model of AD.

METHODS: The experimental groups in this research included the control with no treatment, sham-operated receiving the vehicle (normal saline), and Abeta-lesioned groups. For modeling AD, aggregated Abeta1-40 (10 mug/2 mul on each side) was injected into the hippocampal CA1. Three weeks later, Population Spike (PS) amplitude and slope ratios were determined at different Inter-pulse Intervals (IPI) of 10, 20, 30, and 50 ms as a valid indicator of the short-term presynaptic facilitation and/or depression. In addition, PS amplitude and slope were taken as an index of long-term synaptic plasticity after application of High-frequency Stimulation (HFS) to induce LTP in the medial perforant-dentate gyrus pathway.

RESULTS: No significant differences were noted amongst the experimental groups regarding fEPSP slope and paired-pulse indices as indicators of short-term plasticity. In contrast, fEPSP slope and PS amplitude significantly decreased following the application of HFS in Abeta-injected group. In addition, there was no significant difference between the control and sham-operated groups regarding the mentioned parameters.

CONCLUSION: Findings of this study clearly demonstrated that microinjection of Abeta1-40 into the CA1 could impair LTP in dentate gyrus but could not modify short-term plasticity.

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19.

Crocini Treatment after Maternal Hypoxia Attenuates Spatial Memory Impairment and Expression of BACE1 and HIF-1alpha in Rat Offspring Brain.

Fahimi Truski F, Ghotbeddin Z, Tabandeh MR, Pourmahdi Borujeni M

Basic and Clinical Neuroscience. 11(4):499-506, 2020 Jul-Aug.

[Journal Article]

UI: 33613888

INTRODUCTION: Hypoxia via expression of Hypoxia-Inducible Factor-1 (HIF-1) is an important and effective factor in the onset and progression of memory disorders, such as Alzheimer Disease (AD). The activity of beta-secretase (BACE1) is increased in hypoxia conditions. BACE1 triggers a cascade of pathological events resulting in AD. Crocin acts as a memoryimproving agent but its molecular mechanism is not well-known. Therefore, in this study, the effect of crocin on spatial memory, HIF-1alpha, and BACE1 gene expression was investigated in rat offspring under maternal hypoxia.

METHODS: Female pregnant rats on the 20th day of pregnancy were divided into 4 groups, including sham, crocin-treated, hypoxia, and hypoxia group treated with crocin. In the hypoxia groups, pregnant rats were exposed to 7% oxygen and 93% nitrogen intensity for 3 h. In the crocin-treated group, crocin (30 mg/kg) was injected at P14-28 (i.p). At the end, Morris water maze was used to assess spatial memory and real-time polymerase chain reaction was performed to measure the expression of BACE1 and HIF-1alpha genes in the brain of offspring.

RESULTS: Maternal hypoxia impaired memory compared with the sham group. However, crocin treatment improved cognitive behavior. HIF-1alpha and BACE1 expressions were upregulated in the brain of offspring in the hypoxia group. Crocin treatment could attenuate the expression of both genes.

CONCLUSION: According to our results, down-regulation of HIF-1alpha and BACE1 gene expressions in the brain of rat offspring after crocin treatment can be suggested as a molecular mechanism for crocin to improve spatial memory.

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Protective Effects of Pelargonium graveolens Essential Oil on Methomyl-Induced Oxidative Stress and Spatial Working Memory Impairment in Association with Histopathological Changes in the Hippocampus of Male Wistar Rats.

Amine KM, Kahina C, Nawel H, Faiza Z, Jean G, Mohamed T, Saadia M, Ahsene B
Basic and Clinical Neuroscience. 11(4):433-446, 2020 Jul-Aug.

[Journal Article]

UI: 33613881

INTRODUCTION: Methomyl (MET) is a carbamate insecticide, used in agriculture and public health to eliminate harmful insects. Besides its advantages in agriculture, it causes neurotoxic effects. The aim of this study was to evaluate the effect of MET on Spatial Working Memory (SWM), oxidative stress parameters, and histopathological changes in the hippocampus, as well as the possible protective role of Pelargonium graveolens Essential Oil (EO).

METHODS: Male Wistar rats were randomized into four groups of six animals: group I as the control that received the vehicle; group II received EO (75 mg/kg b.w), group III received MET (2 mg/kg b.w); and group IV received both MET and EO. The rats were administered the respective doses orally by gavage for 28 days. SWM was assessed using Y-maze on the day before the first treatment and day 28 after the last dose. They were sacrificed by decapitation and their brains were taken for assessing oxidative stress parameters and histopathological analysis.

RESULTS: MET treatment caused SWM deficits. Furthermore, drastic changes were observed in aspartate transaminase, alanine aminotransferase, and alkaline phosphatase activities. The level of malondialdehyde significantly increased, whereas antioxidant (glutathioneS-transferase and catalase) enzyme activities significantly decreased. The CA1 region of the hippocampus of rats exposed to MET revealed severe histological alterations. However, supplementation with EO improved SWM and partially restored the activities of antioxidant systems and prevented neuronal cell damage.

CONCLUSION: P. graveolens EO has the potential in mitigating most of the adverse effects in the hippocampus and prevents SWM impairment induced by MET toxicity.

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Year of Publication
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21.

Ibuprofen Protection Against Restrained Chronic Stress-induced Depression in Male Rats.

Nozari M, Nahavandi A, Zeinivand M, Eslami Gharaati M, Godarzi M, Ahmadi M, Jamali-Raeufy N

Basic and Clinical Neuroscience. 11(4):413-422, 2020 Jul-Aug.

[Journal Article]

UI: 33613879

INTRODUCTION: Stress predisposes organisms to depression and cognitive impairments, and seems to interact with metabolic homeostasis. The inflammatory response and the upregulation of proinflammatory cytokines are some of the consequences related to chronic stress. In this study, we investigated the preventive effect of chronic administration of ibuprofen, as an inhibitor of cyclooxygenases, on the cognitive and behavioral alterations and the weight gain reduction induced by simultaneous chronic restraint stress in rats.

MATERIALS AND METHODS: Male Wistar rats were subjected to chronic restraint stress and injected daily with the variable doses of ibuprofen or vehicle, for 21 consecutive days. Then, all animals were tested with the forced swim test and passive avoidance conditioning. Also, the weight of the animals was recorded before and after the interventions. Ultimately, plasma interleukin 6 (IL-6) levels were measured.

RESULTS: Chronic stress increased depressive-like behaviors, impaired learning, and disrupted the normal weight gain. However, the animals that received the highest dose of ibuprofen showed less depressive-like behaviors, a better avoidance memory, and a higher weight gain. However, the level of plasma IL-6 did not differ significantly between the study groups.

CONCLUSION: The administration of ibuprofen prevents the cognitive and behavioral consequences of chronic stress. During the recovery, the plasma levels of IL-6 were not elevated by stress, and the IL-6 levels did not predict the behavioral performance of the stressed animals. The exact mechanisms of the protective effects of ibuprofen against chronic stress need to be further investigated.

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22.

Advancing family dementia caregiver interventions in low- and middle-income countries: A pilot cluster randomized controlled trial of Resources for Advancing Alzheimer's Caregiver Health in Vietnam (REACH VN).

Hinton L, Nguyen H, Nguyen HT, Harvey DJ, Nichols L, Martindale-Adams J, Nguyen BT, Nguyen BTT, Nguyen AN, Nguyen CH, Nguyen TTH, Nguyen TL, Nguyen ATP, Nguyen NB, Tiet QQ, Nguyen TA, Nguyen PQ, Nguyen TA, Pham T

Alzheimer's & Dementia : Translational Research & Clinical Interventions.

6(1):e12063, 2020.

[Journal Article]

UI: 33532542

INTRODUCTION: Low- and middle-income countries have rapidly increasing numbers of people with dementia, yet little evidence on family caregiving interventions. We tested the preliminary efficacy and feasibility of a family caregiving

intervention in northern Vietnam.

METHODS: Nine clusters comprising 60 family caregivers were randomized to a culturally adapted version of a four- to six-session, multicomponent intervention delivered in-home over 2 to 3 months, or enhanced control. Eligible caregivers were ≥ 18 years of age and scored ≥ 6 on the Zarit Burden Inventory (ZBI).

RESULTS: Fifty-one caregivers (85%) completed the study. Using analysis of covariance with 3-month assessment as the outcome and baseline assessment as a covariate, intervention group caregivers had an average ZBI (primary outcome) score 1.2 standard deviation (SD) lower ($P = .02$) and Patient Health Questionnaire-4 (psychological distress) score 0.7 SD lower ($P = .03$) than controls.

DISCUSSION: In the first study of its kind in Vietnam, a culturally adapted, manualized, family caregiver intervention was both efficacious and feasible.

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23.

Effect of Watermelon Extract on Nerve Conduction Velocity, Memory, and T4 Level in Rats.

Jafari Nejad Bajestani M, Yousefi M, Hadjzadeh MA, Hosseini M, Taghipour A, Yousefvand S

Advanced Biomedical Research. 9:53, 2020.

[Journal Article]

UI: 33457336

BACKGROUND: Watermelon is a fruit with the thirst-quenching effect and especially is consumed in summer. In this study, the effect of consumption of watermelon extract (in drinking water) on thyroid hormone level (TT4), animals' weight, water and food consumption, nerve conduction velocity (NCV), and memory in Wistar rats were investigated.

MATERIALS AND METHODS: Twenty-four male Wistar rats were divided into three groups: control, CT 1500 mg/kg, and CT 3000 mg/kg (n = 8, in each group). Watermelon extract was administered for 42 days. The results measured in this study were analyzed with repeated measures or one-way analysis of variance and compared by the least significant difference post hoc. $P < 0.05$ was considered as significant difference.

RESULTS: The results showed that animals' weight in both the treatment groups decreased compared to the control group ($P < 0.05$ and $P < 0.01$, respectively). Water and food consumption and the level of TT4 increased when compared to the control group in both the treated groups (both $P < 0.001$). NCV in both the treated groups decreased in comparison with the control group (both $P < 0.001$). Spatial memory did not change significantly, but passive avoidance memory in both the treated groups significantly decreased in comparison to the control group (respectively, with $P < 0.05$ and $P < 0.01$).

CONCLUSION: Based on these finding, in spite of increase in food and water consumption in the treated groups, animal weight did not increase significantly. Impairment in memory and decrease in NCV were also occurred, but TT4 level was increased in animals treated with watermelon extract.

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24.

High milk consumption is associated with carotid atherosclerosis in middle and old-aged Chinese.

He M, Guo Z, Lu Z, Wei S, Wang Z

International Journal of Cardiology Hypertension. 5:100031, 2020 Jun.

[Journal Article]

UI: 33447760

BACKGROUND: Carotid atherosclerosis (CA) can reflect atherosclerotic status of systemic vessels, and is associated with ischemic stroke and cognitive impairment. Milk consumption is growing in China, and data are limited on the association between milk and CA risk.

METHODS: Participants aged ≥ 40 years in China National Stroke Prevention Project were recruited for carotid ultrasound examination, and those with carotid endarterectomy, carotid stenting stroke or coronary heart disease were excluded. Participants were defined as having CA when intima-media thickness (IMT) ≥ 1 mm or plaques were detected by ultrasonography. For milk consumption, though contents were not analyzed, people were divided into high consumption (≥ 200 ml/day and ≥ 5 day/week) and low consumption (occasional or never) groups. Multivariate logistics regression analyses were done in both full and propensity score-matched population to identify the association between milk consumption and CA.

RESULTS: 84880 participants were included in the final analysis. After being adjusted by age, gender, living location, smoking habits, drinking habits, physical activity, obesity, hypertension, diabetes, and dyslipidemia, high milk consumption is associated with CA in both full population (odds ratio [OR] = 1.26, 95% confidence interval [CI] 1.19-1.33) and propensity score-matched population (OR = 1.25, 95%CI 1.17-1.34). High milk consumption is also a risk factor for IMT thickening (OR = 1.15,

95%CI 1.08-1.23), carotid plaque formation (OR = 1.17, 95%CI 1.09-1.25) and carotid stenosis over 50% (OR = 1.52, 95%CI 1.10-2.14) in the propensity score-matched population.

CONCLUSIONS: High milk consumption is related to CA in middle and old-aged Chinese population. More cautious advice on milk intake should be given in this population for prevention of atherosclerosis.

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25.

Cognitive Disorders in Patients with Multiple Sclerosis.

Sehanovic A, Smajlovic D, Tupkovic E, Ibrahimagic OC, Kunic S, Dostovic Z, Zoletic E, Pasic Z

Materia Sociomedica. 32(3):191-195, 2020 Sep.

[Journal Article]

UI: 33424448

INTRODUCTION: Multiple sclerosis (MS) is a chronic, inflammatory, (auto) immune disease of the central nervous system (CNS). Cognitive disorders are found in over 50% of patients.

AIM: The aim of the study was to determine the distribution of cognitive disorders in people with MS.

METHODS: The prospective study included 135 respondents with MS and 50 healthy respondents. The respondents were divided into three groups: the first group consisted of 85 respondents where the disease lasted longer than one year, the second group consisted of 50 respondents with newly diagnosed MS, the third group

consisted of 50 healthy respondents. Clinical assessment instruments were: Extended Disability Score in Multiple Sclerosis Patients, Mini Mental Status, Battery of Tests to Assess Cognitive Functions: Wechsler Intelligence Scale, Revised Beta Test, Raven Colored Progressive Matrices, Wechsler Memory Scale, Rey Audio Verbal Learning Test -Osterriecht's complex character test, verbal fluency test. RESULTS: Cognitive disorders were present in 40-60% of respondents with MS. Visuospatial, visuoconstructive and visuoperceptive functions are worse in the first group. Mnestic functions (learning process, short-term and long-term memory, recollection, verbal-logical memory) were most affected in both groups of respondents, ranging from 30-60%. Poorer cognitive domains are in the first groups of respondents. Immediate working process memory (current learning), memory, attention, short-term and logical memory is worse in the examinees of the first group. At the beginning of the disease, 16% had verbal fluency difficulties, and as the disease progresses, the difficulties become more pronounced. CONCLUSION: Cognitive disorders are heterogeneous, they can be noticed in the early stages of the disease. They refer to impairments of working memory, executive functions and attention, while global intellectual efficiency is later reduced. Copyright © 2020 Aida Sehanovic, Dzevdet Smajlovic, Emir Tupkovic, Omer C. Ibrahimagic, Suljo Kunic, Zikrija Dostovic, Emina Zoletic, Zejneba Pasic.

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26.

Outcomes and interventions in the elderly with and without cognitive impairment: a longitudinal study.
da Silva ACS, Ansai JH, Cezar NOC, Carvalho Vale FA, Dos Santos JG, de Andrade LP

Dementia & Neuropsychologia. 14(4):394-402, 2020 Dec.

[Journal Article]

UI: 33354293

BACKGROUND: Clinical follow-up studies are necessary for a better understanding of the evolution of cognitive impairment as well as the development of better assessment and intervention tools.

OBJECTIVE: To investigate whether older people with preserved cognition (PC), mild cognitive impairment (MCI) and mild Alzheimer's disease (AD) show differences in clinical outcomes and interventions after a 32-month period.

METHODS: One hundred twenty-four community-dwelling older people were included and classified in one of three groups (PC, MCI and mild AD). Information on clinical outcomes (deaths, new diagnoses, falls, need for assistance or changes in routine and hospitalizations) and interventions (increased use of medication, physiotherapeutic intervention, practice of physical exercise, etc.) in the 32-month period were collected by telephone or during a home visit on a single day.

RESULTS: Ninety-five participants (35 with PC, 33 with MCI and 27 with AD) were reevaluated after 32 months. The need for assistance/changes in routine was significantly higher in the AD group, especially with regard to basic activities of daily living. Unlike the other groups, the PC group did not show "other diagnoses" (urinary incontinence, prolapse, change in vision or autoimmune disease). No significant differences were found regarding other variables.

CONCLUSIONS: Older people with and without cognitive impairment exhibited differences in some clinical outcomes after 32 months, such as need for assistance or changes in their routine and new diagnoses of specific diseases. Therefore, the multidimensionality of geriatric patients should be considered when planning assessments and interventions.

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INTRODUCAO: Estudos com acompanhamento clinico sao necessarios para um melhor entendimento da evolucao do quadro cognitivo e desenvolvimento de melhores estrategias de avaliacao e intervencao.OBJETIVO: Investigar se idosos com cognicao preservada (CP), comprometimento cognitivo leve (CCL) e doenca de Alzheimer (DA) leve apresentam diferencas nos desfechos clinicos e intervencoes realizadas ao longo de 32 meses.METODOS: A amostra foi constituída de 124 idosos residentes na comunidade foram incluidos e classificados em um dos tres grupos (CP, CCL, DA leve). Informacoes sobre desfechos clinicos (obitos, novos diagnosticos, quedas, necessidade de assistencia ou mudancas de rotina e hospitalizacoes) e intervencoes realizadas (aumento do uso de medicamentos, intervencao fisioterapeutica, pratica de exercicios fisicos e outras) foram coletadas, no periodo de 32 meses, por telefone ou durante visita domiciliar em um unico dia.RESULTADOS: Apos 32 meses, 95 idosos (35 CP, 33 CCL e 27 DA) foram reavaliados. A necessidade de assistencia/mudancas de rotina foi significativamente maior no Grupo DA, principalmente para as atividades basicas de vida diaria. Diferentemente dos outros grupos, o Grupo CP nao apresentou "outros diagnosticos" (incontinencia urinaria, prolapso, alteracoes visuais e doenca autoimune). Nenhuma outra diferenca significativa foi encontrada quanto as outras variaveis.CONCLUSAO: Os idosos com e sem comprometimento cognitivo apresentaram diferencas em alguns desfechos clinicos apos 32 meses, como necessidade de assistencia/mudancas de rotina e novos diagnosticos de doencas especificas. Assim, a multidimensionalidade do idoso deve ser considerada ao planejar avaliaco es e intervencoes.

Language: Portuguese

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27.

Normative data for middle-aged Brazilians in the Mattis Dementia Rating Scale.

Carvalho GA, Caramelli P

Dementia & Neuropsychologia. 14(4):350-357, 2020 Dec.

[Journal Article]

UI: 33354287

Despite the advances in the diagnosis of dementia, neuropsychological assessment remains an important tool. The Mattis Dementia Rating Scale (DRS) was designed to evaluate people with suspected dementia and allows for the analysis of different cognitive domains. Considering the numerous cases of early-onset dementia, specific reference standards aimed at the middle-aged population are necessary.

OBJECTIVE: To provide normative data for the middle-aged Brazilian population in DRS and to investigate the influence of education level, age, sex, and intelligence quotient (IQ) on the results.

METHODS: Overall, 120 healthcare professionals and caregivers from a hospital, who were healthy, aged between 45 and 64 years, and had at least four years of formal education, were included in the study. They were equally divided into six groups. In each age group (45-54 and 55-64 years), there were three educational levels: 4-7, 8-11, and 12 or more (12+) years of formal education. The results are presented as mean values, standard deviations, and percentiles. Comparisons between groups were carried out for age, sex, and education level. Age, years of formal education, and IQ were also analyzed as continuous variables by Spearman's correlation.

RESULTS: Concerning education level, the comparison between groups showed differences in the results for the total scale and subscales, except for the Construction subscale. No differences were found for age and sex. Correlations observed for years of formal education and IQ were similar. No correlation was found for age.

CONCLUSIONS: The present study contributes to the evaluation of dementia concerning people younger than 65 years of age and reinforces the importance of education in the interpretation of the scores.

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Other Abstract

Publisher

Apesar dos avancos no diagnostico de demencia, a avaliacao neuropsicologica continua sendo uma importante ferramenta. A Escala de Avaliacao de Demencia de Mattis (Mattis Dementia Rating Scale - DRS) foi projetada para avaliar pessoas com suspeita de demencia e permite a analise de diferentes dominios cognitivos. Dados os numerosos casos de demencia com inicio precoce, sao necessarias normas especificas para a populacao de meia-idade. **OBJETIVO:** Fornecer dados normativos para a populacao brasileira de meia-idade na escala Mattis e investigar a influencia da escolaridade, idade, sexo e quociente de inteligencia (QI) nos resultados. **METODOS:** 120 funcionarios e cuidadores saudaveis de um hospital, com idade entre 45 e 64 anos e com pelo menos quatro anos de escolaridade foram incluidos no estudo, os quais foram divididos igualmente em seis grupos. Havia tres

niveis educacionais para cada faixa etaria (45-54 e 55-64 anos): 4 a 7 anos de estudo (4-7), 8 a 11 (8-11) e 12 anos ou mais (12+). Os resultados sao apresentados como valores medios, desvios padrao e percentis. Foram realizadas comparacoes entre os grupos de acordo com idade, sexo e escolaridade. Idade, anos de estudo e QI foram tambem analisados como variaveis continuas atraves da correlacao de Spearman.RESULTADOS: Para a educacao, a comparacao entre os grupos mostrou diferencas nos resultados da escala total e subescalas, exceto na subescala Construcao. Nao foram identificadas diferencas para idade e sexo. As correlacoes observadas para anos de estudo e QI foram semelhantes. Nenhuma correlacao foi encontrada para idade.CONCLUSAO: O presente estudo contribui para a avaliacao da demencia em individuos com menos de 65 anos e reforca a importancia de considerar o nivel educacional na interpretacao dos escores.

Language: Portuguese

Year of Publication

2020

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28.

Environmental factors between normal and superagers in an Argentine cohort.

Calandri IL, Crivelli L, Martin ME, Egido N, Guimet NM, Allegri RF

Dementia & Neuropsychologia. 14(4):345-349, 2020 Dec.

[Journal Article]

UI: 33354286

Normal aging usually brings age-related cognitive decline. However, there is a group of aged individuals who have exceptional memory performance: the superagers.

OBJECTIVE: Our aim was to identify the environmental factors that could influence exceptional memory performance in a cohort of Argentine individuals.

METHODS: Forty healthy volunteers >80 years of age were classified into two groups, superagers (SA, n=20) and normal agers (NA, n=20), according to the Northwestern SuperAging Program criteria. Participants were neuropsychologically tested and evaluated on environmental aspects: working status, education, bilingualism, cognitive reserve, physical activity, social networking, clinical comorbidities, and longevity of parents and siblings.

RESULTS: Both groups were highly educated (NA=16.3+/-3 years; SA 15.85+/-2.6; p=0.6), 11.8% of the sample was still working without differences between groups.

There were no differences in cognitive reserve inventory (p=0.7), physical activity engagement (p=0.423), or social network index (p=0.73). As for longevity, 44% of the siblings lived longer than 80 years of age (p=0.432) and maternal longevity was linked to SA (NA=46.7%; SA=80%; p=0.045).

CONCLUSIONS: This study is a pilot approximation to the superaging population in Argentina. Our results suggest that environmental factors related to successful aging do not differentiate superaging. SA may depend on variables yet to be identified, probably of a genetic/metabolic order.

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1

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Other Abstract

Publisher

O envelhecimento normal geralmente traz declínio cognitivo relacionado a idade. No entanto, existe um grupo de idosos com desempenho excepcional de memória: os superidosos. OBJETIVO: Nosso objetivo foi identificar os fatores ambientais que podem influenciar o desempenho excepcional da memória em uma coorte de indivíduos argentinos. METODOS: Quarenta voluntários saudáveis com idade >80 anos foram classificados em dois grupos: superidosos (SI, n=20) e idosos normais (IN, n=20), de acordo com os critérios do Programa de Superidosos da Universidade Northwestern. Os participantes foram testados neuropsicologicamente e avaliados nos aspectos ambientais: situação de trabalho, escolaridade, bilinguismo, reserva cognitiva, atividade física e rede social, comorbidades clínicas e longevidade de pais e irmãos. RESULTADOS: Ambos os grupos tinham alto nível de escolaridade (IN=16,3±3 anos; SI=15,85±2,6; p=0,6), 11,8% da amostra ainda trabalhava sem diferença entre os grupos. Não houve diferenças no inventário de reserva cognitiva (p=0,7), prática de atividade física (p=0,423) ou índice de rede social (p=0,73). Quanto a longevidade, 44% dos irmãos viviam mais de 80 anos (p=0,432) e a longevidade materna estava associada a SI (IN=46,7%; SI=80%; p=0,045). CONCLUSÕES: Este estudo é uma aproximação piloto ao super envelhecimento da população na Argentina. Nossos resultados sugerem que os fatores ambientais relacionados ao envelhecimento bem-sucedido não diferenciam os superidosos. Ser superidoso pode depender de variáveis ainda não identificadas, provavelmente de ordem genética/metabólica.

Language: Portuguese

Year of Publication

2020

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The comorbidity conditions and polypharmacy in elderly patients with mental illness in a middle income country: a cross-sectional study.

de Lima JD, Teixeira IA, Silva FO, Deslandes AC

IBRO Reports. 9:96-101, 2020 Dec.

[Journal Article]

UI: 33336105

BACKGROUND: Mental disorders increase the risk factor for developing physical comorbidity conditions, such as cardiometabolic diseases. There is a high prevalence of multimorbidity and polypharmacy in the elderly population which hampers clinical response. Studies have shown that this positive correlation between the aging process and enhancement of physical comorbidities is especially high among older adults who live in low or middle income countries.

OBJECTIVE: To investigate the association between physical disease comorbidities and polypharmacy in older adults with a clinical diagnosis of Alzheimer's disease (AD), mild cognitive impairment (MCI) or major depressive disorder (MDD), living in a middle income country.

METHODS: Cross-sectional study of community-dwelling elderly individuals who are cognitively healthy and those with AD, MCI, or MDD. The severity scale of the Charlson Comorbidity Index (CCI) was calculated to classify the severity of comorbidity condition. Logistic regression model (unadjusted and adjusted for age) were used to calculate odds ratios (OR) and 95 % confidence intervals (CI) for cardiometabolic comorbidity (hypertension, diabetes, dyslipidemia and overweight), and polypharmacy.

RESULTS: Although there was not an increased risk of hypertension, diabetes, and obesity among the groups, elderly people with mental disorders presented higher odds for polypharmacy condition. Polypharmacy was significantly higher for all groups in comparison with cognitively healthy participants: AD (OR 22.00, 95 % CI 6.11-79.11), MDD (OR 14.73, 95 % CI 3.69-58.75) and MCI (OR 10.31, 95 % CI 2.44-43.59). Elderly patients with AD presented more severe comorbidities and higher risks for dyslipidemia.

CONCLUSION: Elderly patients with depression, dementia and mild cognitive impairment have considerably higher odds for polypharmacy. People with dementia also have greater comorbidity severity than those who are cognitively healthy. In middle income countries, there is an urgent need to focus on promoting age-appropriate health approaches for the elderly with mental illness to prevent the development of aggravated cardiometabolic conditions and polypharmacy.

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Version ID

1

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Year of Publication

2020

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30.

Resting State Brain Network Function in Elderly: The Formation of Social Ties.

Oh S, Kim A, Kang E, Choi S

International Journal of Psychological Research. 13(2):59-67, 2020 Jul-Dec.

[Journal Article]

UI: 33329878

BACKGROUND: The purpose of this study is to determine the relevance of the relationship between brain network and the social ties management.

METHODS: Participants are based on 52 Korean seniors aged 65 and older who live in Ganghwa-gun, Incheon. We used a closed-triad index (CTI), which is the most basic unit of analysis in the study of group phenomena. This index is a social networking variable that has been shown to have a different implication depending on the subject's condition and role. After two questionnaire surveys were conducted at three years intervals, participants were classified into an increased group and a decreased group according to the change of CTI. Resting-state fMRI analysis were followed to investigate the difference of brain networks between groups.

RESULTS: According to the analysis of the study, the whole participants who had increased in number of CTI has higher local efficiency than the group of the participants who had no effect or decreased in CTI.

CONCLUSIONS: Our study suggests that social relationship, which is substantially related to brain network, is a major factor in successful aging. Lastly, since there is a restriction that the study cannot explain the causal aspect of the brain network and the triad-relationship, there is a need for further investigation.

Version ID

1

Status

PubMed-not-MEDLINE

Authors Full Name

Oh, Seolah, Kim, Aran, Kang, Eunji, Choi, Sungwon

Other Abstract

Publisher

ANTECEDENTES: El proposito de este estudio fue determinar la relevancia de la relacion entre la red cerebral y el manejo de los lazos sociales. METODO: los participantes son 52 adultos mayores coreanos de 65 anos o mas que viven en Ganghwa-gun, Incheon. Utilizamos un indice de triada cerrada (CTI), que es la unidad de analisis mas basica en el estudio de los fenomenos grupales. Este indice es una variable de red social que ha demostrado tener una implicacion diferente dependiendo de la condicion y el rol del sujeto. Despues de realizar dos encuestas por cuestionario a intervalos de tres anos, los participantes se clasificaron en un grupo aumentado y un grupo disminuido de acuerdo con el cambio de CTI. Se siguio el analisis de fMRI en estado de reposo para investigar la diferencia de las redes cerebrales entre los grupos. RESULTADOS: Segun el analisis del estudio, todos los participantes que habian aumentado en numero de CTI tienen una mayor eficiencia local que el grupo de participantes que no tuvieron ningun efecto o disminuyeron en CTI. CONCLUSIONES: Nuestro estudio sugiere que la relacion social que esta

sustancialmente relacionada con la red cerebral es un factor importante en el envejecimiento exitoso. Por ultimo, dado que existe una restriccion de que el estudio no puede explicar el aspecto causal de la red cerebral y la relacion triada, existe la necesidad de una mayor investigacion.

Language: Spanish

Year of Publication

2020

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31.

Evaluation of a Novel Technology-Based Program Designed to Assess and Train Everyday Skills in Older Adults.

Czaja SJ, Kallestrup P, Harvey PD

Innovation in Aging. 4(6):igaa052, 2020.

[Journal Article]

UI: 33324761

BACKGROUND AND OBJECTIVES: Performance of everyday activities is often challenging for older adults. We evaluated a novel computer-based functional skills assessment and training (CFSAT) program, which includes simulations of everyday tasks (e.g., money and medication management).

RESEARCH DESIGN AND METHODS: The sample included noncognitively impaired (NC) older adults (n = 51) and cognitively impaired (CI) older adults (n = 43), who ranged in age from 60 to 86 years (M = 73.12; SD = 6.06), were primarily female (90%), and ethnically diverse (23% Hispanic, 51% African American).

Participants (stratified by cognitive status) were randomized to 1 of the 2 conditions training alone (CFSAT) or CFSAT with computerized cognitive training and trained up to 24 training sessions. Task performance, using measures of completion time and efficiency (accuracy/completion time), was evaluated at baseline, the final training session, and immediately posttraining with an alternate form of the CFSAT assessment.

RESULTS: Both NC and CI participants demonstrated significant performance improvements across all tasks following training (all ps < .001). The CI participants demonstrated reduced training gains compared to the NC participants (all ps < .001). Training gains did not vary as a function of training conditions.

DISCUSSION AND IMPLICATIONS: The findings suggest that CFSAT is an efficacious program for assessing and training everyday task performance. CFSAT can ultimately be used as an intervention strategy to enhance functional independence for aging adults with and without cognitive impairments.

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Year of Publication
2020

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32.

Montreal Cognitive Assessment as Screening Measure for Mild and Major Neurocognitive Disorder in a Chilean Population.

Bello-Lepe S, Alonso-Sanchez MF, Ortega A, Gaete M, Veliz M, Lira J, Perez Salas CP

Dementia and Geriatric Cognitive Disorders Extra. 10(3):105-114, 2020 Sep-Dec.

[Journal Article]

UI: 33250917

BACKGROUND: The Montreal Cognitive Assessment (MoCA) is a sensitive screening instrument for mild neurocognitive disorder (mild NCD). However, cut-off scores and accuracy indices should be established using representative samples of the population. In this context, the aim of this study was to update the normative values, and diagnostic efficiency statistics of the MoCA to detect mild NCD in the Chilean population.

METHODS: This study included 226 participants from the north, center, and south of the country, classified into 3 groups: healthy elderly (HE; n = 113), mild NCD (n = 65), and major neurocognitive disorder (major NCD; n = 48).

RESULTS: The optimal cut-off score to discriminate mild NCD from HE participants was 20 points with a sensitivity of 82.8% and a specificity of 84.1%. The observed balance between sensitivity and specificity shows a good test performance either to confirm or discard a diagnosis. The cut-off between mild NCD and major NCD from HE participants was 19 points with 85.6% of sensitivity and 90.3% of specificity.

CONCLUSION: Overall diagnostic accuracy can be considered as outstanding (AUC ≥ 0.904) when discriminating HE from both mild NCD and major NCD. These results showed that the MoCA is a suitable tool to identify mild NCD and major NCD.

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Year of Publication
2020

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33.

The protective effect of PK11195 on D-galactose-induced amnestic mild cognitive impairment in rats.

Ye C, Lin L, Zhang P, Chen Y, Huang J, Lin X
Annals of Translational Medicine. 8(18):1190, 2020 Sep.
[Journal Article]
UI: 33241039

BACKGROUND: This study aimed to investigate the preventive effect of translocator protein 18kDa (TSPO) ligand PK11195 on amnestic mild cognitive impairment (aMCI), as well as its influence on astrocytes, in order to identify effective ways to prevent aMCI.

METHODS: Male SD rats were randomly divided into control group (n=10), aMCI group (n=10), PK11195 group (n=10), PK11195 + D-gal group (n=10). The preventive effect of PK11195 on aMCI in rats was evaluated. The cognitive function of rats in four different treatment groups was determined using the Morris water maze (MWM), as well as whole-brain pathology and immunofluorescence of rat brain tissue.

RESULTS: The results of the MWM behavioral test showed that rats pre-treated with PK11195 had improved escape latency and a higher number of platform crossings compared with the aMCI model rats. PK11195 was also shown to prevent the D-galactose (D-gal)-induced senescence of pyramidal cells in the hippocampal CA1 region and to inhibit the apoptosis of astrocytes. At the same time, compared with the aMCI model rats, the TSPO in the brain tissue of rats pretreated with PK11195 had a lower distribution density.

CONCLUSIONS: Our results prove that PK11195 can effectively prevent D-gal-induced decline of learning and memory function as well as inhibit abnormal changes of related cells.

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1

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Year of Publication

2020

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34.

The association of smart mobile phone usage with cognitive function impairment in Saudi adult population.

Al-Khlaiwi TM, Habib SS, Meo SA, Alqhtani MS, Ogailan AA

Pakistan Journal of Medical Sciences. 36(7):1628-1633, 2020 Nov-Dec.

[Journal Article]

UI: 33235587

BACKGROUND & OBJECTIVES: Excessive use of mobile phones has raised a great concern about adverse health effects on human health. The present study's aim was to investigate the association of the usage of smartphones with cognitive function impairment in the Saudi adult population.

METHODS: The present cross-sectional study was conducted in the Department of Physiology, College of Medicine, King Saud University, Riyadh, Saudi Arabia during September 2019 to January 2020. A total of 251 Saudi adults who were using mobile phones were recruited, and knowledge, attitude and practices were assessed by interview using a predesigned proforma. The Montreal Cognitive Assessment

(MOCA) tool was employed to assess the cognitive functions, comparison was made between daily mobile phone usage group and their correlated Montreal Cognitive Score (MOCA).

RESULTS: The mean age for 251 Saudi adult participants was 32.43 +/- 12.8 years. More than 80% of the participants used their mobile phone for more than two hours daily. About 61% of the participants were not aware of the side effect of the radiation generated from mobile phone. The participants showed a decrease in MOCA score with increased daily mobile phone usage (MOCA=26.8 for <1 hour daily usage, 26.1 for 1-2 hours, and 24.6 for >2 hours with $P < 0.05$). In addition, participants showed decreased MOCA score by keeping their mobile phone near their pillow while sleeping; MOCA=24.35 for near pillow groups and >25.5 for the groups that placed their mobile phone away from the pillow.

CONCLUSIONS: Excessive use of mobile phones can cause cognitive function impairment. Strict policies must be implemented to control the use of smartphones in order to minimize the effects on mobile phone radiation on cognition. The media has to be on the forefront in educating the public about the proper usage of mobile phones.

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1

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Year of Publication

2020

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35.

Effect of Pentoxifylline on Apoptotic-Related Gene Expression Profile, Learning and Memory Impairment Induced by Systemic Lipopolysaccharide Administration in the Rat Hippocampus.

Akbari Z, Reisi P, Torkaman-Boutorabi A, Farahmandfar M

International Journal of Preventive Medicine. 11:151, 2020.

[Journal Article]

UI: 33209221

BACKGROUND: Inflammation is one of the effective factors, in the development of

functional disorders of the nervous system. Pentoxifylline (PTX) has an inhibitory effect on inflammatory factors. Therefore the aim of this study was to evaluate the effect of PTX on learning, memory and expression of genes, involved in neuronal survival in the rat hippocampus, following systemic lipopolysaccharide (LPS) injection.

METHODS: Male rats were randomly divided into 5 groups of control, LPS and LPS + PTX, receiving doses of 10, 25 and 50 mg/kg of PTX, respectively. In LPS groups, LPS was injected (5 mg/kg; intraperitoneal), and after one week, rats received intraperitoneal PTX for 14 days, in the treatment groups. Learning and memory were evaluated by object location task (OLT) and novel object recognition (NOR). Then, the hippocampus was dissected in order to measure the expression of the associated genes.

RESULTS: The results showed that peripheral LPS injection caused significant damage ($P < 0.01$) to learning and memory with respect to controls, but PTX with doses of 10 and 50 mg/kg prevented these impairments. Results from reverse transcription polymerase chain reaction (RT-PCR) showed that LPS significantly increased the expression of Bax and TNF- α with respect to controls. PTX in the LPS + PTX group significantly increased the expression of Bcl-2, BAD and Caspase-3.

CONCLUSIONS: Other than the increased Bcl-2 expression, PTX had no significant effect on the expression of other genes, therefore further studies are needed to find out how PTX improves the learning and memory impairments, following the peripheral inflammation.

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Version ID

1

Status

PubMed-not-MEDLINE

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Year of Publication

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Age- and Sex-Specific Prevalence and Modifiable Risk Factors of Mild Cognitive Impairment Among Older Adults in China: A Population-Based Observational Study. Fu J, Liu Q, Du Y, Zhu Y, Sun C, Lin H, Jin M, Ma F, Li W, Liu H, Zhang X, Chen Y, Sun Z, Wang G, Huang G

Frontiers in aging neuroscience. 12:578742, 2020.

[Journal Article]

UI: 33192471

BACKGROUND: Minimal data are available on the prevalence of mild cognitive impairment (MCI) in older Chinese adults. Moreover, the current information on MCI shows important geographical variations.

OBJECTIVE: We aimed to assess the prevalence and risk factors for MCI by age and sex among older adults in a North Chinese population.

METHODS: In this population-based cross-sectional study, we enrolled a random sample of 4,943 adults aged ≥ 60 years between March 2018 and June 2019 in Tianjin, China. Of these, 312 individuals were excluded due to a lack of data (e.g., fasting blood test). As a result, 4,631 subjects were assessed. Individuals with MCI were identified using neuropsychological assessments, including the Mini-Mental State Examination and Activities of Daily Living scale, based on a modified version of the Petersen's criteria.

RESULTS: The mean (SD) age of the 4,631 participants was 67.6 (4.89) years, and 2,579 (55.7%) were female. The overall age- and sex-standardized prevalence of MCI in our study population was 10.7%. There were significant associations of MCI with age [65-69 vs. 60-64 years, OR = 0.74; 95% confidence interval (CI): 0.58, 0.96], physical activity (≥ 23.0 vs. <23.0 MET-hours/week, OR = 0.79; 95% CI: 0.64, 0.96), body mass index (BMI) (OR = 0.92; 95% CI: 0.89, 0.95), grip strength (OR = 0.50; 95% CI: 0.38, 0.67), hypertension (yes vs. no, OR = 1.44; 95% CI: 1.18, 1.77), higher levels of sleepiness (OR = 1.80; 95% CI: 1.36, 2.37), and longer sleep duration (OR = 1.40; 95% CI: 1.14, 1.72). The inverse association between BMI and MCI was stronger in older age groups (P for heterogeneity = 0.003). Moreover, the magnitude of association between triglycerides and MCI was different between the sexes (P for heterogeneity = 0.029).

CONCLUSION: The age- and sex-standardized prevalence of MCI was 10.7% in the study sample. Physical activity, BMI, grip strength, sleepiness, sleep duration, and hypertension were associated with the prevalence of MCI. Additionally, triglycerides and BMI might be differently associated with the presence of MCI for different sexes and age stages, respectively.

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37.

Montreal Cognitive Assessment - Single Cutoff Achieves Screening Purpose.
Yeung PY, Wong LLL, Chan CC, Yung CY, Leung LMJ, Tam YY, Tang LN, Li HS,
Lau ML

Neuropsychiatric Disease & Treatment. 16:2681-2687, 2020.

[Journal Article]

UI: 33192067

BACKGROUND AND PURPOSE: The study evaluated the performance between norm-derived age and education adjusted vs single cutoff scores of the Montreal Cognitive Assessment, Hong Kong version (HK-MoCA) in classifying cognitive impairment in Chinese older adults.

METHODS: Total scores of HK-MoCA were collected from 315 subjects (128 with dementia, 122 with mild cognitive impairment (MCI) and 65 normal) attending a public district hospital-based cognition clinic from 2012 to 2017. The HK-MoCA total scores were evaluated using different cutoffs. Norm-derived age and education adjusted cutoff scores were at 16th, 7th, and 2nd percentiles. Comparison was made with the single cutoff scores validated in a local study with 21/22 for MCI and 18/19 for dementia.

RESULTS: Single cutoff score of HK-MoCA differentiated MCI from normal with sensitivity of 0.861 and specificity of 0.723. To detect dementia, its sensitivity was 0.922, and specificity was 0.923. In identifying cognitive impairment, the sensitivity and specificity were 0.932 and 0.723, respectively. However, age and education adjusted cutoff scores achieved high specificities at all levels of cognitive impairment with trade-off of sensitivities. The accuracy of correctly classifying tested subjects into appropriate groups was 85.3% if single cutoff was used though the consistency between norm-derived cutoffs and expert diagnoses were only 59.0%, 54.2%, and 53.9% at 16th, 7th, and 2nd percentiles, respectively. The consistency decreased with older age and lower education level, and majority of misclassifications were false negatives.

CONCLUSION: HK-MoCA is a convenient screening tool to detect cognitive impairment. Administration time is relatively short, and it has incorporated essential cognitive domains. Single cutoff scores with inherent simple education adjustment achieved screening purpose of mild cognitive impairment and dementia in Chinese older adults.

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38.

Prevalence and Predictors of Neurocognitive Impairment in Ethiopian Population Living with HIV.

Salahuddin M, Manzar MD, Hassen HY, Unissa A, Abdul Hameed U, Spence DW, Pandi-Perumal SR

HIV/AIDS Research and Palliative Care. 12:559-572, 2020.

[Journal Article]

UI: 33116918

BACKGROUND: Modern antiretroviral therapy has extended the life expectancies of people living with HIV; however, the prevention and treatment of their associated neurocognitive decline have remained a challenge. Consequently, it is desirable to investigate the prevalence and predictors of neurocognitive impairment to help in targeted screening and disease prevention.

MATERIALS AND METHODS: Two hundred and forty-four people living with HIV were interviewed in a study using a cross-sectional design and the International HIV Dementia Scale (IHDS). Additionally, the sociodemographic, clinical, and psychosocial characteristics of the patients were recorded. Chi-square and binary logistic regression analysis were used to determine the level of significance among the independent risk factors and probable neurocognitive impairment.

RESULTS: The point prevalence of neurocognitive impairment was found to be 39.3%. Participants' characteristics of being older than 40 years (AOR= 2.81 (95% CI; 1.11-7.15)), having a history of recreational drug use (AOR= 13.67 (95% CI; 6.42-29.13)), and being non-compliant with prescribed medications (AOR= 2.99 (95% CI; 1.01-8.87)) were independent risk factors for neurocognitive impairment.

CONCLUSION: The identification of predictors, in the Ethiopian people living with HIV, may help in the targeted screening of vulnerable groups during cART follow-up visits. This may greatly help in strategizing and implementation of the prevention program, more so, because (i) HIV-associated neurocognitive impairment is an asymptomatic condition for considerable durations, and (ii) clinical trials on neurocognitive impairment therapies have been unsuccessful.

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39.

Knowledge, Attitude and Practice of Elderly Outpatients with Type 2 Diabetes Mellitus in National Geriatric Hospital, Vietnam.

Nguyen AT, Pham HQ, Nguyen TX, Nguyen TTH, Nguyen HTT, Nguyen TN, Dinh DTK, Phan HT, Nguyen SH, Tran BX, Latkin CA, Ho RCM, Ho CSH, Pham T, Vu HTT

Diabetes, Metabolic Syndrome and Obesity Targets and Therapy. 13:3909-3917, 2020.

[Journal Article]

UI: 33116737

BACKGROUND: Diabetes-related complications have become increasingly prevalent and complicated to manage, especially among frail older adults with co-morbidities, poor physical function, and impaired cognition. Thus, knowledge, attitude, and practice (KAP) of elderly diabetic patients play an essential role in controlling the disease and its complications.

PATIENTS AND METHODS: A cross-sectional study was conducted on 176 diabetic patients at the outpatient clinics, National Geriatric Hospital, Hanoi, Vietnam from

August to November 2015. Patients' knowledge, attitude, and practice regarding diabetes mellitus were assessed using a structured questionnaire. Exploratory factor analysis and multivariate Tobit regression model were used to identify factors associated with knowledge, attitude, practice about diabetes mellitus.

RESULTS: Of 176 diabetic patients, patients' knowledge was divided into two main groups: "Diabetes definition and management" (mean score = 57.7 +/- 25.6) and "Targets for diabetic control" (mean score = 66.1 +/- 30.4). Patients scored high points of attitudes toward diabetes severity and its complications (mean score = 95.8 +/- 13.0); however, knowledge and practice scores were only moderate (mean score = 68.1 +/- 16.8). There were only 8.5% of people having daily self-blood glucose monitoring. Advanced age, low educational level, rural area, and duration of diabetes less than 5 years were negative associated with knowledge, attitude, and practice regarding diabetes mellitus. This study also shows that frailty and co-morbidities/diabetic complications could impair patients' knowledge.

CONCLUSION: Older patients with diabetes had high levels of perceived severity attitudes on diabetes mellitus. However, the knowledge and practice levels were still inadequate. Pragmatic diabetic health literacy, counseling and education programs, as well as home-based treatments are viable options to improve health outcomes.

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40.

Effects of HDAC inhibitors on spatial memory and memory extinction in SPS-induced PTSD rats.

Mohammadi-Farani A, Pourmotabbed A, Ardeshirizadeh Y
Research in Pharmaceutical Sciences. 15(3):241-248, 2020 Jun.

[Journal Article]

UI: 33088324

BACKGROUND AND PURPOSE: Neurobiological changes in memory processes seem to play a role in the pathophysiology of post-traumatic stress disorder (PTSD). Memory itself is influenced by PTSD, too. Histone deacetylase inhibitors (HDACIs) have shown promising results in the extinction of fear-related memories in animals and hence they seem to be important for the treatment of PTSD. Data are scarce about the effect of HDACIs in spatial memory formation/extinction in PTSD models. The main goal of the present work is to find the effect of sodium butyrate (NaBu), as an HDACI, on spatial memory and spatial memory extinction in rats exposed to single prolonged stress procedure (SPS).

EXPERIMENTAL APPROACH: Different doses of NaBu were administered subcutaneously for 7 days in different groups of rats after SPS procedure. Learning, memory, and extinction of memory were evaluated in the Morris water maze test of spatial memory in 6 consecutive days.

FINDINGS / RESULTS: The results show that NaBu (0.5 mg/kg) alleviates impaired learning and memory in SPS rats. It also facilitates the extinction of newly formed memory in the animals.

CONCLUSION AND IMPLICATIONS: Our data suggest that the administration of HDACIs after a traumatic experience can prevent the aversive effects of SPS on spatial memory. It also reinforces the notion that extinction of spatial memory

involves the same or similar brain circuitry that is involved in the extinction of fear memories in PTSD patients.

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41.

Protective effect of *Mucuna pruriens* against arsenic-induced liver and kidney dysfunction and neurobehavioral alterations in rats.

Concessao P, Bairy LK, Raghavendra AP

Veterinary World. 13(8):1555-1566, 2020 Aug.

[Journal Article]

UI: 33061227

BACKGROUND AND AIM: Intoxication of arsenic in rats is known to result in neurological effects as well as liver and kidney dysfunction. *Mucuna pruriens* has been identified for its medicinal properties. The aim of the study was to investigate the protective effect of aqueous seed extract of *M. pruriens* on sodium arsenite-induced memory impairment, liver, and kidney functions in rats.

MATERIALS AND METHODS: The experiment was divided into short-term treatment (45 days) and long-term treatment (90 days), with each group divided into nine sub-groups consisting of six animals each. Sub-groups 1 and 2 served as normal, and N-acetylcysteine (NAC) controls, respectively. Sub-groups 3-9 received sodium arsenite in drinking water (50 mg/L). In addition, sub-group 4 received NAC (210 mg/kg b.wt) orally once daily, sub-groups 5-7 received aqueous seed extract of *M. pruriens* (350 mg/kg b.wt, 530 mg/kg b.wt, and 700 mg/kg b.wt) orally once daily and sub-groups 8 and 9 received a combination of NAC and aqueous seed extract of *M. pruriens* (350 mg/kg b.wt and 530 mg/kg b.wt) orally once daily. Following the treatment, the blood was drawn retro-orbitally to assess the liver (serum alanine transaminase [ALT], serum aspartate transaminase, and serum alkaline phosphatase) and kidney (serum urea and serum creatinine) functions. Learning and memory were assessed by passive avoidance test. Animals were sacrificed by an

overdose of ketamine, and their Nissl stained hippocampal sections were analyzed for alterations in neural cell numbers in CA1 and CA3 regions.

RESULTS: In the short-term treatment, groups administered with *M. pruriens* 530 mg/kg b.wt alone and combination of NAC + *M. pruriens* 350 mg/kg b.wt exhibited a significant improvement in memory retention, less severe neurodegeneration, and decrease in serum ALT levels. In long-term treatment, groups administered with *M. pruriens* 700 mg/kg b.wt alone and combination of NAC+*M. pruriens* 350 mg/kg b.wt, respectively, showed better memory retention, decreased neural deficits, and reduced levels of kidney and liver enzymes.

CONCLUSION: The seed extract of *M. pruriens* showed significant enhancement in memory and learning. The number of surviving neurons in the CA1 and CA3 regions also increased on treatment with *M. pruriens*. Serum ALT, serum urea, and serum creatinine levels showed significant improvement on long-term treatment with *M. pruriens*.

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42.

The Role of Nutrition and Inflammation on Cognition in a High-Risk Group for Alzheimer's Disease.

Jackson JM, Bay AA, Barter JD, Ni L, Caudle WM, Serra MC, Wharton W, Hackney ME

JAD Reports. 4(1):345-352, 2020 Aug 28.

[Journal Article]

UI: 33024941

BACKGROUND: Alzheimer's disease (AD) is a prevalent neurodegenerative disease. Treatments are necessary to target people at high risk for AD. Inflammation, particularly tumor necrosis factor alpha (TNFalpha), appears to be an important marker associated with the development of AD pathophysiology. Consuming a high-fat diet induces tissue expression of TNFalpha.

OBJECTIVE: This study investigates the relationship between nutrition, circulating

inflammation, and cognition in African American women (age: M = 59.5 (+/-8.20) [42-73] years) at risk for developing AD.

METHODS: Participants were split into high-fat and low-fat groups based on total dietary fat consumption self-reported on the Lower Mississippi Delta Nutrition Intervention Research Initiative Food Frequency Questionnaire (Delta NIRI FFQ).

RESULTS: A high-fat diet was associated with increased blood serum TNFalpha (p = 0.02) compared to the low-fat diet. In addition, global cognition scores were 9.0% better in those who consumed a higher fat diet (p = 0.04). No significant differences across groups were noted for executive function, dual-tasking, and visuospatial performance.

CONCLUSION: These results indicate that there may be multiple biological pathways involved in AD development, suggesting the need for more holistic approaches to mitigate AD-development risk.

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43.

Normative Data for the Symbol Digit Modalities Test in Older White Australians and Americans, African-Americans, and Hispanic/Latinos.

Ryan J, Woods RL, Britt CJ, Murray AM, Shah RC, Reid CM, Wolfe R, Nelson MR, Orchard SG, Lockery JE, Trevaks RE, Storey E, behalf of the ASPREE Investigator Group on

JAD Reports. 4(1):313-323, 2020 Aug 04.

[Journal Article]

UI: 33024939

BACKGROUND: Processing speed, which can be assessed using the Symbol Digit Modalities Test (SDMT), is central to many brain functions. Processing speed declines with advanced age but substantial impairments are indicative of brain injury or disease.

OBJECTIVE: The purpose of this study was to provide SDMT normative data for older community-dwelling individuals in the U.S. and Australia.

METHODS: The ASPREE trial recruited 19,114 relatively healthy older men and women in Australia and the U.S. from the general community. All participants were without a diagnosis of dementia and with a Modified Mini-Mental State examination score of 78 or more at enrolment. The SDMT was administered at baseline as part of a neuropsychological test battery.

RESULTS: The median age of participants was 74 years (range 65-99), and 56% were women. The median years of education was 12. Ethno-racial differences in SDMT performance were observed and normative data were thus presented separately for 16,289 white Australians, 1,082 white Americans, 891 African-Americans, and 316 Hispanic/Latinos. There were consistent positive associations found between SDMT and education level, and negative associations between SDMT and age. Mean scores for women were consistently higher than men with the exception of Hispanic/Latinos aged ≥ 70 years.

CONCLUSION: This study provides comprehensive SDMT normative data for whites (Australian and U.S.), Hispanic/Latinos, and African-Americans, according to gender, age, and education level. These norms can be used clinically as reference standards to screen for cognitive impairments in older individuals.

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Version ID

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Year of Publication
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44.

Memory training combined with 3D visuospatial stimulus improves cognitive performance in the elderly: pilot study.
Assed MM, Rocca CCA, Garcia YM, Khafif TC, Belizario GO, Toschi-Dias E, Serafim AP
Dementia & Neuropsychologia. 14(3):290-299, 2020 Jul-Sep.
[Journal Article]
UI: 32973982
Studies suggest that the engagement of aged participants in cognitive stimulation programs can reduce expected cognitive decline associated with age.
OBJECTIVE: To evaluate the effects of memory training (MT) associated with three-dimensional multiple object tracking (3D-MOT) NeuroTracker (NT) in the elderly.
METHODS: Forty-four participants (>60 years of age) were recruited and randomly distributed into two groups: experimental (EG; n=22) and comparative (CG; n=22). Both groups performed 12 one-hour MT sessions, twice a week, consisting of specific computerized stimuli associated with teaching of mnemonic strategies; 10 minutes of NT was part only of the EG's sessions. In pre- and post-training periods, both groups were evaluated using a sociodemographic questionnaire, neuropsychological assessment, as well as a specific measure offered by NT.
RESULTS: Both groups benefited from the MT and reported more positive feelings regarding their memory and quality of life. However, the EG obtained better results in tests consistent with the strategies trained and which involved attentional resources, reaction time, visual processing speed, episodic, semantic, subjective and working memory as well as aspects of social cognition.
CONCLUSIONS: This study showed that the combination of MT and 3D-MOT

contributed for a better cognitive performance in the EG. Thus, the results of the present study encourage further research and the development of combined cognitive interventions for the elderly population with and without cognitive deficits.

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Other Abstract

Publisher

Estudos sugerem que o envolvimento de idosos em programas de estimulacao cognitiva pode reduzir o declinio cognitivo esperado associado a idade.OBJETIVO: Avaliar os efeitos de um treinamento de memoria (TM) associado a um estimulo visuoespacial tridimensional (3D-MOT) NeuroTracker (NT) em idosos.METODO: Quarenta e quatro participantes (>60 anos) foram recrutados e distribuidos aleatoriamente em dois grupos: experimental (GE; n=22) e comparativo (GC; n=22). Ambos os grupos realizaram 12 sessoes do TM de uma hora, duas vezes por semana, consistindo em estímulos computadorizados especificos associados ao ensino de estrategias mnemonicas; apenas nas sessoes do GE foram utilizados 10 minutos com o NT. Nos periodos pre- e pos-treinamento, ambos os grupos foram avaliados por meio de questionario sociodemografico, avaliacao neuropsicologica e medidas cognitivas especificas do NT.RESULTADOS: Ambos os grupos se beneficiaram do TM e relataram sentimentos mais positivos em relacao a memoria e a qualidade de vida. No entanto, o GE obteve melhores resultados em testes consistentes com as estrategias treinadas e que envolviam recursos atencionais, tempo de reacao, velocidade de processamento visual, memoria episodica, semantica, subjetiva e de trabalho, alem de aspectos da cognicao

social.CONCLUSOES: Este estudo mostrou que a combinacao do TM e 3D-MOT contribuiu para um melhor desempenho cognitivo no GE. Dessa forma, os resultados do presente estudo incentivam novas pesquisas e o desenvolvimento de intervencoes cognitivas combinadas para a populacao idosa com e sem deficits cognitivos.

Language: Portuguese

Year of Publication

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45.

Dietary interventions and cognition of Alzheimer's disease patients: a systematic review of randomized controlled trial.

Moreira SC, Jansen AK, Silva FM

Dementia & Neuropsychologia. 14(3):258-282, 2020 Jul-Sep.

[Journal Article]

UI: 32973980

It is estimated that by 2030 there will be 82 million people in the world with dementia.

OBJECTIVE: To evaluate the effect of dietary interventions on the cognitive performance of individuals with Alzheimer's disease (AD).

METHODS: A systematic review of randomized controlled trials (RCT) was conducted in the Scopus, PubMed, and Cochrane databases. Thirty-two RCT were included.

RESULTS: Omega-3 fatty acid showed positive effects at different doses. Fortasyn Connect seemed to be effective in the early stages of the disease. Probiotic, Ginseng, Inositol and specialized nutritional formulas seemed to have a positive effect on cognition. Most of the primary studies presented poor methodological quality, included patients with mild AD, small samples, and did not obtain significative results for all the cognitive outcomes.

CONCLUSIONS: The effect of most dietary interventions on cognition in AD patients remains inconclusive, however, several nutrients, isolated or not, show potential to improve cognitive function in AD, especially in its early stages.

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Other Abstract

Publisher

Estima-se que ate 2030 haverá 82 milhões de pessoas no mundo com demência.OBJETIVO: Avaliar o efeito de intervenções dietéticas no desempenho cognitivo de indivíduos com DA.METODOS: Foi realizada uma revisão sistemática de ensaios clínicos randomizados (ECR) nas bases de dados Scopus, PubMed e Cochrane. Trinta e dois ECRs foram incluídos.RESULTADOS: Ácidos graxos ômega-3 apresentaram efeitos positivos em diferentes doses. O Fortasyn Connect mostrou-se efetivo em estágios iniciais. Probióticos, Ginseng, Inositol e fórmulas nutricionais especializadas também demonstraram efeito positivo sobre a cognição. A maioria dos estudos primários apresentou baixa qualidade metodológica, incluiu pacientes com DA leve e amostras pequenas e não obteve resultados significativos para todos os desfechos cognitivos.CONCLUSÕES: O efeito da maioria das intervenções dietéticas em pacientes com DA permanece inconclusivo; entretanto, vários nutrientes, isolados ou não, apresentam potencial para melhorar a função cognitiva na DA, principalmente em seu estágio inicial.

Language: Portuguese

Year of Publication

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46.

Usnic acid improves memory impairment after cerebral ischemia/reperfusion injuries by anti-neuroinflammatory, anti-oxidant, and anti-apoptotic properties.

Erfani S, Valadbeigi T, Aboutaleb N, Karimi N, Moghimi A, Khaksari M
Iranian Journal of Basic Medical Sciences. 23(9):1225-1231, 2020 Sep.

[Journal Article]

UI: 32963745

OBJECTIVES: Cerebral ischemia/reperfusion causes complex pathological mechanisms that lead to brain tissue damage. Usnic acid is a lichen secondary metabolite that has many different biological properties including anti-inflammatory and anti-oxidant activities. Therefore, the objective of the current study was to investigate the neuroprotective effects of usnic acid on apoptotic cell death, neuroinflammation, anti-oxidant enzyme activities, and oxidative stress levels after transient cerebral ischemia/reperfusion.

MATERIALS AND METHODS: Forty-two male Wistar rats were randomly assigned to three groups (sham, ischemia/reperfusion, and ischemia/reperfusion+usnic acid). Ischemia was induced by 20 min occlusion of common carotid arteries. Injection of usnic acid (25 mg/kg, intraperitoneally) and saline was done at the beginning of reperfusion time. Morris water maze was applied to assess spatial memory. The protein expression amount was measured using immunohistochemical and immunofluorescence staining. Spectrophotometric assay was performed to determine the levels of anti-oxidant enzymes.

RESULTS: Usnic acid significantly reduced caspase-3, glial fibrillary acidic protein-positive and ionized calcium-binding adaptor molecule 1-positive cells ($P<0.001$) and enhanced spatial memory disorders ($P<0.05$) due to brain ischemia. In addition, treatment with usnic acid improves effects in the antioxidant system following

cerebral ischemia ($P < 0.05$).

CONCLUSION: Our findings indicate that usnic acid has neuroprotective properties, which possibly is applicable as a promising candidate for cerebral injuries caused by ischemia.

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1

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Year of Publication

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47.

Prevalence of potentially inappropriate prescriptions in primary care and correlates with mild cognitive impairment.

Harasani K, Xhafaj D, Begolli A, Olvera-Porcel MC

Pharmacy Practice. 18(3):2017, 2020 Jul-Sep.

[Journal Article]

UI: 32922574

BACKGROUND: Potentially inappropriate prescribing is clearly associated with adverse health consequences among older people. Nevertheless, scarce evidence exists regarding the prevalence of potentially inappropriate prescriptions (PIP) in Albania, a Western Balkans country.

OBJECTIVE: The aim of this study was to assess the prevalence of PIP among older Albanian patients in primary care and to determine the associated sociodemographic and medical factors, including the presence of mild cognitive impairment (MCI).

METHODS: Cross-sectional study in two primary healthcare centers located in two different cities of Albania, a middle-income country in the Western Balkans. The Montreal Cognitive Assessment (MoCA) tool was applied to evaluate MCI. PIPs were assessed by two trained pharmacists using the Beers criteria 2019 update.

Multivariate logistic regression analysis was conducted for possible risk factors

predicting PIP in the study population.

RESULTS: At least one PIP was identified among 40.23 % of the participants (174 older patients) and 10.35 % had more than one PIP. MCI was detected among 79.31 % of the patients. The most commonly represented drug groups in PIP were diuretics (24.71 %), benzodiazepines in the presence of MCI and antidepressants (both 8.62 %). The lack of electrolytes monitoring was the most common reason for PIP.

According to the multivariate analysis, the only statistically significant association observed was between PIP and number of drugs prescribed [three to four drugs (OR 3.34; 95% CI 1.65:6.76), five or more than five drugs (OR 4.08; 95% CI 1.42:11.69)].

CONCLUSIONS: About four out of 10 older Albanian patients experience PIP in primary care. Further studies are needed for a comprehensive estimation of the prevalence and factors associated with PIP, particularly among elderly with mild cognitive impairment.

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48.

Impaired cerebral hemodynamics in late-onset depression: computed tomography angiography, computed tomography perfusion, and magnetic resonance imaging evaluation.

Wang J, Li R, Liu M, Nie Z, Jin L, Lu Z, Li Y

Quantitative Imaging in Medicine & Surgery. 10(9):1763-1774, 2020 Sep.

[Journal Article]

UI: 32879855

BACKGROUND: Late-onset depression (LOD) is often difficult to recognize when there is an absence of a family history of depression and less severe

psychopathology. Increasing evidence has shown that the development and course of LOD symptomatology are associated with cerebrovascular comorbidities and cerebral microvascular lesions. This study was designed to evaluate the associations of LOD with macrovascular and microvascular changes in the brain by using a multi-imaging method, including computed tomography angiography (CTA), CT perfusion (CTP), and magnetic resonance imaging (MRI), to explore the course and pathomechanism of LOD.

METHODS: A total of 116 participants were divided into two groups. Participants older than 60 years who met the diagnostic criteria of depression [International Classification of Diseases (ICD), 10th Edition] were enrolled in the LOD group, and the remainder were age- and sex-matched into the control group. The cognitive/mood status of all participants was evaluated by an experienced neuropsychologist. Global and regional mean cerebral blood flow (CBF) were measured by CT cerebrovascular perfusion imaging; the stenosis of the bilateral intracranial large arteries (internal carotid artery, anterior cerebral artery, middle cerebral artery, posterior cerebral artery, and vertebral artery) was recorded by CTA; regional white matter hyperintensity (WMH) loads were evaluated by fluid-attenuated inversion recovery (FLAIR) MRI; and the Hamilton Depression Scale (HAMD) was used to evaluate depression status.

RESULTS: Our key findings were the following: (I) participants in the LOD group were more prone to intracranial arterial stenosis (81.1% vs. 74.6%), had more severe stenotic arteries compared with controls ($Z=2.024$, $P<0.05$), and significantly more participants with LOD had severe stenosis of the middle cerebral artery (MCA) (9.4% vs. 0%, $P<0.05$); (II) there was a significant difference in hypoperfusion of the frontal and parietal lobes superposed on global cerebral hypoperfusion between the two groups ($P<0.001$); (III) and there was a significant difference in high WMH loads in deep white matter (DWM) between the two groups ($P<0.05$).

CONCLUSIONS: A low global or regional perfusion state, moderate-to-severe stenosis of MCAs, and high WMH loads could be used as imaging biomarkers to indicate diffuse or localized cerebral macrovascular and microvascular pathology in LOD.

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49.

Effects of Levetiracetam Compared to Valproate on Cognitive Functions of Patients with Epilepsy.

El Sabaa RM, Hamdi E, Hamdy NA, Sarhan HA

Neuropsychiatric Disease & Treatment. 16:1945-1953, 2020.

[Journal Article]

UI: 32848400

OBJECTIVE: This study compared the effect of levetiracetam (LEV) as monotherapy to sodium valproate (VPA) as monotherapy on cognitive functions in patients with epilepsy.

METHODS: This was a comparative prospective study on 50 patients with newly diagnosed epilepsy started on antiseizure medications. Patients were selected from the neurology-outpatient clinics at Minia University Hospital, Minia, Egypt. They were divided into two groups: group treated with LEV and group treated with VPA. All patients were subjected to cognitive function assessment using reaction-time tests, trail-making tests, and Wisconsin card-sorting test before treatment and 3 months after treatment.

RESULTS: Both groups of patients showed reduction in seizure frequency. However, patients on LEV showed significant improvement in measured cognitive functions 3 months after starting treatment, while patients in the VPA group showed significant impairment in measured cognitive functions 3 months after starting treatment.

CONCLUSION: Both groups of patients showed reduction in seizure frequency. However, patients on LEV showed significant improvement in measured cognitive functions 3 months after starting treatment, while patients in the VPA group showed significant impairment in measured cognitive functions 3 months after starting treatment.

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Version ID

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Year of Publication

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50.

Prediction of Amyloid Positivity in Mild Cognitive Impairment Using Fully Automated Brain Segmentation Software.

Kang KM, Sohn CH, Byun MS, Lee JH, Yi D, Lee Y, Lee JY, Kim YK, Sohn BK, Yoo RE, Yun TJ, Choi SH, Kim JH, Lee DY, KBASE Research Group
Neuropsychiatric Disease & Treatment. 16:1745-1754, 2020.

[Journal Article]

UI: 32801709

OBJECTIVE: To assess the predictive ability of regional volume information provided by fully automated brain segmentation software for cerebral amyloid positivity in mild cognitive impairment (MCI).

METHODS: This study included 130 subjects with amnesic MCI who participated in the Korean brain aging study of early diagnosis and prediction of Alzheimer's disease, an ongoing prospective cohort. All participants underwent comprehensive clinical assessment as well as 11C-labeled Pittsburgh compound PET/MRI scans. The predictive ability of volumetric results provided by automated brain segmentation software was evaluated using binary logistic regression and receiver operating characteristic curve analysis.

RESULTS: Subjects were divided into two groups: one with Abeta deposition (58 subjects) and one without Abeta deposition (72 subjects). Among the varied volumetric information provided, the hippocampal volume percentage of intracranial volume (%HC/ICV), normative percentiles of hippocampal volume (HCnorm), and gray matter volume were associated with amyloid-beta (Abeta) positivity (all $P < 0.01$). Multivariate analyses revealed that both %HC/ICV and HCnorm were independent significant predictors of Abeta positivity (all $P < 0.001$). In addition, prediction scores derived from %HC/ICV with age and HCnorm showed moderate accuracy in predicting Abeta positivity in MCI subjects (the areas under the curve: 0.739 and 0.723, respectively).

CONCLUSION: Relative hippocampal volume measures provided by automated brain segmentation software can be useful for screening cerebral Abeta positivity in clinical practice for patients with amnesic MCI. The information may also help clinicians interpret structural MRI to predict outcomes and determine early intervention for delaying the progression to Alzheimer's disease dementia.

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51.

Effects of Cyperus rotundus Extract on Spatial Memory Impairment and Neuronal Differentiation in Rat Model of Alzheimer's Disease.

Shakerin Z, Esfandiari E, Razavi S, Alaei H, Ghanadian M, Dashti G

Advanced Biomedical Research. 9:17, 2020.

[Journal Article]

UI: 32775310

BACKGROUND: Alzheimer's disease (AD) is one of the most common neurodegenerative diseases in the older population and characterized by progressive memory and cognitive impairment. Cyperus rotundus, a traditional medicinal herb, has analgesic, sedative, and anti-inflammatory effects and also used to increase memory in Islamic traditional medicine. This study was designed to consider the

effects of *C. rotundus* extract on memory impairment and neurogenesis in the Beta-Amyloid rats' model.

MATERIALS AND METHODS: Forty-two male Wistar rats were randomly divided into six groups ($n = 7$) for the evaluation of baseline training performance in the Morris water maze test. Then, amyloid-beta (Abeta1-42) was injected in animal hippocampal CA1 bilaterally in four groups. The first probe trial was performed 21 days after Abeta injection. Then, 250, 500, and 750 mg/kg of *C. rotundus* extract were administered to three Abeta-injected groups for 1 month; after that, the second probe trial was performed, and rats were sacrificed after 28 days of the second probe trial. The neurogenesis was detected in the hippocampus, by immunohistochemical staining.

RESULTS: This study showed that spatial memory increased in the behavioral test in AD treated group with *C. rotundus* extract, compared with the AD group ($P = 0.02$).

Immunohistochemical staining revealed that neuronal differentiation has been occurred in the hippocampus in the AD-treated group with *C. rotundus* extract compared with the AD group ($P = 0.01$).

CONCLUSIONS: This study showed that *C. rotundus* extract, repaired spatial memory impairment in the Abeta rats, through increased neurogenesis in the hippocampus, which could be related to the flavonoid components in the extract.

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Year of Publication

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52.

Percutaneous Endoscopic Gastrostomy Tube Removal for Patients With Advanced Dementia: Case Series Study.

Hattori Y, Kojima T, Komura H, Ura N, Akishita M

Annals of Geriatric Medicine & Research. 23(1):27-30, 2019 Mar.

[Journal Article]

UI: 32743282

BACKGROUND: Although the notion of percutaneous endoscopic gastrostomy (PEG) tube placement for patients with dementia has been changing, the number of cases of PEG placement remains high as Japan has become a super-aged society. However, there is insufficient research about the clinical course of dementia patients with PEG, especially regarding PEG extubation after regaining full oral intake. This case series aimed to reveal the demographic data of patients who successfully underwent PEG extubation and to identify clinical factors that might help predict eventual resumption.

METHODS: This retrospective case series was identified in a private, community-based long-term care hospital in Sapporo, Japan. Inclusion criteria for the series were: 1) age, ≥ 75 years, 2) diagnosis of any type of dementia, and 3) resumption of full oral intake after removal of PEG tubes.

RESULTS: Eight female patients were identified. Decreased oral intake was triggered by acute medical conditions, such as infectious enteritis. A trial of oral intake was initiated mostly by speech therapists. A majority of the patients ate pureed food. The patients aged ≥ 85 years with advanced dementia could be weaned from PEG tubes.

CONCLUSION: The series indicates that even patients with limited life expectancy could recover swallowing function by receiving appropriate guidance and care.

Constant evaluation for swallowing function even after PEG insertion may be important for PEG extubation.

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Year of Publication

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53.

Effect of Donepezil on Cognitive Impairment, Quality of Life, and Depression in Multiple Sclerosis Patients: A Randomized Clinical Trial.

Shahpour MM, Barekatin M, Tavakoli M, Badihian S, Shaygannejad V
International Journal of Preventive Medicine. 11:69, 2020.

[Journal Article]

UI: 32742613

BACKGROUND: Cognitive impairment is one of the debilitating consequences of multiple sclerosis (MS) with negative effects on daily life, individual and social activities, quality of life (QOL), and depression. No approved medication is introduced so far for affected individuals. We aimed to evaluate the efficacy of donepezil on cognitive performance, QOL, and depression in MS.

METHODS: This is a double-blinded randomized clinical trial conducted on 100 patients with MS during 2018. Patients were assessed prior to intervention abbreviated mental test (AMT), prospective and retrospective mental questionnaire (PRMQ), everyday memory questionnaire (EMQ), digit span test, Beck depression inventory (BDI), and MSQOL questionnaire. Then patients were randomly divided into two groups of treatment (daily regimen of 10 mg donepezil) and placebo for 3 months. Subjects were reassessed using the same instruments at the end of intervention.

RESULTS: Fifty patients remained in each group at the end of study. The mean age in donepezil and placebo groups was 31.9 +/- 5.89 and 30.65 +/- 5.43 years, respectively. EMQ, PRMQ, digit span test, MSQOL, and depression scores improved following donepezil therapy ($P < 0.001$) while no statistically significant difference was found in the placebo group ($P > 0.05$). Comparison of two groups also showed more favorable scores in donepezil group with respect to all assessment tools ($P < 0.001$).

CONCLUSIONS: Donepezil could effectively improve cognitive impairment in MS patients. Also, its positive effect on QOL and depression could result in a smaller number of interventions in this group of patients.

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Phytotherapy for Cachexia: Where Do We Stand?. [Review]

Kuchta K, Cameron S

Frontiers in Pharmacology. 11:917, 2020.

[Journal Article. Review]

UI: 32733236

BACKGROUND: In contrast to Western medicine which currently offers no approved pharmacotherapy options for cachexia, in Japan multi-component extracts of medicinal plants are used with coverage by the national health insurance. This so called "Kampo" medicine is an example of the modern concept of multi-component/multi-target therapy. For the three traditional preparations Hochuekkito (), Juzentaihoto (), and Rikkunshito (), a multitude of clinical research data relating to cachexia has been published. These preparations are also referred to as "Hozai" (). A similar concept is found in Russian herbal medicine, where the term "Adaptogen" was coined for pharmacologically active substances which enhance adaptive stress repose.

METHODS: Scientific literature-including original Japanese articles-was reviewed regarding the effects of these herbal preparations on cachexia. Cachexia is a complex set of symptoms including muscle atrophy with loss of weight, fatigue, and weakness.

RESULTS: In a 1985 study by Kuroda et al., Hochuekkito showed efficacy in involuntary weight loss and fatigue in 63% of 162 patients with genitourinary cancer. For cancer-related fatigue, a significant improvement was reported within 2 weeks by Jeong et al. in 2010. In patients with chronic fatigue syndrome, Hochuekkito showed an overall improvement with 8-12 weeks of therapy in a 1997 study by Kuratsune et al. In a 2005 randomized placebo-controlled trial by Satoh et al. on 13 geriatric Q1 patients in a 16-week treatment protocol, Hochuekkito showed significant improvement of general health, physical functioning and the Profile of Mood States (POMS). In 71 geriatric COPD patients in a 2009 placebo-controlled randomized study, Tatsumi et al. found a significant body weight increase and a CRP, TNF-alpha, IL-6 decrease over 6 months of therapy. For Juzentaihoto in 48 hepatocellular carcinoma patients, Tsuchiya et al. 2008 documented a significantly longer recurrence-free survival (49 vs. 24 months) as compared to the control group ($p=0.023$). For the much simpler Rikkunshito prescription, a 2011 retrospective study by Fujitsuka et al. on 39 Stage III/IV pancreatic cancer patients treated with Gemcitabine ($n=33$) or Gemcitabine/Rikkunshito ($n=6$) showed a significantly prolonged median survival with 224 vs. 378.5 days ($p < 0.05$). In a 2011 open-label clinical study by Utumi et al. on geriatric cachexia in 6 dementia patients, treatment with Rikkunshito for 4 weeks resulted in a significant body weight increase. In all the above studies, the standardized dosage of 3x2.5 g/d extract granules for most Japanese health insurance-covered Kampo extract-preparations was applied.

CONCLUSION: As there is currently no accepted pharmacotherapy option for cachexia available in the West, a transfer of these East Asian gold standard prescriptions into the European market would be desirable. We were further able to demonstrate that the mTOR, interleucin, and melatonin pathways are modified by herbal compounds which thus counteract cachexia. More research in this field is urgently needed in order to provide new, effective treatments for cachexia patients.

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Year of Publication
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55.

Antiretroviral therapy improves neurocognitive impairment in people living with HIV?
A meta-analysis. [Review]

Gao C, Meng J, Xiao X, Wang M, Williams AB, Wang H
International Journal of Nursing Sciences. 7(2):238-247, 2020 Apr 10.
[Journal Article. Review]

UI: 32685622

OBJECTIVES: Although effective antiretroviral therapy (ART) has been used for more than two decades, HIV-associated neurocognitive disorder remains prevalent. Thus, whether ART can improve neurocognitive impairment is controversial. This review aims to explore the effects of ART on cognitive impairment in people living with HIV (PLWH).

METHODS: A systematic literature search was conducted in eight databases (PubMed, Embase, Web of Science, Cochrane Library, CNKI, VIP, China Biology Medicine disc, and WanFang) to identify studies that compare cognitive function between study groups who are administered and not administered ART. We searched for articles published up to April 2019. Article evaluation and data extraction were independently conducted by two reviewers.

RESULTS: Sixteen articles (6,694 participants)-14 cross-sectional studies and 2 cohort studies-were included in this meta-analysis. The cross-sectional studies demonstrated that ART group did not perform better than the non-ART group (OR = 1.16; 95% CI, 1.03-1.30). However, the cohort studies reported a significant improvement in cognitive function at three months (OR = 4.01; 95% CI, 2.35-6.85) and six months (OR = 9.24; 95% CI, 1.71-49.96) after ART initiation compared with the baseline data. No significant cognitive improvement was found in participants younger than 55 years old, but the two cross-sectional studies showed that ART may improve cognitive function in PLWH under 65 years old with poor physical condition and immune status.

CONCLUSIONS: ART could improve cognitive function in PLWH with poor physical condition and immune status, but it does not considerably improve cognition in the entire PLWH population.

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Year of Publication
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56.

Association of retinal nerve fiber abnormalities with serum CNTF and cognitive functions in schizophrenia patients.

Liu Y, Huang L, Tong Y, Chen J, Gao D, Yang F
PeerJ. 8:e9279, 2020.

[Journal Article]

UI: 32676219

BACKGROUND: Recent studies have reported reductions in retinal nerve fiber layers (RNFL) in schizophrenia. Ciliary neurotrophic factor (CNTF) has shown protective effects on both the neurogenesis and retina. This study aimed at investigating retinal abnormalities and establishing their correlation with serum CNTF and cognitive impairments in schizophrenic Chinese patients.

METHODS: In total, 221 patients diagnosed with schizophrenia and 149 healthy controls were enrolled. Serum CNTF and clinical features of patients were investigated. Cognitive functions were evaluated with Repeatable Battery for the Assessment of Neuropsychology Status (RBANS). RNFL thickness and macular thickness (MT) of both eyes were measured with optical coherence tomography (OCT). T-tests and analysis of covariance were used to compare the variables between the patient and control groups, while multiple linear regression analysis was performed to determine the associations of RNFL thickness, CNTF and cognitive impairments.

RESULTS: RNFL was found thinner in patients than in healthy controls (right: 88.18 +/- 25.84 microm vs. 102.13 +/- 14.32 microm, $p = 0.001$; left: 92.84 +/- 13.54 microm vs. 103.71 +/- 11.94 microm, $p < 0.001$). CNTF was lower in the schizophrenia group (1755.45 +/- 375.73 pg/ml vs. 1909.99 +/- 368.08 pg/ml, $p = 0.001$). Decline in RNFL thickness was found correlated with course of illness and serum CNTF in patients (all $p < 0.05$). Similarly, cognitive functions such as immediate memory and visuospatial functions were also found correlated with decline in RNFL thickness.

CONCLUSION: Decline in RNFL thickness was associated with cognitive impairments of schizophrenia and CNTF serum concentration. The possibility of reduction in RNFL thickness as a biomarker for schizophrenia needs to be further examined.

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57.

The Impact of Educational Intervention on the Anxiety of Family Caregivers of the Elderly with Dementia: A Randomized Controlled Trial.

Zarepour A, Hazrati M, Kadivar AA

International Journal of Community Based Nursing & Midwifery. 8(3):234-242, 2020 Jul.

[Journal Article]

UI: 32656275

BACKGROUND: Due to the increasing population of elderly and the consequent increase in the number of chronic diseases such as dementia, the psychological complications such as anxiety in the family caregivers increase. The aim of this study was to determine the effect of educational intervention on the anxiety of family caregivers of the elderly people with dementia.

METHODS: This randomized controlled trial was performed in the elderly Neurology Clinics in Shiraz from May to August 2017. This study was conducted on 70 families with elderly people with dementia who were randomly divided into an intervention (receiving in groups for seven sessions of educational intervention) and a control group (Conventional care). Data collection tool was Spielberger's Anxiety Inventory (40-items, score=20-80). Data were analyzed by SPSS version 21 using ANOVA test with repeated measures and independent t-test.

RESULTS: One and three months after the interventions, the mean scores of anxiety in the intervention group were 70.51 $\pm$ 3.78 and 70.31 $\pm$ 3.43 and in the control groups they were 76.45 $\pm$ 3.45 and 76.22 $\pm$ 5.08 respectively. The results showed significant differences between the two groups regarding anxiety after the intervention ($P<0.001$).

CONCLUSION: Educational programs held to promote and maintain the physical and mental health of caregivers could reduce the anxiety of family caregivers of the elderly with dementia. Trial Registration Number: IRCT2017080915426N4.

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Year of Publication

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58.

Pharmacological treatment of neuropsychiatric symptoms of dementia: a network meta-analysis protocol.

Huang YY, Dou KX, Zhong XL, Shen XN, Chen SD, Li HQ, Chen KL, Cui M, Dong Q, Tan L, Yu JT

Annals of Translational Medicine. 8(12):746, 2020 Jun.

[Journal Article]

UI: 32647671

BACKGROUND: Neuropsychiatric symptoms (NPS) of dementia are a common issue in dementia patients which can lead to poor medical and functional outcomes. Pharmacological interventions are its treatment of choice. However, whether to use pharmacological treatments in this population and which drug should be preferred remain controversial. We therefore aimed to compare and rank pharmacological interventions for NPS according to their efficacy and acceptability profiles by quantifying information from randomized controlled trials (RCTs).

METHODS: We will include all RCTs reported as double-blind and comparing one active drug with another or with placebo that compare cholinesterase inhibitors (ChEIs), N-methyl-D-aspartic acid (NMDA) receptor modulators, antipsychotics, antidepressants, and mood stabilisers. Studies will be retrieved by searching electronic databases, including Cochrane Central Register of Controlled Trials, PubMed, MEDLINE, Clinicaltrial.govs, EMBASE, and with no date or language restrictions. The primary outcomes were efficacy (change in overall symptoms) and acceptability (all-cause discontinuation). The network meta-analysis (NMA) will be conducted in R software within a Bayesian framework. The quality of evidence will be evaluated using the Cochrane risk of bias tool, and the GRADE approach. We will conduct subgroup analyses to assess the robustness of our findings.

RESULTS: The results of this study will be published in a peer-reviewed journal.

CONCLUSIONS: This systematic review will synthesize the available evidence on the comparative efficacy of different pharmacological approaches in the management of overall NPS, agitation, psychosis, apathy and depressive symptoms in dementia patients. The results of the present NMA will influence evidence-based treatment decisions for clinicians.

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59.

New Users of Herbal Medicine Containing Aristolochic Acids and the Risk of Dementia in the Elderly: A Nationwide, Population-Based Study in Taiwan.

Chiou YE, Chien WC, Chung CH, Chang HA, Kao YC, Tsay PK, Tzeng NS
Neuropsychiatric Disease & Treatment. 16:1493-1504, 2020.

[Journal Article]

UI: 32606702

BACKGROUND: Herbal medicine containing aristolochic acids (HMCAA) was used for inflammatory and infectious diseases. This study aimed to investigate the association between the usage of HMCAA and the risk of dementia.

METHODS: A total of 199 new users of HMCAA were enrolled, along with 597 controls without the usage of HMCAA, at a ratio of 1:3 - matched by age, sex, and comorbidity, between 2000 and 2003 - from the National Health Research Institutes Database (NHRID) of Taiwan, which contains two million randomly sampled subjects,

in this cohort study. We used Fine and Gray's survival analysis (competing with mortality) to compare the risk of developing dementia during a 15-year follow-up period (2000-2015).

RESULTS: In general, HMCAA was not significantly associated with dementia (adjusted subdistribution hazard ratio [SHR] = 0.861, 95% confidence interval [CI] = 0.484-1.532, $p = 0.611$) for the HMCAA-cohort, although differential risk was observed among the groups at risk. The patients with usage of HMCAA aged 85 years were associated with a higher risk in dementia (adjusted SHR: 6.243, 95% CI=1.258-21.084, $p = 0.001$), in comparison to those aged 50-54 years. Furthermore, the patients with usage of HMCAA that had cerebrovascular accidents were associated with an increased risk of dementia.

CONCLUSION: The usage of HMCAA was associated with the risk of developing dementia in the patients aged 85 years.

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60.

Validation of the Argentine version of the Montreal Cognitive Assessment Test (MOCA): A screening tool for Mild Cognitive Impairment and Mild Dementia in Elderly.

Serrano CM, Sorbara M, Minond A, Finlay JB, Arizaga RL, Iturry M, Martinez P, Heinemann G, Gagliardi C, Serra A, Magliano FC, Yacovino D, Rojas MMEY, Ruiz AS, Graviotto HG

Dementia & Neuropsychologia. 14(2):145-152, 2020 Apr-Jun.

[Journal Article]

UI: 32595883

The MoCA is a brief useful test to diagnose mild cognitive impairment (MCI) and mild dementia (MD). To date, no Argentine cross-cultural adapted validations of the Spanish version have been reported.

OBJECTIVE: To validate the MoCA in the elderly and study its usefulness in MCI and MD.

METHODS: This study included 399 individuals over 60 years old evaluated in the Cognitive-Behavioral Department (2017-2018). Patients with <3 years of education, sensory disturbances, psychiatric disorders, or moderate-severe dementia were excluded. The control group comprised cognitively normal subjects. Participants were classified according to neuropsychological assessment and clinical standard criteria into Control, MCI or MD groups. A locally adapted MoCA (MOCA-A) was administered to the patients and controls.

RESULTS: Mean educational level was 10.34 years (SD 3.5 years). MoCA-A score differed significantly among groups ($p < 0.0001$). MoCA-A performance correlated with educational level ($r: 0.406$ $p < 0.00001$). Adopting a cut-off score ≥ 25 (YI=0.55), the sensitivity for MCI was 84.8% and for MD 100%, with specificity of 69.7%. When adding a single point to the score in patients with ≤ 12 years of education, the specificity of the test reached 81%.

CONCLUSION: The MoCA-A is an accurate reliable screening test for MCI and MD in Argentina.

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1

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Other Abstract

Publisher

O MoCA e um teste breve e util para diagnosticar comprometimento cognitivo leve (CCL) e demencia leve. Ate o momento, nenhuma validacao argentina com adaptacao transcultural da versao em espanhol havia sido relatada.OBJETIVO:

Validar o MoCA em idosos e estudar sua utilidade no CCL e demencia

leve.METODOS: Este estudo incluiu 399 individuos acima de 60 anos avaliados no departamento cognitivo-comportamental (2017-2018). Foram excluidos pacientes

com menos de 3 anos de escolaridade, com disturbios sensoriais, disturbios psiquiatricos e demencia moderada a grave. O grupo controle foi cognitivamente

normal. Eles foram classificados de acordo com a avaliacao neuropsicologica e os criterios clinicos padrao em Controles, MCI e demencia leve. A versao adaptada do

MoCA (MOCA-A) foi administrado aos pacientes e controles.RESULTADOS: Media de escolaridade: 10,34 anos (DP: 3,5). O escore MoCA-A foi significativamente

diferente entre os grupos ($p < 0,0001$). O MoCA-A correlacionou-se com a escolaridade ($r = 0,406$ $p < 0,00001$). Com uma pontuacao de corte ≥ 25 (IY=0,55), a

sensibilidade para CCL foi de 84,8% e para demencia leve 100%, com especificidade de 69,7%. Adicionando um ponto unico a pontuacao em pacientes com menos de 12 anos de escolaridade, a especificidade do teste atingiu

81%.CONCLUSAO: O MoCA-A e um teste de rastreamento preciso e confiavel para MCI e demencia leve na Argentina.

Language: Portuguese

Year of Publication

2020

Link to the Ovid Full Text or citation:

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61.

Cognitive stimulation program in mild cognitive impairment A randomized controlled trial.

Gomez-Soria I, Peralta-Marrupe P, Plo F

Dementia & Neuropsychologia. 14(2):110-117, 2020 Apr-Jun.

[Journal Article]

UI: 32595879

Non-pharmacological cognitive interventions in mild cognitive impairment have demonstrated promising results in preventing or delaying cognitive impairment and functional disability. Cognitive stimulation seems to improve and maintain cognitive and social activity.

OBJECTIVE: This study aimed to evaluate the impact of a cognitive stimulation program in mild cognitive impairment (MCI) at the cognitive level on activities of daily living (ADLs), and levels of anxiety and depression.

METHODS: A randomized controlled single-blind trial involving 122 non-institutionalized elderly with a MEC-35 score of 24-27 was conducted. The intervention group (n=54) received the intervention (10-week cognitive stimulation program) and was compared with a control group (n=68) that received no intervention. Follow-up assessments were conducted post-test and at 6 months post-test. The primary outcome was cognitive function determined by changes in scores on the Spanish version (MEC-35) of the Mini-Mental State Examination, while the secondary outcomes were measured by the Barthel Index, Lawton and Brody Scale, Goldberg Questionnaire (anxiety sub-scale) and the Yesavage Geriatric Depression Scale (15-item version).

RESULTS: The intervention group showed a significant improvement in cognitive function at both timepoints, post-test and 6-month follow-up. The Barthel Index was higher in the intervention group, but only on the post-test analysis. The intervention did not improve the performance of instrumental ADLs or depression or anxiety levels.

CONCLUSION: The findings showed cognitive improvements in an elderly population with MCI in the short and medium-term and improved basic ADLs in the short term.

Clinicaltrials.gov Identifier: NCT03831061.

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1

Status

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Other Abstract

Publisher

As intervenções cognitivas não farmacológicas no comprometimento cognitivo leve demonstram resultados promissores na prevenção ou retardo no comprometimento cognitivo e na incapacidade funcional. A estimulação cognitiva parece melhorar e manter a atividade cognitiva e social. OBJETIVO: Nosso objetivo foi o de avaliar um programa de estimulação cognitiva sobre o comprometimento cognitivo leve no nível cognitivo, nas atividades da vida diária, níveis de ansiedade e depressão. METODOS: Um estudo randomizado, controlado e cego, foi implementado em 122 idosos não institucionalizados com escore 24-27 na versão em espanhol do Mini Exame do Estado Mental (MEC-35). O grupo de intervenção (n=54) recebeu a intervenção (programa de estimulação cognitiva de 10 semanas) e foi comparado com um grupo de controle (n=68) que não recebeu nenhuma intervenção. Avaliações de acompanhamento foram realizadas pós-teste e 6 meses pós-teste. O desfecho primário foi a função cognitiva determinada pelas alterações nos escores do MEC-35 e os desfechos secundários foram medidos pelo Índice de Barthel, Escala de Lawton e Brody, Escala de Lawton e Brody, Questionário Goldberg (subescala de ansiedade) e a Escala de Depressão Geriátrica de Yesavage (versão de 15 itens). RESULTADOS: O grupo intervenção apresentou uma melhora significativa na função cognitiva nas duas medidas de tempo, pós-teste e 6 meses de acompanhamento. O Índice de Barthel foi encontrado mais alto no grupo de intervenção, mas apenas na análise pós-teste. A intervenção não melhorou o desempenho de atividades instrumentais da vida diária e níveis de depressão ou ansiedade. CONCLUSÃO: Os achados mostram melhorias cognitivas no comprometimento cognitivo leve da população idosa em curto e médio prazo e melhoraram as atividades básicas da vida diária em curto prazo. Identificador do Clinicaltrials.gov: NCT03831061.

Language: Portuguese

Year of Publication

2020

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62.

Action observation combined with gait training to improve gait and cognition in elderly with mild cognitive impairment A randomized controlled trial.

Rojasavastera R, Bovonsunthonchai S, Hiengkaew V, Senanarong V
Dementia & Neuropsychologia. 14(2):118-127, 2020 Apr-Jun.

[Journal Article]

UI: 32595880

Owing to advancement of medical technology and current knowledge, the population has a longer life expectancy, leading to an increase in the proportion of elderly.

OBJECTIVE: The study aimed to investigate the effect of action observation (AO)

combined with gait training on gait and cognition in elderly with mild cognitive impairment (MCI).

METHODS: Thirty-three participants were randomly allocated to action observation with gait training (AOGT), gait training (GT), and control (CT) groups. The AOGT and GT groups received a program of observation and gait training protocol with the same total duration of 65 min for 12 sessions. For the observation, the AOGT group watched a video of normal gait movement, while the GT group watched an abstract picture and the CT group received no training program. All participants were assessed for gait parameters during single- and dual-tasks using an electronic gait mat system and were assessed for cognitive level using the Montreal Cognitive Assessment (MoCA) at baseline, after training and at 1-month follow-up.

RESULTS: The results showed that the AOGT group had significant improvements in gait speeds during single- and dual-tasks, as well as better MoCA score, while the GT group had significant improvement only in gait speed.

CONCLUSION: The adjunct treatment of AO with gait training provides greater benefits for both gait and cognitive performances in elderly with MCI.

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Other Abstract

Publisher

Com o avanço da tecnologia médica e do conhecimento atual, a população tem uma expectativa de vida mais longa, levando a um aumento na proporção de idosos. OBJETIVO: O estudo teve como objetivo investigar o efeito da observação de ação (AO) combinada com o treinamento da marcha na marcha e cognição em idosos com comprometimento cognitivo leve (CCL). METODOS: Trinta e três participantes foram alocados aleatoriamente para observação de ação com grupos de treinamento de marcha (AOGT), treinamento de marcha (GT) e controle (CT). Os grupos AOGT e GT receberam um programa de observação e protocolo de treinamento de marcha com a mesma duração total de 65 minutos por 12 sessões. Na observação, o grupo AOGT assistiu a um vídeo de movimento normal da marcha, enquanto o grupo GT assistiu a uma figura abstrata e o grupo CT não recebeu nenhum programa de treinamento. Todos os participantes foram avaliados quanto aos parâmetros da marcha durante tarefas simples e duplas, utilizando um sistema eletrônico de esteira da marcha e avaliados quanto ao nível cognitivo, utilizando a Avaliação Cognitiva de Montreal (MoCA) na linha de base, após o treinamento e 1 mês de acompanhamento. RESULTADOS: Os resultados mostraram que o grupo AOGT apresentou melhorias significativas nas velocidades da marcha durante

tarefas simples e duplas, além do escore MoCA, enquanto o grupo GT teve melhora significativa apenas na velocidade da marcha. CONCLUSÃO: O tratamento adjunto da AO com o treinamento da marcha proporciona maiores benefícios tanto do desempenho da marcha quanto do desempenho cognitivo em idosos com CCL.

Language: Portuguese

Year of Publication

2020

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63.

Investigation of Aneusomy of Chromosome 21 in the Micronuclei of 13 Patients with Early Onset Alzheimer's Disease Using Fluorescence in Situ Hybridization: A Pilot Study.

Biglari S, Kamali K, Banihashemi S, Faal Sezavari A, Aghajanjpour-Mir SM, Behjati F Reports of Biochemistry & Molecular Biology. 8(4):446-453, 2020 Jan.

[Journal Article]

UI: 32582804

BACKGROUND: Alzheimer's disease is one of the most common neurodegenerative and dementia disorders in people between the ages of 30 and 65. When symptoms appear in this age group, the disease is referred to as early-onset Alzheimer's disease (EOAD). Unfortunately, the symptoms are progressive and no current treatments are effective.

METHODS: In this research, 13 patients, aged 37 to 65 years with symptoms of early-onset Alzheimer's disease, were studied. First, patient lymphocytes were isolated and cultured in RPMI 1640 medium using a special micronucleus (MN) culture method. Next, the lymphocytes were harvested and prepared on slides. The slides were then examined by fluorescent microscopy using a unique FISH protocol specific for MNs. The patients were divided into groups aged 30-39, 40-49, and 50-65.

RESULTS: We found that 19.76% of the MNs from our EOAD patients originated in chromosome 21. Micronuclei originated in chromosome 21 in 21.20 and 16.52% of patients without and with family histories of Alzheimer's, respectively. This difference was not significant. Also, the percentage of micronuclei originating in chromosome 21 was not dependent on the patient age at the time of the study, or symptom onset age or duration.

CONCLUSION: This study shows that the rate of micronuclei with the origin of chromosome 21 is high in these patients. However, the micronucleus increased has no significant relationship with age and duration of disease or family history of it.

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1

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Year of Publication
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64.

Sleep Quality and its various correlates: A community-based study among geriatric population in a community development block of Purba Bardhaman district, West Bengal.

Das S, Roy RN, Das DK, Chakraborty A, Mondal R

Journal of Family Medicine & Primary Care. 9(3):1510-1516, 2020 Mar.

[Journal Article]

UI: 32509641

BACKGROUND: In the elderly population, sleep problems are prevalent and have known to be associated with many factors. There are many adverse consequences of decreased sleep such as heart disease, diabetes, depression, accidents, impaired cognition, and poor quality of life. Correlates of poor sleep quality have not been well explored in Indian research.

OBJECTIVES: The present study aimed to measure prevalence of poor sleep quality among elderly and its association with different factors.

METHODS: A cross-sectional study was conducted during June-November 2018 in a randomly selected block of Purba Bardhaman district. Cluster random sampling was applied to select required sample of 180 elderly people (≥ 60 years) from 30 villages. Study tools used were Pittsburgh Sleep Quality Index (PSQI), 5-Item Geriatric Depression Scale (GDS), Generalized Anxiety Disorder 7-item scale (GAD-7), Global Physical Activity Questionnaire (GPAQ), and a pretested schedule for sociodemographic and other variables. The study had approval from Institutional Ethics Committee. Chi-square tests and multivariable logistic regression were performed using SPSS V16.

RESULTS: Prevalence of poor sleep quality (GPSQI ≥ 5) was 68.89%. Median Global PSQI (GPSQI) score was 7.00 (4.00-11.00). Multivariable logistic regression revealed that marital status, vital events in past one month, anxiety status, and depression were significantly associated with sleep quality. Those who were unmarried/widowed, having vital events in past one month in the family, and severe anxiety and depression were having significantly higher odds of developing poor sleep quality.

CONCLUSION: Poor sleep quality is high among elderly and measures toward the significant correlates are thus emphasized.

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Year of Publication

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65.

Knowledge regarding Alzheimer's Disease among College Students of Kathmandu, Nepal.

Baral K, Dahal M, Pradhan S

International journal of Alzheimer's disease. 2020:6173217, 2020.

[Journal Article]

UI: 32494366

INTRODUCTION: Alzheimer's, a neurodegenerative disease, is becoming a growing burden and the leading cause of disability among older people, and there is no cure for it. It is set to be the biggest killer among the growing elderly population. The aim of this study was to assess the knowledge of Alzheimer's disease among college students in Kathmandu metropolitan city.

METHODS: This was a descriptive cross-sectional study among 385 randomly selected bachelor students of Kathmandu metropolitan city. The questionnaire included 2 sections. Section I addressed the sociodemographic characteristics of the participants. Section II addressed or covered the Alzheimer's Disease Knowledge Scale (ADKS) test. ADKS contains a set of 30 items, with true and false options. 1 point was given for the correct answer and 0 for the incorrect answer. The final sum was then the total score of the participant. Frequency, percentage, mean, and standard deviation were calculated, and the chi-square test was used to measure the association between two categorical variables.

RESULTS: The mean ADKS (Alzheimer's Disease Knowledge Scale) score is 15.45 +/- 2.95 with the lowest and highest mean total scores of 8 and 26, respectively. 49.5% of the respondents scored above the mean. The number of male and female respondents who scored above the mean is 68 and 95, respectively, with p value

0.71 and odds ratio 0.922. There is no association between gender and knowledge level. Gender seemed to have no effect on the knowledge about Alzheimer's disease on the basis of the Alzheimer's Disease Knowledge Scale (ADKS). However, science students had comparatively better knowledge about disease than management students. The mean score of science and management is 15.9 and 15.04, respectively, with p value 0.004. There is association between knowledge score and faculty.

CONCLUSION: This study concluded that the knowledge level of college students on Alzheimer's disease is below moderate. The findings concluded that there is association between faculty and knowledge score.

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66.

Protein Kinase Ce in the Platelet and Hippocampal Tissue as a Diagnostic Biological Marker in Alzheimer Disease.

Amiri S, Azadmanesh K, Dehghan Shasaltaneh M, Khoshkholgh-Sima B, Naghdi N Basic and Clinical Neuroscience. 10(6):545-556, 2019 Nov-Dec.

[Journal Article]

UI: 32477472

INTRODUCTION: Alzheimer Disease (AD) is a neurodegenerative disorder characterized by the progressive loss of memory and other cognitive functions.

Protein Kinase Ce (PKCe) is an isoform that most effectively suppresses Amyloid Beta (Abeta) production and synaptic loss.

METHODS: In this study, spatial learning and memory for treated rats were evaluated by the Morris water maze test. The activity (total PKC), mRNA expression, and protein level of PKCe in the platelet and hippocampal tissue were evaluated using immunosorbent assay, real-time qPCR, and western blotting analysis, respectively.

RESULTS: The traveled distance was significantly prolonged, and escape latency significantly increased in Abeta-treated groups. PKC activity assay showed that there was a remarkable difference between the Abeta-treated and sham-operated groups

on days 10 and 30 in the hippocampus and also day 30 in platelet after the injection of Abeta. A significant effect in PKC activity was observed between days 0 and 10, days 0 and 30, as well as days 5 and 30. Abeta significantly downregulated the PKCe mRNA expression in the hippocampus of rats on day 30; however, no significant difference was observed in platelet. Western blot analysis demonstrated that Abeta significantly reduced PKCe protein expression in the hippocampus of treated groups on day 30.

CONCLUSION: The expression level of PKCe was downregulated following the injection of Abeta in the hippocampus, but no significant difference was observed between the AD and sham groups in platelet that may be due to the low concentration of PKCe or duration of Abeta exposure in the rat brain.

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Version ID

1

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Year of Publication

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67.

Ceftriaxone improves senile neurocognition damages induced by D-galactose in mice.

Hakimizadeh E, Kaeidi A, Taghipour Z, Mehrzadi S, Allahtavakoli M, Shamsizadeh A, Bazmandegan G, Hassanshahi J, Aflatoonian MR, Fatemi I

Iranian Journal of Basic Medical Sciences. 23(3):368-375, 2020 Mar.

[Journal Article]

UI: 32440324

OBJECTIVES: Ceftriaxone (Cef), a beta-lactam antibiotic, is accompanied by

antioxidant and anti-inflammatory properties. It has been shown that Cef has beneficial effects on Alzheimer's disease. In the current investigation, the effect of Cef in a mice model of aging was investigated.

MATERIALS AND METHODS: Forty male mice were equally aliquoted into four groups as follows: Control (as healthy normal animals), D-galactose (DG) group (treated with 500 mg/kg/day DG for 6 weeks), DG + Cef group (treated with DG plus Cef 200 mg/kg/day for 6 weeks), and Cef group (treated with Cef 200 mg/kg/day for 6 weeks). A battery of behavioral tests was done to evaluate age-related neurocognitive changes. The activities of catalase (CAT), glutathione peroxidase (GPx), and superoxide dismutase (SOD), as well as the level of malondialdehyde (MDA) in the brain, were measured by biochemical methods. Also, to determine the brain damage, histopathological alterations in the hippocampus were measured using hematoxylin and eosin (H&E) staining.

RESULTS: Our results indicate that neurobehavioral dysfunctions of DG can be prevented by co-administration of Cef. We also found that Cef increases the activity of SOD, GPx, and CAT as well as decreasing the level of MDA in the brain of aged mice.

CONCLUSION: Based on our findings, Cef declines neurocognitive dysfunctions in the DG-induced model of aging, possibly through its antioxidative properties.

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1

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Year of Publication

2020

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68.

Tualang honey supplementation as cognitive enhancer in patients with schizophrenia.

Yahaya R, Zahary MN, Othman Z, Ismail R, Nik Him NAS, Abd Aziz A, Dahlan R, Jusoh AF

Heliyon. 6(5):e03948, 2020 May.

[Journal Article]

UI: 32426546

INTRODUCTION: Schizophrenia is a chronic mental illness with clusters of symptoms, including cognitive impairment. This study aimed to explore the effect of Tualang Honey (TH) on cognitive domains, especially as it pertained to the verbal memory of schizophrenia patients.

METHOD: This was a cross-sectional study involved 80 individuals, diagnosed with schizophrenia. The Malay Version Auditory Verbal Learning Test (MVAULT) was used. Data were analysed using SPSS 20.0 software. Intention to treat analysis was applied.

RESULT: A comparison of the total learning score at eight weeks between the two groups based on time effect and time-treatment interaction favoured TH group.

CONCLUSION: This study concludes that by supplementing schizophrenia patients with 8-week of TH did improve total learning performance across domains in the immediate memory among patients with schizophrenia.

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1

Status

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Year of Publication
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69.

Effect of donepezil and memantine on improvement of cognitive function in patients with temporal lobe epilepsy.

Oustad M, Najafi M, Mehvari J, Rastgoo A, Mortazavi Z, Rahiminejad M

Journal of Research in Medical Sciences. 25:29, 2020.

[Journal Article]

UI: 32419786

BACKGROUND: Cognitive impairment is a common complication of patients with temporal lobe epilepsy (TLE). Therefore, the aim of this study was to compare the effects of donepezil and memantine on improving the cognitive function of patients with TLE.

MATERIALS AND METHODS: In a clinical trial study, 70 patients with TLE were divided into two groups of 35 each: 10 mg doses of donepezil (first group) and memantine (second group) were applied for 16 weeks. The level of cognitive function of patients in both groups before and after treatment was determined using Montreal Cognitive Assessment (MoCA) test.

RESULTS: The mean score of MoCA before and after intervention was 23.55 +/- 3.67 and 26.09 +/- 2.5, respectively, in the group treated with memantine, and the mean score of intervention was significantly improved ($P < 0.001$). In the group treated with donepezil, the score before and after the operation was 23.87 +/- 3.18 and 24.35 +/- 2.17, respectively, and no significant difference was observed in this group ($P = 0.38$).

CONCLUSION: Hence, memantine was better than donepezil in the improvement of cognitive impairment in patients with TLE.

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1

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Year of Publication
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70.

Experience of Intensive Care Nurses in Assessment of Postoperative Pain in Patients with Hip Fracture and Dementia.

Krupic F, Grbic K, Senorski EH, Lepara O, Fatahi N, Svantesson E

Materia Sociomedica. 32(1):50-56, 2020 Mar.

[Journal Article]

UI: 32410892

INTRODUCTION: Patients with hip fracture are one of the most demanding groups in the health-care system. One of the most important tasks for nurses is to assess pain and ensure the patients are pain free. Pain assessment in patients with dementia is a well-known challenge for health-care professionals due to the patients' difficulties in verbalising pain problems.

AIM: The aim of this study was to explore the experience of intensive care nurses in assessment of pain in patients with hip fracture and dementia in the postoperative setting.

METHODS: Data were collected through five focus group discussions using open-ended questions and qualitative content analysis. Twenty-one intensive care nurses (6 men and 15 women) participated in the focus group interviews.

RESULTS: Analysis of the data resulted in three main categories: "Communication", "Visual assessment of pain", and "Practical issues" including a number of subcategories. Some of the factors which influence assessment of pain in patients with dementia are the lack of information and knowledge about the patients, which causes loss of time and increased stress. The different forms of communication and ways of assessing pain in these patients were other factors mentioned by nurses as hindrances regarding assessment of the pain.

CONCLUSION: In order to improve assessment of pain, more knowledge and information about the patients are needed and better coordination between the pre- and postoperative departments regarding these patients. In this context, different intervention studies on patient's hip fracture and dementia are needed to increase knowledge and awareness regarding this group of patients.

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71.

Vitamin E therapy prevents the accumulation of congophilic amyloid plaques and neurofibrillary tangles in the hippocampus in a rat model of Alzheimer's disease.

Jahanshahi M, Nikmahzar E, Sayyahi A

Iranian Journal of Basic Medical Sciences. 23(1):86-92, 2020 Jan.

[Journal Article]

UI: 32395206

OBJECTIVES: Vitamin E may have beneficial effects on oxidative stress and Abeta-associated reactive oxygen species production in Alzheimer's disease. But, the exact role of vitamin E as a treatment for Alzheimer's disease pathogenesis still needs to be studied. Hence, we examined the therapeutic effects of vitamin E on the density of congophilic amyloid plaques and neurofibrillary tangles in rats' hippocampi.

MATERIALS AND METHODS: Wistar rats were randomly assigned to control (no drug treatment), sham scopolamine (3 mg/kg)+saline and Sham

scopolamine+sesame oil groups, and three experimental groups that received

scopolamine+vitamin E (25, 50, and 100 mg/kg/day) daily for 14 days after

scopolamine injection. The rats' brains were collected immediately following

transcardial perfusion and fixed in 4% paraformaldehyde. Pathological brain

alterations were monitored through Congo red and bielschowsky silver staining.

RESULTS: Scopolamine treatment led to a significant increase in the density of

congophilic amyloid plaques and neurofibrillary tangles in the hippocampus. IP

injection of vitamin E in three doses (25, 50, and 100 mg/kg/day) significantly

reversed the scopolamine-induced increase of the congophilic amyloid plaque

density and density of neurofibrillary tangles in the hippocampus. Although vitamin E

(25 and 50 mg/kg/day) doses were also effective, but a 100 mg/kg/day dose of

vitamin E was more effective in the reduction of congophilic amyloid plaque and neurofibrillary tangle density.

CONCLUSION: Vitamin E could exert a therapeutic effect in the reduction of congophilic amyloid plaque and neurofibrillary tangle density in the hippocampus of scopolamine-treated rats and it is useful for Alzheimer's disease.

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1

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Year of Publication

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72.

The Relationship between Polyneuropathy and Cognitive Functions in Type 2 Diabetes Mellitus Patients.

Timer SM, Timer E, Yukselen NP, Gokyigit MC

Sisli Etfal Hastanesi Tp Bulteni. 54(1):41-46, 2020.

[Journal Article]

UI: 32377132

OBJECTIVES: Type 2 Diabetes Mellitus (DM) is a risk factor for mild cognitive impairment (MCI), Alzheimer's disease and vascular dementia. However, it is not known which pathophysiological mechanisms lead to impairment in cognitive functions in Type 2 DM. This study aims to compare the cognitive functions of diabetic patients with and without polyneuropathy using standardized Mini-Mental Test (MMSE) and the Montreal Cognitive Assessment Scale (MoCA) and to assess whether the presence of polyneuropathy is a predictive factor for the development of cognitive impairment.

METHODS: Patients with DM who underwent our EMG laboratory for polyneuropathy between January 2014 and January 2015 were included in this study. Patients who underwent electrophysiological examinations were evaluated for polyneuropathy. Patients with polyneuropathy were classified as a patient group and other patients as a control group. In all cases, MMSE and MoCA were administered. The demographic data and educational status of the patients were recorded. Hypertension, coronary artery disease, smoking and alcohol use were questioned. Their complaints, duration of illness and the treatment they were receiving were questioned. Glycosylated hemoglobin (HBA1C) values in the last three months and physical examination findings of patients were recorded. Patients with and without polyneuropathy were compared with statistical methods.

RESULTS: Polyneuropathy was detected in 34 (42%) of the 81 patients who participated in our study. The age, disease duration and HBA1C levels were statistically higher in the polyneuropathy group than in the control group ($p=0.024$, $p=0.000$, $p=0.016$). However, there was no statistically significant difference between MMSE and MoCA scores of these groups. In both groups, there were no patients scoring below the MMSE cut-off value of 24. Seventeen of the 34 patients (50%) in the polyneuropathic group and 19 (40,4%) of the 47 patients in the control group had scores below the MoCA cut-off value 21. However, there was no statistically significant difference between the two groups. We also found that the mean MoCA value of all DM patients was 21, which was the MoCA cut-off value. Also, factors affecting cognitive functions in all Type 2 DM patients were evaluated by logistic regression analysis, and it was found that duration of education was an independent factor affecting cognitive impairment ($OR=8.167$; $p=0.001$).

CONCLUSION: In our study, we did not observe significant differences between MMSE and MoCA scores of Type 2 DM patients with and without polyneuropathy. However, the cross-sectional nature of our study makes it impossible to comment on this issue. To clarify whether the presence of polyneuropathy is a predictive factor in the development of cognitive impairment in Type 2 DM, there is a need for a larger sample group and long-term follow-up studies. It has also been shown that patients with Type 2 DM may have low scores according to the MOBID cut-off value even though peripheral neurologic involvement findings are not observed. In the Type 2 DM population, it has also been shown that MoCA may be affected by education level.

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Year of Publication

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73.

Irisin protects the substantia nigra dopaminergic neurons in the rat model of Parkinson's disease.

Zarbakhsh S, Safari M, Aldaghi MR, Sameni HR, Ghahari L, Khaleghi Lagmouj Y, Rahimi Jaber K, Parsaie H

Iranian Journal of Basic Medical Sciences. 22(7):722-728, 2019 Jul.

[Journal Article]

UI: 32373292

OBJECTIVES: Exercise ameliorates the quality of life and reduces the risk of neurological derangements such as Alzheimer's (AD) and Parkinson's disease (PD). Irisin is a product of the physical activity and is a circulating hormone that regulates the energy metabolism in the body. In the nervous system, Irisin influences neurogenesis and neural differentiation in mice. We previously demonstrated that co-treatment of bone marrow stem cells (BMSCs) with a neurotrophic factor reduce Parkinson's symptoms. Our goal in this project was to evaluate whether Irisin with BMSCs can protect the dopaminergic (DA) neurons in PD.

MATERIALS AND METHODS: 35 adult male Wistar rat weighing (200-250 g) were chosen. They were separated into five experimental groups (n=7). To create a Parkinson's model, intranasal (IN) administration of the MPTP (1-methyl-4-phenyl-1,2,3,6-tetrahydropyridine) was used. The BMSCs (2x10⁶) and Irisin (50 nm/ml) was used for 7 days for treatment after creation of the PD model. After completion of the tests (4 weeks), their brains were used for the TUNEL and immunohistochemical (IHC) assays.

RESULTS: One of the important results of this study was that the Irisin induce BMSCs transport into the injured area of the brain. Co-treatment of the Irisin with BMSCs increased tyrosine hydroxylase-positive neurons (TH+) in substantia nigra (SN) and striatum of the PD mice brain. In this group, the number of TUNEL-positive cells significantly decreased. Behavioral symptoms were better in the combination group and Irisin simultaneously.

CONCLUSION: Co- treatment of Irisin with BMSCs protects the DA neurons from degeneration and apoptotic process after MPTP injection.

Version ID

1

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Year of Publication

2019

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74.

Magnesium Increases the Protective Effect of Citicoline on Aluminum Chloride-induced Cognitive Impairment.

Hosseini-Sharifabad A, Rabbani M, Seyed-Yousefi Y, Safavi M

Clinical Psychopharmacology & Neuroscience. 18(2):241-248, 2020 May 31.

[Journal Article]

UI: 32329305

OBJECTIVE: Alzheimer's disease is a popular neurodegenerative disorder which is growing in the elderly people. Exposure to environmental pollutant like aluminum could trigger or accelerate its involved mechanisms like tau phosphorylation. The current study will evaluate the effect of alone or co-administration of Citicoline or/and magnesium on the aluminum chloride induced memory impairment.

METHODS: Male albino mice were randomly divided into different groups (n = 7). Memory impairment was induced via orally administration of 300 mg/kg Aluminum Chloride for 28 days. Based on respective group, animals received 100, 250, 500 mg/kg of Citicoline or 50, 100, 150 mg/kg of Magnesium sulfate (MgSO₄), intraperitoneally. In co-administration, 50 mg/kg of MgSO₄ injected concomitantly with 100, 250, or 500 mg/kg of Citicoline. Rivastigmine (2 mg/kg intraperitoneally) was used as a positive control. Memory was evaluated using the Object Recognition Task (ORT) and Passive Avoidance Test (PAT).

RESULTS: The studied doses of Citicoline or MgSO₄ when administered individually showed significant increase in the discrimination index in ORT and latency time in the PAT compared to the Aluminium chloride (AlCl₃) treated group. Concomitant injection of 50 mg/kg MgSO₄ with the different doses of Citicoline strongly increased the above indices values in comparison to each alone.

CONCLUSION: The findings show, individual administration of Citicoline or MgSO₄ inverted the AlCl₃-induced memory impairment in a dose independent manner. The addition of MgSO₄ to the Citicoline showed a synergistic effect in the PAT and likely additive effect in the ORT.

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1

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Year of Publication
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75.

Experimental Effects of Acute High-Intensity Resistance Exercise on Episodic
Memory Function: Consideration for Post-Exercise Recovery Period.

Loprinzi PD, Green D, Wages S, Cheke LG, Jones T
Journal of Lifestyle Medicine. 10(1):7-20, 2020 Jan 31.
[Journal Article]

UI: 32328444

BACKGROUND: The present experiments evaluated the effects of acute high-
intensity resistance exercise on episodic memory.

METHODS: Two experiments were conducted. For Experiment 1, participants (N =
40; Mage = 21.0 years) were randomized into one of two groups, including an
experimental exercise group and a control group (seated for 20 min). The
experimental group engaged in an acute bout of resistance exercises (circuit style
exercises) for 15 minutes, followed by a 5-min recovery period. Memory function was
subsequently assessed using a multiple trial (immediate and delay), word-list
episodic memory task (Rey Auditory Verbal Learning Test, RAVLT), and then
followed by a comprehensive, computerized assessment of episodic memory
(Treasure Hunt task, THT). The THT involved a spatio-temporal assessment of what,
where, and when components of episodic memory. Experiment 2 evaluated if altering
the recovery period would influence the potential negative effects of high-intensity
resistance exercise on episodic memory function. For Experiment 2, participants (N =
51) were randomized into the same acute resistance exercise protocol but either with
a 10-min recovery period, 20-min recovery period, or a control group.

RESULTS: For Experiment 1, for RAVLT, the exercise group performed worse
(Fgroup x time = 3.7, p = .001, eta 2p = .09). Across nearly all THT outcomes, the
exercise group had worse spatio-temporal memory than the control group. These
results suggest that high-intensity resistance exercise (with a 5-min recovery) may
have a detrimental effect on episodic memory function. For Experiment 2, for RAVLT,
the exercise with 10-min recovery group performed better (Fgroup x time = 3.1, p =
.04, eta 2p = .11). Unlike Experiment 1, exercise did not impair spatio-temporal
memory, with the 20-min exercise recovery group having the best "where"
component of episodic memory.

CONCLUSION: Collectively, the results from these two experiments suggest that
acute high-intensity resistance exercise may impair episodic memory when a short
exercise recovery period (e.g., 5-min) is employed, but with a longer recovery period
(10+ min), acute high-intensity resistance exercise may, potentially, enhance
episodic memory.

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Version ID

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Year of Publication

2020

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76.

Prevalence of psychiatric disorders among older adults in Jodhpur and stakeholders
perspective on responsive health system.

Patel M, Bhardwaj P, Nebhinani N, Goel AD, Patel K

Journal of Family Medicine & Primary Care. 9(2):714-720, 2020 Feb.

[Journal Article]

UI: 32318408

BACKGROUND: Ageing is an inescapable reality of human existence. The elderly
population of India is steadily increasing with growing mental health needs which
pose many challenges for the health care system. The aim of this study is to assess
anxiety, depression, and cognitive disorders among urban and rural elderly and to
explore the availability of social support mechanisms and a responsive health system
for elderly.

METHODS: This study is a mixed-method approach. For a quantitative study, a
community-based cross-sectional survey is conducted in Jodhpur, Rajasthan. A total
of 330 elderly persons aged 60 years and above are randomly screened for
depression (GDS), anxiety (GAD), and cognitive impairment (HMSE). Further for a
qualitative study, in-depth interviews are conducted with 7 key informants including
policy and program managers, service providers, and facilitators from the state. For
quantitative data analysis, Excel and SPSS are used and for Qualitative data
analysis, Thematic Framework Approach is used.

RESULTS: The mean age of the respondents is 67.9 +/- 7.8. The prevalence of
severe depression is 17%, severe anxiety is 10.3%, and cognitive impairment is
51.2%. The prevalence of all the three is more in rural elderly as compared to urban
elderly as well as more in female individuals as compared to males. Qualitative
analysis revealed that there are challenges in early identification of mental disorders
at both the levels: service providers and elderly. Psychological and financial issues

are also seen in elderly who are not supported by their children. There are cases of fear for elder abuse and influence of western culture in the society.

CONCLUSION: There is a sizeable prevalence of psychological issues in elderly population. Therefore, there is a need to adopt holistic and integrated psychogeriatric services for the improvement of quality of life in elderly.

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1

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Year of Publication

2020

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77.

Screening for Cognitive Frailty Using Short Cognitive Screening Instruments: Comparison of the Chinese Versions of the MoCA and Qmci Screen.

Xu Y, Lin Y, Yi L, Li Z, Li X, Yu Y, Guo Y, Wang Y, Jiang H, Chen Z, Svendrovski A, Gao Y, Molloy DW, O'Caoimh R

Frontiers in Psychology. 11:558, 2020.

[Journal Article]

UI: 32308636

BACKGROUND: Cognitive frailty describes cognitive impairment associated with physical decline. Few studies have explored whether short cognitive screens identify frailty. We examined the diagnostic accuracy of the Chinese versions of the Quick Mild Cognitive Impairment (Qmci-CN) screen and Montreal Cognitive Assessment (MoCA-CN) in identifying cognitive frailty.

METHODS: Ninety-five participants with cognitive symptoms [47 with mild cognitive impairment (MCI), 34 with subjective cognitive disorder, and 14 with dementia] were included from two outpatient rehabilitation clinics. Energy (work intensity) and physical activity levels were recorded. Cognitive frailty was diagnosed by an interdisciplinary team using the IANA/IAGG consensus criteria, stratified on the Clinical Frailty Scale (CFS). Instruments were administered sequentially and randomly by trained assessors, blind to the diagnosis.

RESULTS: The mean age of the sample was 62.6 +/- 10.2 years; median CFS score was 4 +/- 1 and 36 (38%) were cognitively frail. The Qmci-CN had similar accuracy in

differentiating the non-frail from cognitively frail compared to the MoCA-CN, AUC 0.82 versus 0.74, respectively ($p = 0.19$). At its optimal cut-off ($\leq 55/100$), the Qmci-CN provided a sensitivity of 83% and specificity of 67% versus 91% and 51%, respectively, for the MoCA-CN ($\leq 23/30$). Neither was accurate in separating MCI from cognitive frailty but both accurately separated cognitive frailty from dementia.

CONCLUSION: Established short cognitive screens may be useful in identifying cognitive frailty in Chinese adults with cognitive complaints but not in separating MCI from cognitive frailty. The Qmci-CN had similar accuracy to the MoCA-CN and a shorter administration time in this small and under-powered study, necessitating the need for adequately powered studies in different healthcare settings.

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Year of Publication
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78.

Curcumin Effects on Memory Impairment and Restoration of Irregular Neuronal Distribution in the Hippocampal CA1 Region After Global Cerebral Ischemia in Male Rats.

Kamali Dolatabadi L, Emamghoreishi M, Namavar MR, Badeli Sarkala H

Basic and Clinical Neuroscience. 10(5):527-539, 2019 Sep-Oct.

[Journal Article]

UI: 32284841

INTRODUCTION: Global Cerebral Ischemia (GCI) causes neuronal damage with subsequent neurological and cognitive impairments. Curcumin has anti-inflammatory, antioxidant, and neuroprotective properties, which makes it a potential candidate for improving GCI-induced impairments. This study aimed to investigate the effects of curcumin on the neurological and memory deficits, as well as spatial neuronal distribution in the Cornu Ammonis 1 region after GCI in rats.

METHODS: 56 Sprague-Dawley male rats were randomly assigned into 4 groups of sham (n=14), control (n=14), curcumin 50 mg/kg (n=14), and curcumin 100 mg/kg (n=14). Each group was divided into the two subgroups of short-term (7 days) and long-term (28 days) treatment periods. The Neurological Severity Score (NSS), passive avoidance task, and the traction test were performed at postoperative days of 0, 1, 2, 3, 7, 14, 21, and 28. The novel object recognition test and Voronoi tessellation were carried out on days 7 and 28 after GCI.

RESULTS: Curcumin 100 mg/kg significantly decreased neurological severity score on postoperative days of 7 and 28 compared with the control ($P<0.001$) and curcumin 50 mg/kg groups ($P<0.05$ - $P<0.001$), respectively. Also, curcumin 100 mg/kg significantly increased step-through latency times on postoperative days of 3-28 and 14-28 compared with the control ($P<0.05$ - $P<0.001$) and curcumin 50 mg/kg groups ($P<0.01$ - $P<0.001$). Moreover, it increased the novelty preference index during the novel object recognition test in the 28-day treatment subgroup after GCI.

Curcumin (100 mg/kg) could maintain the neuronal aggregation in the CA1 region after GCI at a level near to what is generally observed in normal rats.

CONCLUSION: Curcumin could improve memory and neurological deficits and restore irregular neuronal distribution in the CA1 region after GCI in a time-dependent manner, and its higher dose was more effective than its lower dose. Curcumin may have beneficial effects on reducing brain complications after ischemia.

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79.

High anxiety and depressive symptoms in partners of type 1 diabetes persons in a sample of the Brazilian population.

Buin E, Pavin EJ, Silveira MSVM

Diabetology & metabolic syndrome. 12:23, 2020.

[Journal Article]

UI: 32211074

BACKGROUND: Type 1 diabetes (T1D) affects psychologically not only the persons with diabetes themselves but affects their family members. Few studies were conducted to investigate mental health in T1D partners. This study aims: (1) to investigate the frequency of depressive and anxiety symptoms in T1D partners and, (2) to investigate the associations among partners' depressive and anxiety symptoms and their sociodemographic and behavioral characteristics, and (3) to investigate the associations among partners' depressive and anxiety symptoms and clinical, laboratory and demographic characteristics of their T1D spouses in a Brazilian population.

METHODS: In a transversal study 72 T1D partners were interviewed. Partners were invited to take part in the study during their T1D spouses' routine consultations.

Those who consented to take part in the study signed the consent form. This study followed the principles of the Declaration of Helsinki and was approved by the University Ethics in Research Committee. Inclusion criteria were T1D partners age ≥ 18 and T1D diagnosis > 6 months. Exclusion criteria were cognitive impairment,

history of major psychiatric disorders, and severe chronic and terminal diseases. Depressive symptoms were evaluated by the depression subscale of the Hospital Anxiety and Depression scale (HADD) and anxiety symptoms were evaluated by the anxiety subscale of the same instrument (HADA). T1D partners were divided into subgroups according to score ≥ 8 and < 8 in both subscales. Demographic and clinical data were obtained from interview. Descriptive analyses were undertaken using means and percentages, as appropriate. Differences between groups were assessed by the Mann-Whitney test for numerical variables, by the Chi Square test or by Fisher's exact test for categorical variables, as appropriate. All analyses were undertaken using SAS version 9.2 for Windows. Statistical significance was set at 0.05.

RESULTS: Of all 72 T1D partners, 72.2% were male, mean age was 42.7 ± 14.1 years old, years of school attendance were 11.8 ± 3.9 years, and 48.5% had income reaching until 3 Brazilian minimal wages. Forty-three percent reported high anxiety symptoms (HADA ≥ 8) and 18.1% reported high depressive symptoms (HADD ≥ 8). Comparing T1D partners group with HADA ≥ 8 and < 8 , the first one was associated with CGM use (41.94% vs 19.51%; $p = 0.03$). Similarly, comparing T1D partners group with HADD ≥ 8 and < 8 , the first one was associated with (1) longer duration of T1D of their spouses (28.6 ± 7.1 vs 22.4 ± 12.2 ; $p = 0.02$); (2) less years of school attendance of T1D partners (9.3 ± 3.2 vs 12.3 ± 3.8 ; $p = 0.02$), and (3) higher number of hypoglycemic episodes requiring other person's intervention (6.3 ± 8.9 vs 2.4 ± 4.7 ; $p = 0.009$). Seventy-six percent of partners who helped personally in their spouses' hypoglycemia recovery had HADD ≥ 8 vs 44.7% with HADD < 8 ($p = 0.03$). Likewise, 84.6% vs 54.2% of partners in which their spouses have T1D chronic complications had HADD ≥ 8 and < 8 , respectively ($p = 0.04$).

CONCLUSION: This study showed a high frequency of relevant anxiety and depressive symptoms in this T1D partner population. Several issues related to T1D of their spouses were associated with these symptoms. These results emphasize the need to incorporate the psychological and psychiatric aspects into T1D partners' education and care.

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80.

Biomarkers in Alzheimer's disease: Evaluation of platelets, hemoglobin and vitamin B12.

Dos Santos GAA, Pardi PC

Dementia & Neuropsychologia. 14(1):35-40, 2020 Jan-Mar.

[Journal Article]

UI: 32206196

Currently, the most likely hypotheses as the cause of Alzheimer's disease are deposition of amyloid beta peptide in the cerebral cortex and hyperphosphorylation of Tau protein. The diagnosis of Alzheimer's disease is based on the exclusion of other diseases, behavioral assessments, and blood and imaging tests. Biotechnology has created interesting perspectives for the early detection of Alzheimer's disease through blood analysis, with special attention to platelets, hemoglobin and vitamin B12.

OBJECTIVE: To evaluate the concentrations of platelets, hemoglobin and vitamin B12 in the blood of older adults with and without dementia of Alzheimer's disease.

METHODS: A case-control study involving 120 individuals was conducted, seeking to establish a correlation between changes in platelet, hemoglobin and vitamin B12 concentrations in patients with confirmed AD and in individuals in the inclusion group without AD. The study met the established ethical requirements.

RESULTS: Hemoglobin and platelet levels were statistically lower in patients with AD. The biochemical evaluation in AD patient and healthy groups for vitamin B12 showed a decrease in the levels of this compound in patients with AD.

CONCLUSION: We demonstrated the feasibility of the use of blood biomarkers as predictive markers for the diagnosis of AD.

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Other Abstract

Publisher

Atualmente, as hipoteses mais provaveis como causa da doenca de Alzheimer sao a deposicao do peptideo beta amiloide no cortex cerebral e a hiperfosforilacao da proteina Tau. O diagnostico da doenca de Alzheimer baseia-se na exclusao de outras doencas, avaliacoes comportamentais e exames de imagem e sangue. A biotecnologia criou perspectivas interessantes para a deteccao precoce da doenca de Alzheimer, pela analise sanguinea, com atencao especial as plaquetas, hemoglobina e vitamina B12. OBJETIVO: Avaliar as concentracoes de plaquetas, hemoglobina e vitamina B12 no sangue de idosos com e sem demencia de Alzheimer. METODOS: O estudo de caso-controle envolveu 120 individuos,

buscando correlacao entre mudancas nas concentracoes de plaquetas, hemoglobina e vitamina B12 em pacientes com DA confirmada e individuos do grupo de inclusao, sem DA.RESULTADOS: Os niveis de hemoglobina e plaquetas sao estatisticamente mais baixos em pacientes com DA. A avaliacao bioquimica em pacientes com DA e grupos saudaveis para vitamina B12 mostrou uma diminuicao nos niveis deste composto em pacientes com DA.CONCLUSAO: Demonstramos a viabilidade do uso de biomarcadores sanguineos como marcadores preditivos para o diagnostico de DA.

Language: Portuguese

Year of Publication

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81.

The Association Between Body Mass Index and Comorbidity, Quality of Life, and Cognitive Function in the Elderly Population.

Rambod M, Ghodsbini F, Moradi A

International Journal of Community Based Nursing & Midwifery. 8(1):45-54, 2020

Jan.

[Journal Article]

UI: 32039279

BACKGROUND: Debate still exists regarding physical and cognitive factors associated with Body Mass Index (BMI) in the elderly population. This study aimed to determine the association between BMI and comorbidity, Quality of Life (QOL), and cognitive function in the elderly population.

METHODS: This cross-sectional study was conducted from October 2017 to January 2018. The participants included 246 old people who referred to Imam Reza elderly clinic, Shiraz, Iran. The data were collected using Leipad Quality of Life Questionnaire and Mini-Mental State Examination (MMSE). Indeed, weight and height were measured to assess the BMI. The data were entered into SPSS, version 21, and analyzed using ANOVA, Chi-square test, Pearson correlation coefficient, and multiple regression analysis.

RESULTS: This study showed that 104 (47%) of the participants were overweight and obese. The mean $\pm$ SD score of QOL was 46.14 $\pm$ 12.01. Additionally, 93 (37.8%) of the participants had cognitive impairment. The results showed a significant difference among normal weight, overweight, and obese groups regarding the mean scores of QOL ($P<0.001$) and cognitive function ($P<0.001$). Moreover, 29% of the changes in BMI was explained by QOL, cognitive function, and having hypertension and cancer. Among these variables, the associations between BMI and QOL ($r=-0.52$, $P<0.001$) and cognitive function ($r=-0.28$, $P<0.001$) were significant.

CONCLUSION: The results showed that half of the old people suffered from overweight and obesity. Moreover, roughly one-third had cognitive impairment. In addition, BMI was associated with QOL and cognitive function in the elderly.

Therefore, healthcare workers might use these findings to design prevention and treatment programs targeting the elderly population.

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82.

Effects of Tai Chi Chuan on Inhibitory Control in Elderly Women: An fNIRS Study.

Yang Y, Chen T, Shao M, Yan S, Yue GH, Jiang C

Frontiers in Human Neuroscience. 13:476, 2019.

[Journal Article]

UI: 32038205

BACKGROUND: Inhibitory control is a sub-ability of executive function and plays an important role in the entire cognitive process. However, declines in inhibitory control during aging significantly impair the quality of life of elderly people. Investigating methods to delay the decline of inhibitory control has become a focal point in current research. Tai Chi Chuan (TCC) is one effective method used to delay cognitive declines in older adults. However, the specific effects of TCC on inhibitory control and the mechanisms through which TCC may improve cognition in older adults have not been comprehensively investigated.

OBJECTIVE: The study explores possible neurological mechanisms related to the effects of TCC interventions on inhibitory control in older people using a functional near-infrared spectroscopy (fNIRS) technique and reaction times (RTs).

METHODS: A total of 26 healthy, elderly people who had not received TCC training completed all study procedures. The subjects were randomized to either the TCC group or the control group. Subjects in the TCC group were taught TCC by a certified instructor and trained for 8 weeks. The control group continued to perform general daily activities. The Flanker task was administered to every participant to evaluate inhibitory control pre- and post-intervention. While participants were performing the Flanker task, fNIRS data were collected.

RESULTS: Post-intervention, significant differences for incongruent flankers were found only for the TCC intervention group. Faster RTs were observed for the incongruent flankers in the TCC group than in the control group ($p < 0.05$). Analysis of the fNIRS data revealed an increase in oxy-Hb in the prefrontal cortex during the incongruent flankers after the TCC exercise intervention.

CONCLUSION: The TCC intervention significantly improved inhibitory control in older adults, suggesting that TCC is an effective, suitable exercise for improving executive function and neurological health in elderly people.

CLINICAL TRIAL REGISTRATION: Chinese Clinical Trial Register,
ChiCTR1900028457.

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83.

Differences in Psychotropic Drug Prescribing Between Ethnic Groups of People with
Dementia in the United Kingdom.

Jones ME, Petersen I, Walters K, Bhanu C, Manthorpe J, Raine R, Mukadam N,
Cooper C

Clinical Epidemiology. 12:61-71, 2020.

[Journal Article]

UI: 32021472

PURPOSE: To test hypotheses that minority ethnic people with dementia in the UK
receive fewer anti-dementia drugs and more psychotropic and anticholinergic drugs

associated with harms.

PATIENTS AND METHODS: We analyzed UK primary care electronic health records from The Health Improvement Network (THIN) database (2014-2016), comparing psychotropic drug prescribing initiation and duration between people with dementia from White, Black, and Asian ethnic groups. We repeated analyses in people (aged 50+) without dementia, to explore whether any differences found reflected prescribing patterns in the general older population, or were specific to dementia.

RESULTS: We included 53,718 people with and 1,648,889 people without dementia. Among people with dementia, compared to White ethnic groups, Asian people were less likely to be prescribed anti-dementia drugs when they were potentially indicated (adjusted prevalence rate ratio 0.86 (95% Confidence Interval 0.76-0.98)), and received them for on average 15 days/year less. Compared to White groups, Asian and Black individuals with dementia were no more likely to take an antipsychotic drug, but those that had were prescribed them for 17 and 27 days/year more, respectively (190.8 (179.6-199.1) and 200.7 (191.1-206.5) days). Black people were less likely to be prescribed anxiolytics/hypnotics (0.60 (0.44-0.8)), but the duration these drugs were prescribed was similar across ethnic groups. Asian people were more likely to be prescribed anticholinergic drugs (1.43 (1.19-1.73)), in analyses unadjusted for cardiovascular comorbidities. Among people without dementia, those in the Asian and Black ethnic groups were less likely to be prescribed psychotropic drugs, relative to people from White groups.

CONCLUSION: Among people with dementia, Asian groups received less potentially beneficial symptomatic treatments, and Asian and Black groups were prescribed antipsychotic drugs for longer than White ethnic groups. Our findings may indicate care inequalities.

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84.

Radioiodine Therapy of Graves' Disease and the Uptake Paradox.

Malapure SS, Mukherjee A, Bal C

Indian Journal of Nuclear Medicine. 35(1):17-20, 2020 Jan-Mar.

[Journal Article]

UI: 31949364

PURPOSE OF THE STUDY: Radioiodine (¹³¹I) therapy is approved and well-accepted modality for the treatment of hyperthyroidism. The dosage of ¹³¹I for successful treatment is based on many factors; however, an objective tool to determine the dose was missing. In a retrospective study, we found that high ¹³¹I uptake values required more dose to achieve desirable results contrary to the belief.

MATERIALS AND METHODS: Clinically and scintigraphically proven Graves' disease patients with high ¹³¹I uptake (>50%) were accrued for this study and block randomized into low-dose (Group I) and high-dose (Group II) groups. Low activity (5 mCi) was administered in Group I and higher activity (10 mCi) in Group II. The patients were followed up after 3 months with thyroid function tests to determine the outcome.

RESULTS: A total of 344 patients were analyzed at the end of 3 months, with 174 in low-dose group and 170 in high-dose group. Euthyroidism/hypothyroidism was achieved in significantly higher number of patients as compared to the low-dose group.

CONCLUSION: The higher dose of ¹³¹I is required to achieve euthyroidism/hypothyroidism in patients with high ¹³¹I uptake.

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85.

A Comparison Study of Cilostazol and Aspirin on Changes in Volume of Cerebral Small Vessel Disease White Matter Changes: Protocol of a Multicenter, Randomized Controlled Trial.

Han HJ, Kim BC, Youn YC, Jeong JH, Kim JH, Lee JH, Park KH, Park KW, Kim EJ, Oh MS, Shim YS, Park HY, Yoon B, Yoon SJ, Cho SJ, Park KC, Na DL, Park SA, Lee JM, Choi SH

Dementia and Neurocognitive Disorders. 18(4):138-148, 2019 Dec.

[Journal Article]

UI: 31942173

BACKGROUND AND PURPOSE: Cerebral small vessel disease (CSVD) is the most common cause of vascular dementia and a major contributor to mixed dementia. CSVD is characterized by progressive cerebral white matter changes (WMC) due to chronic low perfusion and loss of autoregulation. In addition to its antiplatelet effect, cilostazol exerts a vasodilating effect and improves endothelial function. This study aims to compare the effects of cilostazol and aspirin on changes in WMC volume in CSVD.

METHODS: The comparison study of Cilostazol and aspirin on cHAnGES in volume of cerebral smaLL vEsSEL disease white matter chaNGEs (CHALLENGE) is a double blind, randomized trial involving 19 hospitals across South Korea. Patients with moderate or severe WMC and ≥ 1 lacunar infarction detected on brain magnetic resonance imaging (MRI) are eligible; the projected sample size is 254. Participants are randomly assigned to a cilostazol or aspirin group at a 1:1 ratio. Cilostazol slow release 200 mg or aspirin 100 mg are taken once daily for 2 years. The primary outcome measure is the change in WMC volume on MRI from baseline to 104 weeks. Secondary imaging outcomes include changes in the number of lacunes and cerebral microbleeds, fractional anisotropy and mean diffusivity on diffusion tensor imaging, and brain atrophy. Secondary clinical outcomes include all ischemic strokes, all vascular events, and changes in cognition, motor function, mood, urinary symptoms, and disability.

CONCLUSIONS: CHALLENGE will provide evidence to support the selection of long-term antiplatelet therapy in CSVD.

TRIAL REGISTRATION: ClinicalTrials.gov Identifier: NCT01932203.

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The Effect of Multi-tasking Exercise Intervention on Cognitive Function in Elderly and Cognitive Impairment Patients: a Pilot Multicenter Study.

Lee HJ, Lee M, Park H, Park JH, Park KW

Dementia and Neurocognitive Disorders. 18(4):122-129, 2019 Dec.

[Journal Article]

UI: 31942171

BACKGROUND AND PURPOSE: The purpose of this study was to investigate the impact of multi-tasking exercise on cognitive and motor function in patients with mild cognitive impairment (MCI) and cognitive normal elderly (CNE).

METHODS: A total of 144 subjects were recruited from 14 regional dementia centers in Busan Metropolitan City. With the exception of 77 subjects unsuitable for the study, 67 subjects participated in the intervention. The Korean version of Mini-Mental State Examination for Dementia Screening (MMSE-DS), Subject Memory Complain Questionnaire (SMCQ), and the Korean version of the Short-Form Geriatric Depression Scale (SGDS-K) were used to determine changes in cognitive function and depressive symptoms pre-post intervention in patients with MCI and CNE, respectively. To observe changes in motor function, 5-chair sit to stand and 5-meter walking speed on the lower body strength and walking ability were also measured in both groups.

RESULTS: For the MCI group, changes in cognitive and depressive symptoms on the tasks of MMSE-DS ($p=0.006$), SMCQ ($p=0.000$), SGDS-K ($p=0.000$) and the result of the motor function analysis on the items of 5-chair sit to stand ($p=0.000$), normal speed ($p=0.004$) and maximal speed ($p=0.002$) of 5-meter walking speed ($p<0.005$). In the CNE group, all the items except MMSE-DS ($p=0.068$) and SGDS-K ($p=0.095$) showed statistically significant improvement.

CONCLUSIONS: Multi-tasking exercise intervention, including exercise and cognitive tasks, may possibly improve the cognitive and motor function in patients with MCI and CNE.

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87.

Comparison of cholinesterase inhibitor safety in real-world practice.

Carney G, Bassett K, Wright JM, Maclure M, McGuire N, Dormuth CR

Alzheimer's & Dementia : Translational Research & Clinical Interventions. 5:732-739, 2019.

[Journal Article]

UI: 31921965

INTRODUCTION: Cholinesterase inhibitors (ChEIs) are widely used to treat mild to moderate Alzheimer's disease and related dementia. Clinical trials have focused on placebo comparisons, inadequately addressing within-class comparative safety.

METHODS: New users of ChEIs in British Columbia were categorized into five study cohorts: low-dose donepezil, high-dose donepezil, galantamine, rivastigmine patch, and oral rivastigmine. Comparative safety of ChEIs assessed hazard ratios using propensity score adjusted Cox regression.

RESULTS: Compared with low-dose donepezil, galantamine use was associated with a lower risk of mortality (adjusted hazard ratio: 0.84, 95% confidence interval: 0.60-1.18), cardiovascular serious adverse events (adjusted hazard ratio: 0.78, 95% confidence interval: 0.62-0.98), and entry into a residential care facility (adjusted hazard ratio: 0.72, 95% confidence interval: 0.59-0.89).

DISCUSSION: Given the absence of randomized trial data showing clinically meaningful benefit of ChEI therapy in Alzheimer's disease, our study suggests preferential use of galantamine may at least be associated with fewer adverse events than treatment with donepezil or rivastigmine.

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88.

Evaluation of Postoperative Delirium: Validity and Reliability of the Nursing Delirium Screening Scale in the Turkish Language.

Cinar F, Eti Aslan F

Dementia and Geriatric Cognitive Disorders Extra. 9(3):362-373, 2019 Sep-Dec.

[Journal Article]

UI: 31911787

PURPOSE: Postoperative delirium is the most well-known form of postoperative cognitive impairment in all patient groups, especially in the elderly. Delirium is a syndrome that causes serious consequences, increasing mortality and morbidity rates and extending the length of hospital stay. The aim of this study was to evaluate the validity and reliability of the Turkish version of the Nursing Delirium Screening Scale (Nu-DESC).

METHOD: One hundred twelve patients who were hospitalized for a surgical operation in the orthopedics, neurosurgery, and general surgery clinic of a state hospital for 3 months were evaluated concurrently (and independently for delirium). Patients were observed by clinical nurses 3 times over a 24-h period. The presence of delirium was diagnosed by 2 neurologists according to DSM-IV criteria. Student's t test, the chi² test, and the Mann-Whitney U test were used, and construct validity, intrascale factor analysis, interrater reliability, and specificity and sensitivity (ROC) analyses were performed for descriptive analysis. SPSS 25.0 and MedCalc18.6 were used for statistical analysis.

RESULTS: Delirium was detected in 28 patients according to the Nu-DESC. The ICC (intraclass correlation) is 0.97 in the 95% confidence interval from 0.96 to 0.98 for agreement between nurses and neurologists for the total Nu-DESC score. Weighted kappa rates were between 0.78 and 0.92. In the ROC analysis of the Nu-DESC scale, the optimum cutoff value calculated for the 1,344 observations and 112 patients was determined as >1 according to the maximum sensitivity and the specific situation. Sensitivity at the cut-off point was 92.27; specificity was determined as 92.72. The Youden index was found to be $J = 0.845$ ($0 < J = 0.845 < 1$).

CONCLUSION: We believe that Turkish translation of Nu-DESC is valid and reliable for clinicians, nurses, and researchers and will contribute to delirium studies.

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89.

The experimental research on neuroplasticity in rats' hippocampus subjected to chronic cerebral hypoperfusion and interfered by Modified Dioscorea Pills.

Li HB, Liang WB, Zhou L

Heliyon. 6(1):e02897, 2020 Jan.

[Journal Article]

UI: 31909235

BACKGROUND: Chronic Cerebral Hypoperfusion (CCH) is a common, crucial and tough problem for old people. It easily leads to Lacunar Infarction and even Vascular Dementia (VD). Western medicine has the advantage to relieve some VD symptoms but fails to cure it. Some classic Chinese medicines have good efficacies to treat and delay the cerebral functional decline resulted from CCH. Among them Modified Dioscorea Pills (MDP) has been proven to have a convincing effect in curing VD. So far the knowledge about neuroplasticity in CCH is little known and the underlying interfered mechanism by MDP on neuroplasticity has not yet been explored. This study explores the changes of neuroplasticity involving neurogenesis, angiogenesis and synaptogenesis in CCH and interfered by MDP.

METHODS: 40 male SD rats were divided into the Sham operated Group, the Model Group and the MDP Group according to a Random Number Table. Bilateral Common Carotid Arteries Occlusion (BCCAO) was adopted to prepare CCH models. MDP condense decoction had been administered by gavage to rats in the MDP Group (10g.Kg-1.d-1) for 45 days; Rats in the other two groups were accepted normal salts as substitution with same dosage and course. Through Morris Water Maze (MWM) test, pathological observation of hippocampus, ultrastructural study on synapse, Real Time Polymerase Chain Reaction (RT-PCR) and immunohistochemistry detection, the capacities of intelligence of rats, the morphological character of hippocampus CA1 zone and the synapse associated protein and gene such as Growth Associated Protein (GAP-43) mRNA, Vascular Endothelial Growth Factor (VEGF) mRNA, Microtubule-associated Protein (MAP)-2, Synaptophysin (SYP), Postsynaptic Density protein (PSD)-95 and Micro Vessel Density (MVD) were determined. Through one-way ANOVA the data was analyzed and when $P < 0.05$ the result was considered significant.

RESULTS: Compared to the Model Group, rats in the MDP Group achieved much better behavioral performance ($P < 0.05$); more neurons and more synapses regenerated; the expression of SYP, PSD-95 and MAP-2 up-regulated ($P < 0.05$); The expressions of GAP-43 mRNA and VEGF mRNA in the Model Group were higher

than those in the Sham operated Group ($P<0.05$), but they reached the highest in the MDP Group ($P<0.05$); The count of MVD in the Sham operated Group is the lowest, it is higher in the MDP Group and it reaches highest in the Model Group ($P<0.05$).

CONCLUSIONS: Some key genes promoting neuroplasticity such as GAP-43 mRNA and VEGF mRNA remarkably up-regulated in CCH, they only boost angiogenesis but fail to facilitate neurogenesis and synaptogenesis in CCH. However, accompanied by furtherly up-regulation of these two key genes, MDP obviously improves neurogenesis, synaptogenesis and temperate angiogenesis in CCH which may be underlying its good efficacy.

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90.

Comparision of Vital Surgical Parameters, after Administration of Midazolam and Dexmedetomidine for Conscious Sedation in Minor Oral Surgery.

Sivasubramani S, Pandyan DA, Ravindran C

Annals of Maxillofacial Surgery. 9(2):283-288, 2019 Jul-Dec.

[Journal Article]

UI: 31909006

AIM: The aim of this study is to compare the efficacy between midazolam and dexmedetomidine in relation to vital parameters, sedation score, pain score, cognitive judgment, and postoperative amnesia to the event in conscious sedation for minor oral surgical procedure.

MATERIALS AND METHODS: A sample size of 30 patients were selected in each group: Group M (midazolam) and Group D (dexmedetomidine).

RESULTS: The mean heart rate (HR) and systolic and diastolic blood pressure measurements were significantly higher in Group M after the 20th min when compared to Group D. The visual analog scale (VAS) scores of pain were not statistically significant between the two groups during the procedure, but at the time of discharge, statistically significant VAS scores were found between the two groups. Nine (30%) patients in Group D and 21 (70%) patients in Group M showed cognitive judgment impairment with the Stroop Color and Word Test at the 30th min.

CONCLUSION: The dexmedetomidine group of patients had reduced blood pressure

and HR. No significant differences were noted in oxygen saturation or in respiratory rate between the two drugs. Patients had better sedation, analgesia, lesser cognitive impairment, and amnesia in the dexmedetomidine group.

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91.

Relatively young T1D adults using fixed doses of insulin have higher diabetes distress levels in a sample of patients from a Brazilian tertiary hospital.

Silveira MSVM, Bovi TG, Pavin EJ

Diabetology & metabolic syndrome. 11:104, 2019.

[Journal Article]

UI: 31890038

BACKGROUND: Elevated rates of anxiety and depressive symptoms in Type 1 Diabetes patients (T1D) and high rates of diabetes-specific distress (DD) have been shown. Several factors may be responsible for increase the DD levels such as age, life changes, lack of familiar support, education, insulin regimens (IRs) and chronic complications. The goals of this study were: 1-to compare DD levels, anxiety and depressive symptoms according to age (< and >= 25 years old), 2-to evaluate the association between DD levels, anxiety and depressive symptoms and IRs, and 3-to evaluate the association between DD levels, anxiety and depressive symptoms and chronic complications.

METHODS: In a cross-sectional study, T1D patients receiving outpatient care at Unicamp tertiary hospital were included. Inclusion criteria were age at least 18 years old and diagnosis of T1D for 6 months. Exclusion criteria were cognitive impairment, major psychiatric disorders, severe diabetes-related complications, and pregnancy. Depressive symptoms were evaluated by the depression subscale of the Hospital Anxiety and Depression Scale (HAD-D) and the anxiety symptoms by the anxiety subscale of the same instrument (HAD-A). DDS scale assessed DD. Glycemic control was evaluated by HbA1C. The latest lipid panel results were recorded and IRs and chronic complications were obtained through chart review.

RESULTS: Of all 70 patients, 70% were younger than 25 years old. No differences

were found between two groups according to gender, education, and income ($p = 0.39$, $p = 0.87$, and $p = 0.52$, respectively). HbA1c mean was 10% in both groups ($p = 0.15$). Older patients had higher levels of total DD and physician DD than younger ($p = 0.0048$ and $p = 0.0413$; respectively). Total DD and DD on subscales 1 and 2 were higher in patients using fixed doses of insulin compared to variable doses according to carbohydrates count ($p = 0.0392$, $p = 0.0383$ and $p = 0.0043$, respectively). No differences were found between anxiety and depressive symptoms and age and IRs. Similarly, no differences were found among DD levels, anxiety and depressive symptoms in patients with and without chronic complications.

CONCLUSIONS: When providing education and care for T1D patients, health providers should consider age, patient's developmental stage, with its related demands and the burden of insulin regimen.

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Year of Publication

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92.

The effectiveness of cognitive training on improving cognitive function and sleep quality in community-dwelling elderly in Iran.

Keramtejad M, Azadi A, Taghinejad H, Khorshidi A

Sleep Science. 12(2):88-93, 2019 Apr-Jun.

[Journal Article]

UI: 31879540

BACKGROUND AND AIM: Given the ever-increasing of the older adults population and in order to achieve healthy and active ageing goals and improvement in the cognitive function and sleep quality in older adult, this study aimed to evaluate the effect of cognitive training program on improving cognitive function and ageing-related sleep quality in community-dwelling elderly in Iran, in 2018.

METHODS: This was an experimental study. The sample comprised 420 older adults who were a member of the comprehensive health center in one of the southern cities of Iran. 164 had a cognitive problem and sleep disorder, of whom 108 were selected by the available method and based on inclusion criteria. Participants were randomly allocated to an experimental group ($n=54$) and a control group ($n=54$). Experimental group samples were undergoing the intervention for two months. Data were collected using MMSE questionnaire, Pittsburgh sleep quality, insomnia severity index and

Clinical Dementia Rating Scale (CDR). Data were collected one month before-and-after the intervention. Data were analyzed using SPSS software.

RESULTS: There was no significant difference between the mean score of cognitive function and sleep quality in both the intervention group and the control group before the intervention ($p>0.05$). At the post-test, the mean (SD) of elderly adults' cognitive function in the intervention and control groups were 2.7 (1.3) and 3.44 (1.7), respectively ($p=0.017$). Moreover, the mean (SD) of sleep quality in older adults in the intervention and the control group was 6.76 (2.3) and 9.25 (2.36), respectively ($p<0.001$).

CONCLUSION: Given the obtained results in the current study the cognitive training promotes cognitive function and sleep quality in older adults. Therefore, since this program is effective, low-cost and applicable, it can be used to improve cognitive function and sleep disorder in the older adult population.

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1

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93.

Effect of psychotic symptoms on cognitive impairment in patients with bipolar disorder.

Achalia RM, Nagendra B, Achalia G, Chopade M, Sable A, Venkatasubramanian G, Rao NP

Industrial Psychiatry Journal. 28(1):115-122, 2019 Jan-Jun.

[Journal Article]

UI: 31879457

BACKGROUND: A considerable proportion of patients with bipolar disorder (BD) have psychotic symptoms during the illness. This subset of BD due to its genetic susceptibility and family segregation has considerable overlap with schizophrenia. However, the extent to which BD patients with psychotic symptoms and without psychotic symptoms differ on neurocognitive functions is still not completely clear.

AIM: The aim of this study was to examine the neurocognitive functions in BD patients with psychotic symptoms and BD without psychotic symptoms in comparison

with healthy volunteers (HVs).

MATERIALS AND METHODS: Thirty patients with Diagnostic and Statistical Manual of Mental Disorders, Fourth Edition BD (16 with psychotic symptoms) and thirty age- and sex-matched HVs were recruited in the study. Clinical severity was assessed using structured rating scales. The presence of psychotic symptoms was assessed using the Lifetime Dimensions of Psychosis Scale (LDPS). All patients underwent tests, namely continuous performance test, Stroop Word-Color interference test, and Wisconsin Card Sorting Test, to measure executive functions. Differences between groups were examined using analysis of covariance with age and sex as covariates.

RESULTS: There was a significant difference between groups on the performance of all the three tests, with patients performing poorer than HVs. While the HVs differed from both BD with psychotic symptoms and without psychotic symptoms, there was no difference between BD patients with and without psychotic symptoms. There was no significant correlation between LDPS score and scores on neurocognitive tests.

CONCLUSION: The study findings, at least with respect to cognitive function, suggest that BD with psychotic symptoms may not be a categorically distinct subtype of BD. Cognitive functions need to be assessed in all patients with BD, regardless of psychotic symptoms, and remediation interventions need to be provided.

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94.

Psychiatric morbidity, cognitive dysfunction and quality of life in drug-naive patients with Parkinson's disease: A comparative study.

Dhar SS, Jeenger J, Singroha V, Sharma M, Mathur DM
Industrial Psychiatry Journal. 28(1):13-18, 2019 Jan-Jun.

[Journal Article]

UI: 31879441

BACKGROUND: To better understand the psychiatric disorders and cognition in Parkinson's disease (PD) and its impact on quality of life (QoL), patients need to be studied soon after diagnosis, before initiation of dopamine replacement therapy.

AIM: This study aims to compare the nature and frequency of psychiatric morbidity, cognitive dysfunction, and quality of life in drug-naïve patients with PD and healthy controls.

MATERIALS AND METHODS: The cross-sectional, comparative study was conducted in tertiary care center. Fifty drug-naïve PD patients and fifty healthy controls were included and assessed on Modified Hoehn and Yahr scale, PD Questionnaire 8, Kolkata cognitive screening battery, General Health Questionnaire-12, and Hamilton Anxiety and Depression Rating Scale (HAM-A and HAM-D).

RESULTS: The mean scores of HAM-A and HAM-D of patients with PD were significantly higher than that of the comparison group. The patients with PD had statistically significant impairment in verbal fluency, Mini-Mental State Examination, calculation, memory immediate recall, visuoperceptual ability, and memory (delayed recall and recognition) in comparison to patients without PD. No statistically significant difference was observed with respect to object naming between the two groups.

CONCLUSION: QoL of a PD patient is adversely affected by both the motor and nonmotor symptoms of the disease such as depression, anxiety, apathy, sleep disturbances, and cognitive impairment. The link between nonmotor symptoms and reduced QoL has important implications for the management of PD because the nonmotor symptoms often appear before patients are given anti-parkinsonian therapy. Screening of nonmotor symptoms in early stage of disease will decrease the morbidity and mortality and improve the QoL.

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Comparative Study of Cognitive Function Between Treatment-Resistant Depressive Patients and First-Episode Depressive Patients.

Rao D, Xu G, Lu Z, Liang H, Lin K, Tang M

Neuropsychiatric Disease & Treatment. 15:3411-3417, 2019.

[Journal Article]

UI: 31849475

OBJECTIVE: Despite reports of cognitive dysfunction during the acute phase of depression, there is a lack of studies in patients with treatment-resistant depression (TRD). The aim of this study was to investigate the cognitive function profile of TRD and compare cognitive dysfunction between subjects with TRD and first-episode depression.

PATIENTS AND METHODS: The study included 31 patients with TRD and 53 with first-episode depression. Cognitive function was assessed by a series of neuropsychological tools such as the verbal fluency test, Modified Wisconsin Card Sorting Test (M-WCST), Tower of Hanoi test, Chinese-revision of the Wechsler Adult Intelligence Scale (WAIS-RC), and Trail Making Test A and B.

RESULTS: There were no significant demographic differences between the TRD, first-episode depression, and normal control groups (gender, age, years of education). The full-scale, verbal, and performance intelligence quotients measured with the WAIS-RC were also not significantly different ($p > 0.05$). The normal group scores were all significantly better than TRD and first-episode depression, and the TRD group performed significantly worse than subjects with first-episode depression on Trail Making Test B, two WCST subscales, and the profile score of the Tower of Hanoi test (all $p < 0.05$).

CONCLUSION: Patients with depression exhibited global impairments in cognitive function, and these were more common in TRD. Poor executive function may play an important role in TRD.

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Year of Publication
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96.

Discourse in aging: Narrative and Persuasive.
Babaei Z, Ghayoumi-Anaraki Z, Mahmoodi-Bakhtiari B
Dementia & Neuropsychologia. 13(4):444-449, 2019 Oct-Dec.
[Journal Article]
UI: 31844499

The growth in the elderly population has posed a social, economic and health challenge for the twenty-first century.

OBJECTIVE: Aging is often characterized by changes in cognitive functions which affect the receptive and expressive capabilities of language. Since language plays a significant role in human life, we evaluate the existence of age-related differences in narrative and persuasive discourses.

METHODS: The narrative discourse of 91 adults and persuasive discourse of 92 adults, aged from 19 to 75 years and stratified into four age groups, were examined.

RESULTS: There was a statistically significant difference between coherence in the elderly group and each of the other three age groups for both types of discourse. There was also a significant difference for the cohesion variable between the elderly and the first age group for narrative discourse only.

CONCLUSION: The results of this study demonstrate that discourse is influenced by aging and type (genre) of discourse task. Therefore, it is essential for clinicians to take into account the linguistic needs of elderly and incorporate these into their clinical programs. Also, this finding can help clinicians to distinguish between discourses of normal aging and other neurologic disorders (for example dementia, right hemisphere damage, aphasia).

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Publisher

O aumento da populacao idosa colocou um desafio social, economico e de saude

para o século XXI.OBJETIVO: O envelhecimento é frequentemente caracterizado pela presença de alterações nas funções cognitivas que afetam as capacidades receptivas e expressivas da linguagem. Como a linguagem desempenha um papel significativo na vida humana, avaliamos a existência de diferenças relacionadas à idade nos discursos narrativos e persuasivos.METODOS: O discurso narrativo de 91 adultos e o discurso persuasivo de 92 adultos com idade entre 19 e 75 anos, divididos em quatro faixas etárias, foram examinados.RESULTADOS: Existe uma diferença estatisticamente significativa entre a coerência no grupo de envelhecimento e cada um dos outros três grupos nos dois tipos de discursos. Existe também uma diferença significativa para a variável coesão entre idosos e a primeira faixa etária apenas no discurso narrativo.CONCLUSÃO: Os resultados deste estudo demonstram que o discurso é influenciado pelo envelhecimento e tipo da tarefa discursiva. Portanto, é essencial que os médicos levem em consideração as necessidades linguísticas dos idosos e as incorporem em seus programas clínicos. Além disso, esses resultados ajudam os médicos a distinguir entre discursos de envelhecimento normal e outros distúrbios neurológicos (por exemplo, demência, lesões do hemisfério direito, afasia).
Language: Portuguese
Year of Publication
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97.

Cognitive performance of children with spinal muscular atrophy: A systematic review.

Polido GJ, de Miranda MMV, Carvas N, Mendonça RH, Caromano FA, Reed UC, Zanoteli E, Voos MC

Dementia & Neuropsychologia. 13(4):436-443, 2019 Oct-Dec.

[Journal Article]

UI: 31844498

Spinal muscular atrophy (SMA) is genetic and progressive, caused by large bi-allelic deletions in the SMN1 gene, or the association of a large deletion and a null variant.

OBJECTIVE: To evaluate the evidence about cognitive outcomes in spinal muscular atrophy (SMA).

METHODS: Searches on the PUBMED/Medline, Web of Knowledge and Scielo databases retrieved 26 studies (1989 to 2019, descriptors "spinal muscular atrophy" and "cognition"). Nine studies were selected according to the eligibility criteria: (1) cognition tested in individuals with SMA; (2) written in English or Spanish. The Risk of Bias in Non-Randomized Studies of Interventions was used to describe design, bias, participants, evaluation protocol and main findings. This study was registered on the International prospective register of systematic reviews (PROSPERO).

RESULTS: Three studies described normal cognition. In another three studies, cognitive outcomes were above average. Cognitive impairment was found in three studies. Poor cognitive performance was more frequently reported in studies that were recent, included children with SMA type I and that employed visual/auditory attention and executive function tests. Protocols and cognitive domains varied, precluding meta-analysis.

CONCLUSION: The severity of motor impairment may be related to cognitive

outcomes: studies that included a higher number/percentage of children with SMA type I found cognitive impairment. The establishment of gold-standard protocols is necessary. Further studies should compare the cognitive outcomes of subjects with SMA types I to IV.

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Other Abstract

Publisher

A atrofia muscular espinhal (SMA) e genetica e progressiva, causada por grandes delecoes bi-alelicas no gene SMN1, ou pela associacao de uma grande delecao e uma variante nula.OBJETIVO: Avaliar as evidencias sobre o desempenho cognitivo na atrofia muscular espinhal (AME).METODOS: Pesquisas nas bases de dados PUBMED/ Medline, Web of Knowledge e Scielo localizaram 26 estudos (1989 a 2019, descritores "atrofia muscular espinhal" e "cognicao"). Nove estudos foram selecionados de acordo com os criterios de elegibilidade: (1) testaram a cognicao em pessoas com AME; (2) escritos em ingles/espanhol. A avaliacao do risco de vies em estudos com intervencoes nao-randomizadas foi utilizada para descrever o desenho experimental, vies, amostra, protocolo de avaliacao e principais achados. Este estudo foi aprovado no Registro Internacional Prospectivo de Revisoes Sistematicas (PROSPERO).RESULTADOS: Em tres estudos, foi registrado que a cognicao estava preservada. Em tres estudos, o desempenho cognitivo estava acima da media. O comprometimento cognitivo foi encontrado em tres estudos. Desempenho cognitivo pobre foi mais frequentemente relatado em estudos recentes, estudos que incluíram crianças com AME tipo I e estudos que incluíram atencao visual/auditiva e testes de funcao executiva. Protocolos e dominios cognitivos variaram muito, portanto nao foi possivel a realizacao de metanalise.CONCLUSAO: A gravidade do comprometimento motor pode estar relacionada ao desempenho cognitivo: estudos que incluíram maior numero/porcentagem de crianças com AME tipo I encontraram alteracoes no desempenho cognitivo. O estabelecimento de protocolos padrao-ouro e necessario. Novos estudos devem comparar o desempenho cognitivo de pessoas com AME tipos I a IV, ou seja, com diferencas no prognostico e no desempenho motor.

Language: Portuguese

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98.

Traditional Chinese Medicine Shenmayizhi Decoction Ameliorates Memory And Cognitive Impairment Induced By Scopolamine Via Preventing Hippocampal Cholinergic Dysfunction In Rats.

Wu Q, Cao Y, Liu M, Liu F, Brantner AH, Yang Y, Wei Y, Zhou Y, Wang Z, Ma L, Wang F, Pei H, Li H

Neuropsychiatric Disease & Treatment. 15:3167-3176, 2019.

[Journal Article]

UI: 31814724

PURPOSE: Clinical trials have illustrated that Shenmayizhi decoction (SMYZ) could improve the cognitive functions in patients with dementia. However, the mechanism needs to be explored.

METHODS: Fifty adult male rats (Wistar strain) were divided into five groups equally and randomly, including control, model, and SMYZ of low dose, medium dose and high dose. Rats in each group received a daily gavage of respective treatment. Rats in control and model group were administrated by the same volume of distilled water. Memory impairment was induced by intraperitoneal administration of scopolamine (0.7 mg/kg) for 5 continuous days. Four weeks later, Morris water maze (MWM) was performed to evaluate the spatial memory in all rats. Then, rats were sacrificed and the hippocampus was removed for further tests. Furthermore, Western blot analysis was employed to assess the levels of acetylcholine M1 receptor (M1), acetylcholine M2 receptor (M2), acetylcholinesterase (AChE) and cholineacetyltransferase (ChAT). AChE and ChAT activities were determined.

RESULTS: The SMYZ decoction significantly improved behavioral performance of rats in high dose. The SMYZ decoction in three doses exhibited anti-acetylcholinesterase activity. In addition, a high dose of SMYZ promoted ChAT activity. Moreover, a high dose of SMYZ increased the level of ChAT and declined the level of AChE assessed by Western blotting. Besides, an increased level of M1 receptor was found after treatment.

CONCLUSION: Shenmayizhi decoction could mitigate scopolamine-induced cognitive deficits through the preventative effect on cholinergic system dysfunction.

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99.

Reversion From Mild Cognitive Impairment To Normal Cognition: False-Positive Error Or True Restoration Thanks To Cognitive Control Ability?.

Chung JY, Yoon HJ, Kim H, Choi KY, Lee JJ, Lee KH, Seo EH
Neuropsychiatric Disease & Treatment. 15:3021-3032, 2019.

[Journal Article]

UI: 31749620

PURPOSE: Relatively little attention has been paid to the meaning of reversion from mild cognitive impairment (MCI) to cognitively normal (CN), compared to MCI progression studies. The purpose of the study was to investigate the characteristics contributing to reversion from MCI to CN and to identify the associated factors with such reversion.

PATIENTS AND METHODS: We retrospectively identified 200 individuals who initially diagnosed as MCI and completed the second visit from the National Research Center for Dementia (NRCD) registry in Korea. Participants underwent comprehensive clinical and neuropsychological assessments. Factors associated with reversion were examined by a independent-samples t-test, chi2 test, and logistic regression. Longitudinal change was examined by a repeated measures analysis of variance (rANOVA).

RESULTS: Based on the second assessment, 78 (39%) individuals were found to have reverted to CN (rMCI) and 118 (59%) remained with MCI (sMCI). Four (2%) progressed to Alzheimer's disease dementia and they were excluded from further analysis. Over a wide range of socio-demographic, clinical, and neuropsychological variables, group difference was significant only in neuropsychological tests of cognitive control. Both groups showed improvement in several neuropsychological tests, implying a practice effect, but the rMCI group showed greater improvement.

CONCLUSION: Reversion from MCI to CN might not be a false-positive error but a true recovery from cognitive impairment. Our results suggest that cognitive control ability may be a characteristic favorable for the restoration of cognitive function. Therefore, assessment of cognitive control might facilitate the development of appropriate interventions for MCI as well as prognosis evaluation.

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100.

Antibody Negative Autoimmune Encephalitis- Does it Differ from Definite One?.

Pradhan S, Das A, Das A, Mulmuley M

Annals of Indian Academy of Neurology. 22(4):401-408, 2019 Oct-Dec.

[Journal Article]

UI: 31736559

CONTEXT: Autoimmune encephalitis (AE) is an emerging cause of non-infective encephalitis, presentations of which vary widely. Traditionally the diagnosis of AE is based on detection of antibodies in a patient with clinical picture suggestive of AE.

AIM: To evaluate the clinical characteristics and response to immunotherapy in patients with antibody negative autoimmune encephalitis and to compare them with definite cases.

SETTINGS AND DESIGN: A prospective follow-up study was done in patients presenting with presumptive symptoms of AE from January 2017 to January 2019.

The study was done in a tertiary care institute of Northern India.

PATIENTS AND METHODS: Demographic and clinical parameters were noted and relevant investigations for management were done according to well-defined protocol. The patients were treated with immunomodulatory therapy in the form of steroids and/or intravenous immunoglobulins (IVIg). They were followed up for treatment response and relapse at 2 monthly intervals.

STATISTICAL ANALYSIS USED: The data was expressed as either proportions or mean/median. Chi-square test/Independent T test was used to compare antibody positive and antibody negative group.

RESULTS: Out of 31 patients with presumptive AE, 16 patients tested positive for autoimmune antibodies (definite AE). Incidences of seizure, behavioral abnormalities, dementia and altered sensorium were similar between the 2 groups ($p > 0.05$).

Complete or partial response was seen in all treated patients in both groups with no significant difference ($p 0.716$). CSF protein concentration and cellularity were higher in the definite group although only high protein concentration could reach statistical significance ($p 0.002$). Malignancy could be confirmed after extensive search in 2 out of 16 patients with definite AE and in 1 out of 15 antibody negative AE patients.

CONCLUSIONS: Clinical presentation of antibody negative cases does not differ significantly from definite ones. Since treatment response is also similar in both the groups, starting immunotherapy in a patient presenting with presumptive symptoms of AE, while ruling out other common mimickers, seems to be the need of the hour in the management of this evolving entity.

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